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MAINS SMASHER

SOCIAL JUSTICE

A COMPREHENSIVE SOLUTION FOR
UPSC MAINS 2026



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1 Women in India

Amartya Sen has aptly noted "**Women's empowerment is not just equity but a vital force for progress**". Indian women increasingly lead in politics, STEM, and grassroots movements, nonetheless, systemic gaps in safety, education, and representation persist, demanding sustained reform.

UPSC Mains PYQs

1. Women's social capital complements in advancing empowerment and gender equity. Explain. **(2025)**
2. "Though women in post-Independent India have excelled in various fields, the social attitude towards women and the feminist movement has been patriarchal." Apart from women education and women empowerment schemes, what interventions can help change this milieu? **(2021)**
3. "The reservation of seats for women in the institutions of local self-government has had a limited impact on the patriarchal character of the Indian Political Process." Comment. **(2019)**

1. Constitutional Provisions for Women in India

Article	Provision
Fundamental Rights	
Article 14	Equality before the law and equal protection of the law.
Article 15(1)	Prohibits discrimination on grounds of religion, race, caste, sex, or place of birth.
Article 15(3)	Permits the State to make special provisions for women and children.
Article 16	Equality of opportunity in public employment.
Directive Principles of State Policy (DPSP)	
Article 39(a)	Directs the State to ensure equal means of livelihood for men and women.
Article 39(d)	Mandates equal pay for equal work.
Article 42	Directs the State to ensure just and humane conditions of work and maternity relief.
Fundamental Duties	
Article 51A(e)	Enjoins citizens to renounce practices derogatory to women's dignity.
Other Provisions	
Article 243D	Mandates not less than one-third reservation for women in PRIs.
Article 243T	Mandates not less than one-third reservation for women in urban local bodies.

2. Status of Women in India

Indicator	Fact
Sex Ratio	• Overall sex ratio was 943 females per 1,000 males in 2011 **(Census 2011),** which is projected to improve to 952 by 2036 **(Economic Survey 2024).** • As per NFHS -5, the sex ratio of the total population (females per 1000 males) is 1020 for the country as a whole, with sex ratio rural at 1037 and sex ratio urban at 985.
Sex Ratio at Birth	Improved to 929 in 2015-16 from 918 in 2014-15 (NFHS-5, **2019-21).**
Female Literacy	The female literacy rate reached 74.6%, with rural female literacy at 70.4% **((PLFS) 2023–24).**
Higher Education	The Gross Enrolment Ratio (GER) for girls at the higher secondary level reached 60.9% **(UDISE+ 2024–25).**

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Maternal Mortality Ratio (MMR)	Declined to 97 per lakh live births in 2018-20 from 130 in 2014-16 **(Sample Registration System, 2018-20).**
Institutional Deliveries	• **NFHS-6:** 90.6% of births occur in institutions, up from 88.6% in **NFHS-5, 2019-21.** • **Rural Areas: 89.0%** of births were institutional • **Urban Areas: 95.2%** of births were institutional
Anaemia Prevalence	57.2% of non-pregnant women aged 15-49 are anaemic **(NFHS-5, 2019-21).** **NFHS 6: Pregnancy Iron Folic Acid Supplementation Uptake:** 100+ Days Consumption: Increased to 54.9% (up from 44.1%), 180+ Days Consumption: Increased to 37.8% (up from 26.0%)
Female Labour Force Participation Rate (LFPR)	40% in 2025, with rural female LFPR at 45.9% **(PLFS Annual Report Jan-Dec 2025).**
Women in MSMEs	Women own 20.5% of registered MSMES, approximately 4,667,278 enterprises **(Udyam Portal, 2024).**
Political Representation	14% of Lok Sabha MPs (78 out of 543) and 46% of PRI elected representatives are women in 2024 **(Election Commission of India, 2024; MoPR, 2024).**
Crimes Against Women	4,41,534 cases registered in 2024, averaging 1,210 cases per day. **(NCRB Crime in India Report, 2024).**
Child Marriage	• **NFHS 6: 20.1%** of women aged 20-24 were married before age 18, down from 23.3% in **NFHS-5, 2019-21.** • **State-Level Variations:** High prevalence in Bihar (34.6%), Jharkhand (28.1%), West Bengal vs. Low prevalence in Kerala (2.9%), Goa (6.8%), and Chandigarh (5.2%).

3. Committees and Commissions on Women's Issues

Committee	Key Recommendations
National Commission on Women (1992)	• Advocated for **stronger laws on domestic violence** and **workplace harassment**; recommended **amendments to the Dowry Prohibition Act.** • **Example**: *Protection of Women from Domestic Violence Act, 2005*.
Justice Verma Committee (2012)	• Recommended **criminalising marital rape**, fast-track courts for sexual violence, and stricter penalties for acid attacks. • **Example**: *Criminal Law (Amendment) Act, 2013.*
High-Level Committee on Status of Women (2015)	• Proposed a **Gender Atlas**, **women's hostels**, and **increased budget allocation** for women's safety. • **Example**: *One-Stop Centres (Sambal)*.
Swarna Jayanti Shahari Rozgar Yojana Review Committee (2010)	• Emphasised **women's inclusion in urban livelihood programmes** and skill development. • **Example**: Women SHG Formation under *Deendayal Antyodaya Yojana-NULM*.
Pam Rajput Committee (2012)	• Recommended **gender budgeting**, women's reservation in legislatures, and measures to **address violence against women.**
Kumud Bansal Committee (2005)	• Focused on **improving girls' education**, recommending infrastructure like hostels and scholarships for rural girls. • **Example**: *Sarva Shiksha Abhiyan's* gender-focused interventions.
Hema Committee (2019)	• **Investigated sexual harassment** in the Malayalam film industry, recommending **Internal Complaints Committees** and **gender sensitisation programs**.

4. Key Feminist Theories on Social Justice

Scholar /Thinker	Key Idea	Relevance to Social Justice	Example
Amartya Sen	**Capability Approach**: Emphasises enhancing women's capabilities (education, health, economic opportunities) to achieve **substantive freedom**, rather than mere equality in laws.	**Addresses systemic barriers** by focusing on enabling women to realise their potential, aligning with the social justice's aim of **equitable outcomes**.	Sen's approach justifies *Beti Bachao, Beti Padhao* for improving girls' education and survival. Lakhpati Didi Scheme: Goes beyond credit access to ensure women have the capability (skills/market link) to earn ₹1 lakh/year.

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Martha Nussbaum	**Central Human Capabilities**: Lists ten capabilities (such as bodily integrity, political participation) essential for women's dignity, advocating state intervention to ensure these.	**Promotes affirmative action** and protective laws to **counter patriarchal oppression**, ensuring women's inclusion in **social justice frameworks**.	Nussbaum's framework supports *One-Stop Centres for violence survivors.* The POSH Act (2013): Protects "Bodily Integrity" and "Senses/ Thought" by ensuring a safe environment for women to work.
Kimberlé Crenshaw	**Intersectionality**: Highlights how gender intersects with caste, class, and region, creating compounded discrimination for marginalised women (such as Dalit women).	**Ensures social justice** addresses overlapping vulnerabilities, advocating targeted policies for **equitable resource distribution.**	Crenshaw's lens explains *high anemia rates* among rural women *(NFHS-5)* due to *class-gender intersections.* **Stand-Up India:** Specifically targets SC/ST women entrepreneurs, recognizing the double burden of caste and gender.
Sylvia Walby	**Patriarchy as a System**: Views patriarchy as a dynamic system operating through institutions (family, state, economy), requiring structural reforms for gender equality.	Calls for **dismantling patriarchal norms** via policy and cultural shifts, aligning with social justice's **transformative goals.**	Walby's theory supports *Nari Shakti Vandan Adhiniyam* to reform political structures.
Naila Kabeer	**Empowerment as Agency**: Defines empowerment as women's ability to make strategic life choices through access to resources, agency, and achievements.	Emphasises economic and social empowerment as core to social justice, ensuring women's autonomy in decision-making.	Kabeer's framework validates *Deendayal Antyodaya Yojana-NRLM's* impact on 8.9 crore women.

5. Status of Women in India

Achievements	Challenges
• **Improved Demographics:** The overall sex ratio has improved to 1020 (NFHS-5), reflecting a more balanced gender composition • **Example**: *Kerala and Puducherry* had favourable sex ratios in 2011 (Census 2011).	• **Skewed Sex Ratios**: Practices like female foeticide in many states persist due to son preference, leading to regionally skewed sex ratio (NFHS-5). • **Example**: Illegal sex-selection persists in affluent urban pockets.
• **Educational Gains**: Female literacy has risen to 74.6% (PLFS 2023–24), while the Gross Enrolment Ratio (GER) for girls at the higher secondary level reached 60.9% in 2024–25 (UDISE+). • **Example**: *Vigyan Jyoti* promotes women in *STEM fields.*	• **Educational Disparities**: Only 8.3% of females aged 15 years & above complete graduation (as per NSS 75th round, 2017-18), and the average number of years completed in formal education by rural females is 6.4 years (as per NSS 79th Round, 2022-23). • **Example**: *Dropout rates remain high in Bihar* due to early marriage.
• **Health Improvements**: MMR declined to 97/lakh live births (2018-20), and institutional deliveries rose to 88.6% (NFHS-5). • **Example**: *PM Matru Vandana Yojana* incentivises maternal care.	• **Health Inequities**: 57.2% of non-pregnant women are anaemic (NFHS-5), with only 52.2% receiving adequate antenatal care. • **Example**: *Rural Jharkhand* lacks sufficient healthcare facilities.
• **Economic Participation**: Women own 20.5% of MSMEs (Udyam Portal) and constitute 47.6% of startup founders (DPIIT, 2023). • **Example**: *Lakhpati Didi* initiative is creating mass micro-entrepreneurship.	• **Low LFPR**: Female LFPR is 40% (PLFS Jan-Dec 2025). • **Example**: Urban women face a high unemployment rate at 6.4%in the 15-29 age group (PLFS Jan-Dec 2025).
• **Political Representation**: 46% of PRI elected members are women (2024), bolstered by the *106th Amendment Act* for 33% reservation. • **Example**: *Women-led panchayats* prioritize water and sanitation *(Esther Duflo study).*	• **Underrepresentation**: Only 14% of Lok Sabha MPs are women (2024), with just 13.7% in the Council of Ministers.

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6. Welfare Schemes and Their Impact

Scheme: Objective	Impact	Challenges
Beti Bachao, Beti Padhao: Improve the Child Sex Ratio (CSR) and girl child education.	Increased institutional focus on the girl child; CSR showing improvement in 104 districts. **Example**: *Haryana's campaigns* improved the sex ratio at birth.	Limited reach in high-prevalence states like **Bihar, where child marriage exceeds 40%.** **Publicity over Substance:** nearly 80% of funds were spent on media ads rather than health/ education.
PM Matru Vandana Yojana: Cash incentives for maternal and child health.	Supported more than 4.5 crore women with maternity benefits (2025). **Example**: Increased *institutional deliveries in rural UP.*	**Delayed disbursements** and low awareness in remote areas like tribal areas in Odisha.
Deendayal Antyodaya Yojana-NRLM: Empower women through SHGs for livelihoods.	10.05 crore rural women mobilised into more than 90.90 lakh SHGs, enhancing financial inclusion. **Example**: *SHGs in Tamil Nadu* boosted micro-enterprises.	**Lack of market linkages and credit access** limits scalability in rural areas. Most SHG loans remain small survival credits rather than larger entrepreneurial capital.
Nari Shakti Vandan Adhiniyam, 2023: Reserve 33% of seats for women in legislatures.	Expected to increase women's political voice post-implementation. **Example**: *Rwanda's zipper method* achieved 60% female MPs.	**Delayed implementation** and resistance from patriarchal structures.
Sambal (One-Stop Centres): Offer medical, legal, and shelter support for violence survivors.	Successfully integrated into **Mission Shakti**; 896 OSCs are now functional 24/7. **Ex:** Integration with the **112 Universal Helpline** for rapid rescue.	**Inadequate infrastructure** and trained staff in rural areas like Jharkhand.

7. Ways to Reduce Gender Inequality in India

Intervention	Description	Impact Potential	Example
Social Norm Campaigns with Men's engagement	Combined focus on challenging patriarchal stereotypes and engaging men/boys as partners in change.	**Shifts societal attitudes**, reducing tolerance for violence.	**GEMS Program (Gender Equity Movement in Schools):** Sensitises young boys to reduce future violence and bias.
Legal & Institutional Reform	**Fast-track courts** and **women-friendly police stations** to ensure justice.	Increases the **deterrence effect** and encourages victims to report crimes without fear.	*Uttar Pradesh state's FTCs disposed of over 12 lakh cases in 2023 by maintaining over 370 functional FTCs.* Scheme of Fast Track Special Courts: 725 FTSCs, including 392 exclusive POCSO Courts are functional(2025)
Political Mainstreaming	Ensuring women are "decision-makers" in policy.	Shifts public spending toward **human capital** (health, education).	**Nari Shakti Vandan Adhiniyam (106th Amendment Act):** mandates 33% of women in Parliament to influence national laws.
Economic Agency & Asset Ownership	Promote **financial inclusion** through microfinance and asset ownership.	Enhances women's agency and **bargaining power.**	**PM Awas Yojana:** By mandating the house be in the woman's name, it transforms her from a resident to an owner.
Digital Literacy Initiatives	**Equip women with digital skills** to access opportunities and report violence.	Bridges the digital divide, **empowering rural women.**	**Internet Saathi by Google & Tata:** Trained over 30 million rural women to use the internet for livelihoods.

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8. Way Forward

Aspect	Analysis	Recommendations
Social Justice Perspective	Empowerment is the primary tool to dismantle the "Triple Burden" of patriarchy, caste, and poverty.	**Strengthen monitoring of schemes**, integrate gender budgeting, and promote **community-driven awareness** to **dismantle patriarchal norms.**
Intersectionality	Challenges vary across caste, class, and region. Dalit women face compounded discrimination, while rural women lack access to digital and healthcare resources.	**Localised Policy Design:** Implement "sub-plans" within Mission Shakti specifically for **Particularly Vulnerable Tribal Groups (PVTGs)** and Dalit entrepreneurs.
Economic Imperative	A 10% rise in women's labour force participation could add $770 billion to India's GDP by 2030. (McKinsey).	**Formalise informal sector** work, expand care economy investments, and promote women-led enterprises through credit and market linkages.
Global Alignment	Aligning with CEDAW and **adopting models like Rwanda's zipper method** can accelerate progress toward **Viksit Bharat by 2047.**	**Enforce CEDAW compliance,** criminalise marital rape, and adopt global best practices like gender quotas in legislatures.

India's progress on **women's status aligns with SDG-5**, aiming for gender equality and empowerment. Though strides have been made, sustained efforts are vital to ensure full participation and **foster inclusive, equitable development.**

Legislation for Women in India

Legislation for women in India aims to **promote gender equality, protect rights**, and address **historical and structural discrimination.** These laws span domains like safety, employment, health, inheritance, and justice to empower women and **ensure their dignity and security.**

1. Family and Personal Laws

A. Protection of Women from Domestic Violence Act, 2005

Key Provisions	Details	Impact
Protection orders	Restricts perpetrators from contacting or harming the victim.	Enhances victim safety, but underreporting due to stigma.
Residence orders	Allows women to remain in the shared household.	Prevents homelessness; empowers women to assert housing rights.
Monetary relief	Provides compensation for losses or expenses due to abuse as financial support.	Ensures economic stability; implementation varies by region.
Counselling and legal aid	Facilitates access to support services for victims.	Promotes holistic recovery; limited awareness hinders access.

B. Dowry Prohibition Act, 1961

Key Provisions	Details	Impact
Prohibition on dowry exchange	Bans the act of giving, taking, or abetting dowry. Includes "presents" given without free consent.	Aims to curb dowry-related violence; cultural acceptance undermines enforcement.
Stringent Penalties	Minimum 5 years imprisonment and a fine of ₹15,000 or the value of the dowry (whichever is more).	Deters dowry practices; high pendency & low conviction rates weaken the impact.
Fines for non-compliance	Monetary penalties alongside imprisonment.	Intended as an additional deterrence, but poor monitoring and legal delays dilute implementation.

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C. Hindu Succession Act, 1956 (Amended 2005)

Key Provisions	Details	Impact
Equal Coparcenary Rights (Section 6)	Daughters are recognised as coparceners by birth with the same rights and liabilities as sons in ancestral property.	Promotes gender equality in inheritance and weakens patriarchal property structures.
Equal Partition Rights	Married and unmarried daughters can equally demand a partition of joint family property.	Enhances women's financial independence and property access.
Absolute Ownership (Section 14)	Property possessed by a Hindu female is held as her absolute property, abolishing limited ownership.	Strengthens women's economic security and decision-making power over property. (can sell, gift, or mortgage property to fund businesses, education, etc.)
Abolition of Gender-Biased Disqualifications	Repealed Sections 23 and 24, removing restrictions on women's inheritance and partition rights.	Protects widows and daughters from discriminatory inheritance practices.
Expanded Class I Heirs	The 2005 amendment expanded Class I heirs to include additional descendants through female lineage.	Ensures wider and more equitable inheritance protection for descendants.
Testamentary Freedom (Section 30)	Hindus can transfer their share of property, including coparcenary share, through a will.	Enhances individual control over property, though misuse may lead to disinheritance of daughters.

D. Hindu Marriage Act, 1955

Key Provisions	Details	Impact
Section 24: Maintenance	Financial support for either spouse lacking income during marriage or divorce.	Ensures financial stability; legal delays hinder timely relief.
Section 25: Permanent Alimony	Permanent financial support to either spouse after divorce under Section 25	Supports economic independence; societal pressure to reconcile limited use.
Section 13B: Mutual Consent Divorce	Allows divorce by mutual consent when both parties agree to dissolve the marriage.	Provides exit from abusive marriages; underutilised due to stigma.

2. Workplace and Economic Empowerment Laws

A. Equal Remuneration Under the Code on Wages, 2019

Point	Description
Equal Remuneration	Employers cannot discriminate on the ground of gender in payment of wages for the same or similar work.
Non-Discrimination in Recruitment	Employers cannot discriminate in recruitment for the same or similar work, except where women's employment is legally restricted.
Replacement of Earlier Law	The Code replaced the Equal Remuneration Act, 1976.
Use of 'Gender'	The Code uses the broader term "gender" instead of only "men and women."
Coverage	Applies across organised and unorganised sectors.

Impact of Equal Remuneration Provisions

Point	Description
Gender Equality and Empowerment	Promotes equal economic rights and workplace equality.
Reduction in Wage Gap	Aims to reduce gender-based wage discrimination.
Inclusive Protection	Extends protection beyond the traditional male-female framework.
Fair Labour Practices	Encourages transparent and non-discriminatory employment practices.

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B. Maternity Benefit Act, 1961 (Amended 2017)

Key Provisions	Details	Impact
26 weeks of paid maternity leave	Increased from 12 to 26 weeks for the first two children (12 weeks for subsequent children).	Enhances maternal & infant health; limited reach in informal sectors.
Mandatory Crèche Facility	Mandatory for establishments with 50+ employees; allows 4 daily visits.	Supports working mothers; inconsistent implementation in smaller firms.
Work-from-home options post-leave	Allows flexibility post-26-week leave, subject to employer agreement.	Improves retention; awareness is low among rural workers.
12 weeks' leave for commissioning mothers	Provides 12 weeks of leave for mothers who adopt a child below 3 months or use a surrogate.	Supports surrogacy; limited awareness of provision.
Nursing & Miscarriage Leave	Two nursing breaks daily until the child is 15 months and 6 weeks' leave for medical termination of pregnancy.	Protects women's & children's health post-loss; underutilised due to stigma.

C. Sexual Harassment of Women at Workplace (Prevention, Prohibition, and Redressal) Act, 2013

The legislation was heavily derived from the landmark Supreme Court ruling in the Vishaka and Others v. State of Rajasthan (1997) case.

Key Provisions	Details	Impact
Internal Complaints Committees (ICC)	Mandatory for every office with 10+ employees; 50% of members must be women. Mandatory External Member (NGO/Legal background) to ensure unbiased trial.	Fosters safe workplaces; poor ICC implementation in small firms.
Requires POSH policies	Employers must adopt anti-harassment guidelines and conduct training.	Promotes accountability & prevents underreporting due to fear of retaliation.
Provides redressal mechanisms	Ensures timely, confidential resolution of complaints.	Empowers victims to seek justice, delaying small organisations.
Fines for non compliance	Penalties up to ₹50,000 for failing to form ICCs or implement policies.	Deters employer negligence; enforcement is inconsistent.
Covers students and patients	Extends protection to schools, colleges, and hospitals.	Broadens the safety net; but low awareness in non-corporate settings.
Local Committee (LC)	District-level committees for the Informal Sector and small shops with <10 employees.	Enhances access to justice; LC's functionality varies by region.

3. Protection Against Violence and Exploitation

A. BNS Section 75 (Formerly IPC 354A)

Feature	Details
Legal Definition	• Criminalises unwelcome sexual advances and explicit behaviour against women.
Offenses Covered	• Physical contact and explicit advances • Demands or requests for sexual favors • Showing pornography against her will • Making sexually colored remarks
Severe & Minor Punishment	• Up to 3 years of rigorous imprisonment and/or fine (for physical advances, favours, or pornography). • Up to 1 year imprisonment and/or fine (for sexually coloured remarks).
Bail Status	• Non-bailable for physical advances, favours, and pornography. • Bailable for sexually coloured remarks.
Jurisdiction	• Broad criminal law applicable anywhere in India allows direct FIR filing with police.

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B. Protection of Children from Sexual Offences Act (POCSO), 2012

Key Provisions	Details	Impact
Child Protection	Defines sexual assault, harassment, and pornography involving minors (under 18) and applies to both boys & girls.	Heavily safeguards girls; societal silence hinders reporting.
Child-friendly procedures	Ensures sensitive handling of cases, like Zero contact with the accused during testimony	Reduces trauma for victims.
Fast-tracks trials for speedy justice	Special courts for POCSO cases.	Accelerates justice; backlog persists in courts.

4. Reproductive and Health Rights

A. Medical Termination of Pregnancy Act, 1971 (Amended 2021)

Key Provisions	Details	Impact
Abortion up to 24 weeks	Extended from 20 to 24 weeks for "special categories" (survivors of sexual assault, minors, and disabled women), unmarried women included.	Enhances reproductive autonomy; rural access limited.
Spousal Consent	No spousal consent required; it empowers women's independent decisions.	Strengthens personal liberty; social stigma persists.
Provisions for minors	Allows abortion with guardian consent.	Protects vulnerable girls; awareness low in remote areas.
Doctors & Medical Boards	One doctor is required for up to 20 weeks; two doctors for 20–24 weeks. No upper limit for abortion if a State-level Medical Board (post 24 weeks) confirms substantial fetal abnormalities.	Technological Alignment: Recognises that some anomalies are only detectable late in pregnancy, preventing forced Continuation of pregnancy.
Confidentiality (Section 5A)	Ensures the privacy of women seeking abortion.	Upholds dignity by reducing social stigma; enforcement varies by facility.
Rights of Unmarried Women	2021 amendment included unmarried women under contraceptive failure as a ground for abortion. In 2022, the Supreme Court (X v. Principal Secretary, Health & Family Welfare Department (2022)) extended access to abortion up to 24 weeks for unmarried women on par with married women.	Eliminates marital-status discrimination and strengthens reproductive choice; access remains constrained by stigma and provider bias.

B. Prohibition of Child Marriage Act, 2006

Key Provisions	Details	Impact
Minimum Age of Marriage	Currently 18 for girls and 21 for boys. Any marriage involving a minor is voidable at the option of the minor. **Example:** NFHS-6 shows a decline in child marriages (20.1%) due to enforcement.	Promotes education and health; cultural traditions resist change.
Right to Annulment	A child can file a petition for annulment up to 2 years after reaching majority.	Empowers girls to exit forced marriages; underutilised.
Penalties for facilitators	Punishes parents, guardians, and solemnisers (priests/clerics) with imprisonment up to 2 years and/or a fine of ₹1 lakh.	Deters violations; enforcement is inconsistent in rural areas.

C. Surrogacy (Regulation) Act, 2021 (and 2024 Rules)

Key Provisions	Details	Impact
Altruistic Mode Only	Permits surrogacy only for Indian married couples or widows/divorcees (aged 23-50) without commercial gain. Prohibits commercial surrogacy.	Protects surrogates from exploitation; restricts access for single women.
Conditions for the Surrogate	Surrogacy (Regulation) Amendment Rules, 2023 relaxed conditions regarding donor gametes (allowing a donor egg or sperm if one spouse has a certified medical condition).	Expands eligibility for infertile couples with certified medical conditions while maintaining regulated access and preventing misuse.

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Regulatory Authority	National and State Surrogacy Boards monitor clinics. All clinics must be registered and follow strict Standard Operating Procedures (SOPs).	Strengthens regulation; Curbs illegal "IVF-surrogacy shops"; bureaucratic delays are possible.
Mandatory insurance for surrogate	Covers the surrogate's health for 36 months post-delivery (2024 Rules).	Enhances surrogate welfare; enforcement challenges remain.

5. Institutional and Support Mechanisms

A. National Commission for Women Act, 1990

Key Provisions	Details	Impact
Establishes NCW for advocacy	Addresses gender discrimination and violence. **Example:** NCW handled 2,500 workplace harassment complaints in 2021.	Amplifies women's voices; urban-centric focus limits reach.
Recommends policy reforms	Advises the government on women's rights laws.	Shape gender-sensitive policies; resource constraints persist.
Investigate rights violations	Probe cases of discrimination or abuse, however, final findings are purely recommendatory;	Ensures accountability; limited enforcement powers.

B. Legal Services Authorities Act, 1987

Key Provisions	Details	Impact
Free legal aid at central level	For women unable to afford legal support.	Enhances access to justice; bureaucratic delays hinder.
Lok Adalats (People's Courts)	Special courts for amicable settlement of disputes, especially relevant in rural areas.	Quick, low-cost resolution; useful for family and civil matters. May not address complex legal issues.
State-level legal services	Extends aid through state authorities.	Broadens reach; awareness low in rural areas.
District-level support	Local legal aid for grassroots access.	Supports marginalised women; inconsistent implementation.

C. Nari Shakti Vandan Adhiniyam, 2023

Key Provisions	Details	Impact
Reservation for women	Reserves one-third of seats for women in Lok Sabha.	Enhances women's political representation; delayed implementation.
Women's reservation in State Assemblies	33% reservation for women in State Legislative Assemblies and the Delhi NCT Assembly.	Promotes gender equity in governance; delimitation delays impact rollout.
Applies to SC/ST reserved seats	Extends reservation to seats reserved for Scheduled Castes and Tribes.	Ensures inclusive representation and complexity in seat allocation.
Effective post-delimitation	Reservation to begin after the census and delimitation, valid for 15 years.	Long-term impact on politics; uncertainty in timeline.

6. Challenges in Implementation

Challenge	Description	Example
Weak Enforcement	Delayed investigations and poor victim support hinder justice delivery, causing low reporting and prolonged legal outcomes.	**NCRB (2024):** Reported that the pendency rate for crimes against women remains over **90% in courts**.

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Societal Norms	Deep-rooted patriarchal beliefs often normalise violence and discourage women from seeking legal recourse, perpetuating gender inequalities.	**NFHS-5:** Found that **77% of women** who experienced physical or sexual violence never sought help or told anyone, **citing "family honour"** as the primary reason. The percentage of ever-married women (age 18-49) who have ever experienced physical and/or sexual violence from their spouse has decreased from **29.2% in NFHS-5** to **22.3% in NFHS-6**
Lack of Awareness	Many women, particularly in rural areas, remain unaware of their legal rights, limiting their ability to access protections and benefits.	**NFHS-5 (2019-21)** noted low awareness of the MTP Act in rural Bihar, restricting reproductive choices.
Administrative Bottlenecks	Insensitive handling by police and bureaucratic inefficiencies create barriers, often deterring women from pursuing legal remedies.	**Human Rights Watch (2018)** documented victim-blaming in domestic violence cases in Uttar Pradesh.

7. Way Forward

Strategy	Action	Committee Recommendations
Strengthen Implementation	Train police and administrative officials for gender-sensitive handling; ensure timely charge-sheeting and case monitoring.	**Justice Verma Committee (2013)** recommended setting up fast-track courts and specialised police units for the swift handling of sexual offence cases.
Awareness Campaigns	Leverage media and community programs to educate women about their legal rights.	**NCW (2021):** Suggested nationwide campaigns to promote POSH Act awareness.
Economic Empowerment	Promote education and skill development to foster financial independence among women.	**Parliamentary Committee on Women (2020):** Advocated vocational training for rural women.
Shift Social Norms	Launch public campaigns to challenge patriarchal attitudes and promote gender equality.	**UN Women India (2022):** Recommended media-driven gender sensitisation programs.
Infrastructure Development	Create safe spaces and women-friendly police stations to support survivors.	**Ministry of Women and Child Development (2023):** Proposed more One Stop Centres for victims.

Despite progressive laws, implementation gaps and societal biases hinder their full impact. Bridging these gaps is crucial for real empowerment. As **B.R. Ambedkar** rightly said, **"I measure the progress of a community by the degree of progress which women have achieved."**

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2 Children

In India, a child is defined as an individual below 18 years of age, as per the National Policy for Children (2013) and the Protection of Children from Sexual Offences (POCSO) Act, 2012. Recognised as a vulnerable group, children require focused attention, protection, and opportunities to ensure their well-being and holistic development within the framework of social justice.

UPSC Mains PYQs

1. The National Commission for Protection of Child Rights has to address the challenges faced by children in the digital era. Examine the existing policies and suggest measures the Commission can initiate to tackle the issue. **(2025)**
2. The Right of Children to Free and Compulsory Education Act, 2009, remains inadequate in promoting an incentive-based system for children's education without generating awareness about the importance of schooling. Analyse. **(2022)**
3. Examine the main provisions of the National Child Policy and throw light on the status of its implementation. **(2016)**

1. Constitutional Provisions for Children

Aspect	Description	Examples
Right to Education	Article 21A ensures free and compulsory education for children aged 6-14, **a fundamental right under the Right to Education Act, 2009.**	Overall school enrollment in rural India for the 6-14 age group exceeds 98%, remaining consistently high for nearly two decades as per ASER 2024.
Protection from Hazardous Work	Article **24 prohibits child labour** below 14 in hazardous sectors like factories and mines, ensuring child safety.	Child and Adolescent Labour Rules, 1988 mandate DM as District Nodal Officer and Task Force to execute monitoring, rescue, and enforcement actions at the district level.
Freedom from Exploitation	Article 23 bans forced labour and trafficking, safeguarding children from exploitation.	Since inception, **TrackChild Portal** has traced over 4.46 lakh missing children, with 3.97 lakh reunited with families under **Mission Vatsalya.**
Directive Principles	Article **39(e) & (f)** direct the state to ensure children's health, strength, and freedom from abuse, promoting holistic development.	**M.C. Mehta v. Union of India (1991): (Prohibited** children from working in match/firework industries).
Equality and Non- Discrimination	Article **15(3):** Empowers the State to make **"Special Provisions"** for women and children (Protective Discrimination).	**POCSO Act** and **Juvenile Justice Act** to provide a child-centric justice system.

2. Child Labour

Present Situation

- Over **10.1 million children** (5-14 years) engaged in child labour (Census 2011), down from 12.6 million (2001).
- **Uttar Pradesh and Bihar** lead in child labour (NCRB 2023).
- In the past five years (2020-2025), **616 children** engaged in child labour were rescued in Gujarat.

Causes

- **Intergenerational Poverty Trap:** As per ***Amartya Sen's*** capability deprivation theory, families prioritise survival over schooling.

Impacts

- **Physical and Cognitive Harm:** Exposure to hazardous materials and long working hours lead to stunting and developmental delays (**WHO**).

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- **Educational Inequity:** Poor infrastructure and inaccessible schools push children into work. 60% of child labour linked to poverty (**ILO 2023**).
- **Weak Enforcement & Governance Gaps:** Low conviction rates and lack of labour inspections.
- **Socio-cultural Acceptance:** Normalization of child work in informal sectors, especially in rural, caste-stratified economies.
- **Seasonal & Distress Migration:** Children accompany migrating parents and are exploited in brick kilns, farms, and factories.
- **Informal Sector Pull:** Demand for cheap, compliant labour in unregulated sectors.
- **Digital & Urban Divide:** Lack of access to digital education post-COVID forced many back into work.
- **Educational Deprivation:** High dropout rates, weak learning outcomes; violates Article 21A. **Example:** The Annual Average Dropout Rate for the State of Gujarat at the secondary level was recorded at 21% as per the Samagra Shiksha Abhiyan study.
- **Perpetuation of Intergenerational Poverty:** Lack of skills = no upward mobility (echoed by **Gurupadswamy Committee**).
- **Social Marginalisation:** Reinforces exclusion of SC/ST/ minority children; affects dignity and self-worth.
- **Psychological Trauma:** Early exposure to exploitation can cause anxiety, depression, and identity loss (***UN CRC, 1989***).
- **National Economic Loss:** Undermines demographic dividend and lowers productivity.

Steps Taken

A. Schemes:

- **National Child Labour Project (NCLP):** Rehabilitates children through education and vocational training.
  - ♦ **Example:** As of 2023, ***14.3 lakh children were rescued, rehabilitated***, and mainstreamed since inception.
- **PENCIL Portal:** To ensure effective enforcement of the Child Labour Act.

B. Legislative Measures:

- **Child Labour Act, 1986 (Amended 2016):** Bans child labour under 14 and in hazardous work.
- **Juvenile Justice Act, 2015:** Protects rescued children.

C. Institutional Measures:

- **NCPCR:** Monitors child rights violations.
- **District Task Forces:** Conduct rescue operations.

Way Forward

- **The 52nd Report of the Parliamentary Standing Committee on Labour (December 2023)** → recommended establishing a uniform definition of a "child" across all labour laws to eliminate child labour.
  - ♦ recommends stricter FIR registration.
  - ♦ Creating a national database
- **Social Protection:** Cash transfers to curb poverty.
- **Gurupadswamy Committee (1979):** It recommended rehabilitation and education in sectors like carpet weaving. → It also recommended banning child labour in hazardous sectors and regulating it in non-hazardous work due to prevailing poverty.
- **NGO Collaboration:** Partner with Bachpan Bachao Andolan.

3. Child Marriage

Present Situation

- **20.1**% of women aged 20-24 married before 18 (NFHS-6, 2023-24), down from 23.3% (NFHS-5, 2019-21).
- Bihar, Jharkhand and Assam (as per NFHS-6) lead in child marriage.

Causes

- **Feminisation of Poverty:** Girls viewed as economic burdens; early marriage reduces dowry costs.
  - ♦ **Example:** Pandemics/economic shocks worsen household poverty, forcing families into cost-cutting child marriages, driving PCMA (2006) cases up nearly threefold, from 395 (2017) to 1,050 (2021).
- **Gender Norms & Patriarchal Structures:** Roles socially constructed around motherhood (***Sylvia Chant's*** theory).
- **Education Deprivation:** Girls with no education are 6 times more likely to marry early (**UNICEF**).
- **Customary Laws & Social Sanctions:** Practices like *gauna* and *bal vivah* supersede legal norms.
  - ♦ **Example:** India is ***home to the largest absolute number of child brides*** globally.
- **Weak Legal Deterrence:** Low conviction under PCMA (2006); inadequate **CMPOs**.
- **Disaster-linked Vulnerability:** Natural disasters and crises like COVID-19 increase child marriages.
- **Lack of Safe Spaces:** Fear of sexual violence often forces early marriage.

Impacts

- **Reproductive & Maternal Health Risks:** Higher risk of obstetric fistula, maternal mortality, and low birth weight.
- **Educational & Skill Deficit:** Married girls often drop out, limiting skill development (***Amartya Sen's*** "capability deprivation").
- **Reinforcement of Gender Subordination:** Entrenches dependency and male dominance.
- **Cycle of Intergenerational Poverty:** Early marriage perpetuates poverty for future generations.
- **Mental Health Impact:** Risk of depression and isolation due to marital abuse (***UN CRC, 1989***).
- **Increased Risk of Gender-Based Violence:** Domestic violence, marital rape, and control over mobility.
  - ♦ **Example:** The percentage of ever-married women (age 18-49) who have ever experienced physical and/or sexual violence from their spouse is 22.3% in NFHS-6.
- **Violation of Rights:** Breach of Article 21 and UN Convention on the Rights of the Child.

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Steps Taken

A. Schemes:
- **Beti Bachao Beti Padhao:** Promotes girl child education.
- **Kanyashree Prakalpa (West Bengal):** Cash transfers to delay marriage (7.2 million beneficiaries by 2023).

B. Legislative Measures:
- **Prohibition of Child Marriage Act, 2006:** Sets minimum marriage age at 18, and anyone who performs, conducts, or directs a child marriage can be imprisoned for up to 2 years and fined up to ₹1 lakh.
- **The Prohibition of Child Marriage (Amendment) Bill, 2021**, proposed raising girls' marriage age to 21. (The bill is not active now).

C. Institutional Measures:
- **NCPCR:** Monitors child marriage violations.
- **Child Marriage Prohibition Officers:** Prevent illegal marriages.

Way Forward & Recommendations

- **Legal Reform & Uniformity:** Implement the ***Jaya Jaitley Committee*** recommendation to raise marriage age to 21.
- **Education as a Social Vaccine:** Promote universal secondary education (SDG-4 and Art. 21A).
- **Community Engagement:** Initiate social norm transformation via ASHA, Anganwadi, and peer groups.
- **Conditional Cash Transfers (CCTs):** Replicate schemes like ***Kanyashree*** (West Bengal) and ***Ladli Laxmi*** (Madhya Pradesh).
- Ensure proper appointment, training, and monitoring of Child Marriage Prohibition Officers.
- **Data & Early Warning Systems:** Create district-wise risk indices using dropout data and marriage registrations.
- **Gender-Sensitive Curriculum:** Include modules on reproductive health and consent in schools.

4. Child Malnutrition

Present Situation

- 32.3% stunted, 16.3% wasted, 29.2% underweight (NFHS-6). Bihar & Jharkhand are the worst affected.
- 35% (declining from the previous 40%) stunting in Jharkhand (NFHS-6).

Causes

- **Multidimensional Poverty:** Deprivation in income, nutrition, housing, and sanitation (Global MPI) → *State of Food Security 2025*—around 42.9% of India's population could not afford a healthy diet.
- **WASH Deficit:** Poor Water, Sanitation and Hygiene leads to diarrhoeal diseases, impairing nutrient absorption.
- **Maternal Undernutrition & Adolescent Pregnancy:** Low maternal BMI contributes to low birth weight (*The Lancet*).
- **Food Insecurity & Knowledge Gap:** Lack of dietary diversity and nutrition illiteracy leads to malnutrition (**UNICEF, 2022**).
- **Weak Governance:** Leakages in **ICDS** and **PDS** lower programme impact.
- **Climate Vulnerability:** Droughts and floods reduce food security (IPCC, 2023).

Impacts

- **Cognitive Deficits:** Undernourished children face stunted brain development, limiting performance → ***Sánchez et al.'s*** study found stunting at age 5 negatively affects later executive function.
- **Immunological Compromise:** Weakened immunity leads to higher morbidity (pneumonia, TB).
- **Economic Implications:** Malnutrition may cost India **2-3% of its GDP** annually (World Bank/UNICEF).
- **Perpetuation of Intergenerational Inequality:** Poor nutrition in childhood creates a biological trap of poverty.
- **Social Marginalisation:** Tribal and Dalit children face long-term exclusion → Malnourished children generally score significantly lower in assessments (**UNICEF**).

Steps Taken

A. Schemes:
- **POSHAN Abhiyaan:** Targets malnutrition reduction. **Poshan Tracker** has 9 crore beneficiaries.
- **Mid-Day Meal Scheme:** Provides nutritious meals in schools.

B. Legislative Measures:
- **National Food Security Act, 2013:** Ensures food rights.
- **Maternity Benefit Act, 2017:** Supports maternal nutrition.
- **PMMVY:** Provides direct cash transfers for nutrition support.

C. Institutional Measures:
- **ICDS:** Provides supplementary nutrition.
- **Anganwadi Centres:** Monitor child health.

Way Forward & Recommendations

- **Life-Cycle Approach:** Focus from pregnancy to adolescence (**POSHAN 2.0**).
- **Community-Based Monitoring:** Strengthen *Jan Andolan* using SHGs and PRI members.
- **Fortification of Staples:** Expand iron, folic acid, and iodine fortification in rice and oil.
- **Integrated WASH-Nutrition Strategy:** Link *Swachh Bharat* and *Jal Jeevan Mission* to POSHAN.
- **Adolescent Nutrition Focus:** Revive Weekly Iron Folic Acid Supplementation (**WIFS**).
- **Data-Driven Targeting:** Use Real-Time Monitoring Systems (RTNMS) in Anganwadi Centres.
- **Nutrition-Smart Agriculture:** Promote kitchen gardens and millets (**Shree Anna**).
- **Budgetary Allocation:** Ensure performance-linked disbursal under Saksham Anganwadi.
- **SDG Synergy:** Align with **SDG 2** (Zero Hunger), **SDG 3**, and **SDG 6**.

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5. Child Abuse

Present Situation

- Nationwide, a total of 1,87,702 cases of crimes against children were registered in 2024→ National crime rate against children stands at 42.3 cases per lakh child population (NCRB 2024).
- In its latest "Crime in India 2024" report, the National Crime Records Bureau (NCRB) recorded over 69,000 cases of crimes against children under POCSO Act, with more than 44,126 specific cases registered under Sections 4 and 6 of the POCSO Act.

Causes

- **Socioeconomic Vulnerability:** Poverty, homelessness, and child labour increase susceptibility to exploitation (*UNICEF 2023*).
- **Culture of Silence:** Deep-rooted social stigma prevents disclosure, particularly in intra-familial abuse cases.
- **Legal Systemic Gaps:** Low conviction rates, delay in forensic examination, and **vacancies** in Special POCSO courts.
- **Awareness Deficit:** Absence of structured Child Protection Education in schools (as reported by *the Kailash Satyarthi Foundation*).
- **Dysfunctional Family Environment:** Exposure to domestic violence, parental substance abuse, or neglect.
- **Digital Vulnerability:** Rise of online grooming, cyberbullying, and child sexual abuse material (CSAM).
  - **Example:** *National Cyber Crime Reporting Portal (NCRP)*, under the Cyber Crime Prevention against The Women and Children (CCPWC) scheme recorded *1.94 lakh child pornography incidents* as of April 2024.

Impacts

- **Psychological Trauma:** Long-term Post-Traumatic Stress Disorder (**PTSD**), anxiety, and depression (NIMHANS 2023).
  - **Example:**Clinical evaluations by SAMVAD (NIMHANS) reveal that childhood trauma acts as a "compounding psychological debt", manifesting as treatment-resistant depression or higher self harm vulnerability in late adolescence.
- **Attachment Disorders:** Difficulty in forming trusting relationships due to betrayal trauma.
- **Educational Disruption:** Low attendance, school dropouts, and learning disabilities.
- **Perpetuation of Violence:** Risk of intergenerational transmission — victims may become future abusers (*Harvard School of Public Health*).
- **Social Alienation:** Loss of self-esteem, stigma, and withdrawal from peer groups.
- **Public Health Burden:** Abuse survivors are at higher risk of substance abuse and chronic diseases.

Steps Taken

A. Schemes:
- **CHILDLINE 1098:** 24/7 helpline for child protection, *now integrated with the Emergency Response Support System-112.*
- **Mission Vatsalya:** An integrated Digital platform for all the stakeholders related to child protection services.
- **Example**: Mission Vatsalya's integrated ERSS-112 architecture slashes response times, handling lakhs of child distress calls annually.

B. Legislative Measures:
- **POCSO Act, 2012:** Addresses child sexual abuse.
- **Juvenile Justice Act, 2015:** Ensures child-friendly justice.

C. Institutional Measures:
- **NCPCR:** Investigates **child abuse cases**.
- **Child Welfare Committees:** Provide rehabilitation.

Way Forward & Committee Recommendations

- **Fast-Track Courts:** Justice Verma Committee (2013) recommends speedy POCSO trials. The Verma Committee urged child-centric justice systems.
- **Awareness Campaigns:** Promote child safety education. (such as *UNICEF's #EndViolence*).
- **Counselling Services:** Expand mental health support.
- **Community Vigilance:** Strengthen local monitoring.

6. Other Issues

Issue	Description
1. Child Trafficking	• According to NCRB 2022, a total of 6,036 human trafficking victims were identified, of which 2,878 were children, including 1,059 girls. **Article 23** aims to address it.
2. Street Children	• 1.8 million **street children** (MWCD 2023) face exploitation and malnutrition.
3. Child Beggary	• Prevalent in urban areas, linked to poverty and trafficking. **The Juvenile Justice Act** and NCPCR interventions aim to curb it. • **Example:** Indore administration launched an anti-begging drive, offering a ₹1,000 reward for reporting forced child begging cases.
4. Children in Conflict with Law	• **Juvenile delinquency** rising (NCRB 2023). **The Juvenile Justice Act, 2015**, emphasises **rehabilitation** over punishment.

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7. National Commission for Protection of Child Rights (NCPCR)

NCPCR is a statutory body established under the **Commission for Protection of Child Rights (CPCR) Act, 2005.** It works under the administrative control of the Ministry of Women and Child Development. It consists of a Chairperson (person of eminence in child welfare) and six members (at least two must be women).

Roles and Responsibilities

- **Monitoring Mandate:** Monitors the implementation of the **RTE Act, 2009; POCSO Act, 2012;** and **Juvenile Justice Act, 2015**.
- **Inquiry:** Investigates complaints and *suo motu* takes notice of matters relating to deprivation of child rights.
- **Review:** Reviews existing laws and policies to ensure they align with the UN Convention on the Rights of the Child (UNCRC).
- **Awareness:** Promotes child rights literacy and safeguards through creative mass media campaigns and seminars.

Successes and Key Initiatives

- **Digital Monitoring:** Developed portals like 'GHAR' (Go Home and Reunite) for restoration and 'Baal Swaraj' for online tracking of children in need of care.
- **POCSO Tracking**: Launched a module for real-time tracking of POCSO cases to ensure accountability and timely rehabilitation.
- **Policy Guidelines:** Issued comprehensive guidelines for child participation in the entertainment industry (including OTT and social media content).

Limitations and Challenges

- **Non-Binding Nature:** Its recommendations are advisory; it lacks the power to enforce its decisions directly on the government.
- **Resource Constraints:** Often faces shortages in specialised manpower and funding for large-scale field investigations.
- **Implementation Gap:** Dependency on state-level machinery (SCPCRs), which are often under-resourced or non-functional in some states.

NCPCR and the Digital Era: New Challenges in the Digital Era

- **Cyber-Grooming & Predators:** Anonymity online allows predators to manipulate children for sexual exploitation.
- **Cyberbullying:** Trolling and harassment on social media lead to severe psychological trauma and mental health disorders.
- **Commercial Exploitation:** Parents/guardians using children for social media content creation/monetisation without safe working conditions.
- **Digital Divide & Literacy:** Low digital literacy among parents in rural areas leads to "shared device risks", exposing children to adult-oriented content.
- **Data Privacy:** Excessive data harvesting by EdTech and gaming platforms without informed child consent.

Existing Policies

- **POCSO Act, 2012:** Robust for physical abuse but faces procedural hurdles in defining and prosecuting "Digital Grooming."
- **IT Act, 2000:** Addresses cybercrimes but is not child-centric in its punitive measures.
- **IT Rules, 2021:** Mandates intermediaries (social media) to remove harmful content, but enforcement of "age verification" remains weak.
- **Digital Personal Data Protection Act, 2023:** Requires parental consent for processing children's data but lacks clear "screen time" or "privacy-by-design" mandates.

Measures the NCPCR Can Initiate

- NCPCR should push to include **"Safety - by -Design"** (default privacy, no dark patterns, automated screen-time nudges, etc.).
- **Age-Assurance Standards:** Collaborate with MeitY to develop age-verification standards for EdTech and social media platforms.
- Launch a **"Digital Parenting"** mission to educate parents on using parental controls and recognising signs of online abuse.
- **Single-Window Reporting:** Create a child-friendly **'Report-Remove'** interface that directly links to the NCPCR portal.
- **Digital Work Guidelines:** Formulate strict labour regulations for "Kid-influencers" on platforms like YouTube and Instagram to ensure their earnings and education are protected.

8. Landmark Supreme Court Judgments for Child Justice

Judgment	Domain	Ratio / Contribution
Unni Krishnan, J.P. v. State of Andhra Pradesh (1993)	Right to Education	Recognised **education up to 14 years** as a Fundamental Right under Article 21, paving the way for **Article 21A**.
Sheela Barse v. Union of India (1986)	Juvenile Justice	Directed separate treatment of juveniles, **speedy inquiry, legal aid**, and protection from custodial abuse.

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Bandhua Mukti Morcha v. Union of India (1984)	Bonded Child Labour	Recognised rehabilitation of bonded labourers, including children, as part of **Article 21**.
M.C. Mehta v. State of Tamil Nadu (1996)	Child Labour	Directed withdrawal of children from **hazardous industries,** compensation, and **compulsory education**.
Sampurna Behura v. Union of India (2018)	Juvenile Justice	Directed all States to fully implement the **Juvenile Justice Act, 2015**, including Child Welfare Committees and Juvenile Justice Boards.
Independent Thought v. Union of India (2017)	Child Marriage & Sexual Abuse	Read down Exception 2 to Section 375 IPC, holding **sexual intercourse with a wife below 18 years amounts to rape**.
Bachpan Bachao Andolan v. Union of India (2011 & 2014)	Missing & Trafficked Children	Directed mandatory FIRs for missing children and strengthened anti-trafficking and rehabilitation mechanisms.
Gaurav Jain v. Union of India (1997)	Children of Sex Workers	Directed rehabilitation, education, and mainstreaming of children of sex workers to prevent intergenerational exploitation.

"Children are agents of change." As Nelson Mandela reminds us, investing in children today is the surest path to a just, equitable and progressive tomorrow.

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3 Elderly and Senior Citizens

India is undergoing a demographic shift, with its elderly (60+ years) population projected to reach 20% by 2050 (UNFPA, 2023). The 2011 census recorded over 10 crore elderly, a number rising rapidly. This underscores the need for policies ensuring elderly care, health, and financial security, in line with Article 41 of the Constitution, which mandates state support for old age and disability.

Present Trends

Current Demographics

- 149 million elderly people (60+ years), 10.5% of India's population (2022, UNFPA India Ageing Report 2023). 104 million in 2011 (8.6%, Census 2011).
- In 2011, Tamil Nadu's elderly proportion was 10.4%. Projections show it rising to 18.2% to 20% of the state's total population by 2031.

Projected Growth

- The elderly population will reach 347 million (20.8%) by 2050; it will surpass children (0-15 years) by 2046. Decadal growth: 41%.
- Kerala projects 22.8% elderly by 2036 (UNFPA 2023).

Rural-Urban Divide

- 71% elderly population (73 million) in rural areas, 29% (31 million) urban (Census 2011). Rural elderly face greater vulnerability.
- Most of Bihar's elderly reside in rural areas.

State Variations

- Southern states (Kerala, Tamil Nadu) and Northern states (Himachal Pradesh, Punjab) have higher elderly population shares than the national average.
- Kerala's elderly share is 16.5%, vs. 8-9% in UP (UNFPA 2023).

1. Issues of the Elderly Population in India

Aspect	Details	Examples
Healthcare Challenges	• Rise in multi-morbidity and chronic non-communicable diseases (NCDs) like hypertension and diabetes. High prevalence of geriatric depression, dementia, and mental health disorders.	• 75% suffer from at least one chronic disease, with growing cases of geriatric depression and functional disabilities (LASI 2020).
Economic Vulnerability	• Lack of universal pension coverage, rising financial dependence on families, and the feminisation of poverty—especially among widows and single elderly women—heighten economic insecurity among the elderly.	• Only **20%** of the elderly have any form of pension; **47%** are fully dependent on family (**UNFPA Ageing Report 2023**). • 90% of the elderly in the unorganised sector lack pensions.
Social Isolation	• The elderly face increased emotional and mental health issues, including loneliness and neglect, due to reduced social interaction and support. • Weakening of intergenerational bonds due to urban migration and nuclear families. • "Empty Nest Syndrome" in states like Kerala	• ~18.7% of the elderly live exclusively with their spouse, and ~5.7% of older adults live by themselves (LASI 2020).
Elder Abuse	• Widespread prevalence of ageism and intra-family conflicts over property or caregiving. • Low institutional reporting and social silence around abuse.	• Approximately 5.2% of India's senior citizens experience elder abuse, with neglect (52.6%) and economic exploitation (26.5%) ranking as the most common forms.
Digital Divide	• Significant digital exclusion due to a lack of digital literacy and infrastructure in rural areas, and dependency on intermediaries for accessing welfare schemes.	• A survey by the Agewell India Foundation found that 95% of elderly women in India are digitally illiterate.
Housing and Mobility	• Poor adoption of universal design in public buildings and transport. • Lack of affordable and accessible senior housing or assisted-living facilities. Mobility impairments restrict participation in public and social life.	• Absence of ramps/low-floor buses in Tier-II cities restricts elderly mobility • Nearly 44% of India's elderly have at least one functional limitation (LASI).

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2. Feminisation of Ageing

Aspect	Details	Examples
Sex Ratio	Women outnumber men in older age groups due to higher life expectancy, leading to a skewed elderly sex ratio. This longevity gap amplifies gender-specific challenges.	India Ageing Report 2023: A rising number of elderly women, especially widows, in states like Tamil Nadu, are facing poverty, illiteracy, and social vulnerability.
Socio-Economic Challenges	Patriarchal structures limit women's access to income and property, with many elderly women financially dependent. Economic precariousness is acute among widows.	Nearly 66% of older women don't own assets and many feel financially insecure (HelpAge India 2023).
Health Disparities	Elderly women face higher rates of chronic illnesses and mental health challenges, particularly depression, exacerbated by widowhood and social isolation.	Nearly 35% of elderly women in UP report depression (NIMHANS 2023). Almost a third of the elderly population exhibits depressive symptoms, with high prevalence of undiagnosed depression. (Niti Ayog)
Social Vulnerabilities	Widowed and rural elderly women experience social exclusion, elder abuse, and neglect, with very old dependent women at heightened risk.	A significant proportion of rural elderly women live alone or with limited support.
Policy Gaps	Gender-neutral policies fail to address elderly women's unique needs, particularly in rural areas. Targeted interventions for gendered aging are urgently needed.	Overall pension coverage for the elderly are lower for rural women. 78% of the elderly population without a pension; Only 18% of seniors are covered by Health insurance

3. Issues with Existing Government Mechanisms

Aspect	Details	Examples
Limited Awareness	Limited awareness of entitlements, legal rights, and welfare schemes prevents many elderly persons from accessing social protection and grievance redressal mechanisms, particularly in rural and digitally excluded regions.	Awareness for **Elderline** (14567) under the **Atal Vayo Abhyuday Yojana** (AVYAY) and **Annapurna scheme** remains critically low.
Inadequate Coverage	Social protection schemes like National Social Assistance Programme (NSAP) offer meagre pensions, failing to address economic vulnerability.	Only 2.2 crore elderly beneficiaries under IGNOAPS.
Urban Bias	Geriatric care and old age homes are skewed toward urban areas, neglecting the rural majority. This geographic inequity marginalises rural elderly healthcare access.	Most registered old-age homes, or assisted living facilities are located in or around tier-1 and tier-2 cities.
Implementation Delays	Stalled reforms, like the Senior Citizens Amendment Bill, 2019, hinder progress on home-based care. Bureaucratic inefficiencies perpetuate policy inertia.	Bill pending since 2019, stalling reforms (MoSJE 2024).
Digital Exclusion	Rural elderly are often unable to access digital platforms like SAMPANN and SACRED due to the pervasive digital divide. E-governance barriers limit access to critical services.	Approximately 85% of India's elderly population is digitally illiterate.

4. Measures for Elderly Welfare in India

Aspect	Measures
Healthcare Access	• **National Programme for Health Care of the Elderly(NPHCE):** Geriatric care at primary, secondary & tertiary levels. • **Ayushman Bharat-PMJAY:** Extended to all persons aged 70+ regardless of income (from 2024), with ₹5 lakh cover. • **Rashtriya Vayoshri Yojana (RVY):** Free assistive devices for BPL senior citizens. • **National Institute of Ageing (NIA):** Research and specialized treatment for geriatric diseases.

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Financial Security	• **Indira Gandhi National Old Age Pension Scheme (IGNOAPS):** Provides financial assistance to senior citizens below the poverty line. • **Pradhan Mantri Vaya Vandana Yojana (PMVVY):** Offers a guaranteed pension plan for senior citizens. • **Reverse Mortgage Scheme:** Allows senior citizens to mortgage their property and receive regular income while residing in it. • **Atal Pension Yojana (APY):** Provides pension benefits to workers in the unorganised sector.
Housing & Mobility	• **Integrated Programme for Older Persons (IPOP):** Supports the establishment of old age homes, daycare centres, and other services for the elderly. • **Day Care Centres for Senior Citizens:** Community-based centres offering recreational and healthcare support. • **Senior Citizen Welfare Funds:** Utilized for the welfare of unclaimed deposits in public funds.
Protection & Legal Rights	• **Maintenance and Welfare of Parents and Senior Citizens Act (2007):** Ensures financial and emotional support from children. • **Elder Line (14567):** National helpline for senior citizens' grievances. • **Legal Aid Services:** Free legal assistance is provided to senior citizens to address cases of abuse and property disputes.

5. Silver Economy

The "silver economy" encompasses all economic activities, products, and services designed to meet the needs of the aging and senior populations (typically those aged 50+ or 60+). It leverages this demographic's growing purchasing power, accumulated wealth, and consumer demands.

Drivers of Silver Economy

- **Active Aging Model:** India is shifting towards active aging where seniors remain healthier, independent and economically productive. The 45-64 age group currently holds the highest wealth share, making older individuals a strong consumer base while also contributing through continued employment and experience based roles.
- **Health Care Services:** Advancements in healthcare have led to a global rise in life expectancy, creating a growing market for elder-specific products and services.
    - **Example:** Over 75% of elderly in India suffer from chronic diseases, creating strong demand for home-based healthcare (LASI 2020)
- **Consumer Market:** By 2030, India's Silver Economy is expected to manage nearly $1.5 trillion in assets, reflecting high consumption demand in healthcare, housing, insurance and wellness sectors driven by ageing population needs.
- **Technological Integration:** Telemedicine, wearable devices like fall detectors and GPS trackers and remote monitoring systems are transforming elderly care. The remote patient monitoring market is projected to grow at 12.7% CAGR, reaching ₹56.94 billion by 2030.
- **Rise of Senior Housing:** Senior living infrastructure is expected to grow by 300% by 2030, reflecting increasing demand for age friendly housing, assisted living communities and long term residential care facilities.

Challenges To Silver Economy

- **Healthcare Infrastructure Gaps:** India has only around 300-350 geriatricians for over 120 million elderly people , while older adults account for nearly 60% of ICU admissions, showing a severe mismatch in healthcare capacity and specialised care availability.
- **High Medical Costs:** Medical inflation reached 14% in 2024, the highest in Asia, making treatment for chronic illnesses expensive and unaffordable, especially for seniors without stable income sources or insurance coverage.
- **Low Insurance Coverage:** Only about 18% of senior citizens are covered under health insurance, leading to high unaffordable expenditure and reduced access to quality healthcare services.
- **Rural-Urban Divide:** Nearly 70% of elderly population lives in rural areas, where they travel over 28 km for healthcare compared to 10 km in urban regions, highlighting accessibility issues in medical infrastructure.
- **Financial Insecurity:** Around 78% of elderly individuals do not receive pensions, especially those from the unorganised sector, forcing dependence on family support and reducing financial independence.
- **Digital Divide:** Limited digital literacy among seniors restricts access to telehealth, online banking and e-commerce, preventing them from fully benefiting from modern service delivery systems.

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6. Way Forward

Aspect	Details
Healthcare Expansion	• Scale up geriatric care units to 500 by 2030; integrate mental health services (NITI Aayog 2024). Subsidise private healthcare costs. • **Example:** The Vayomithram project in Kerala is a successful initiative focused on providing comprehensive healthcare to senior citizens.
Universal Pension	• Implement universal pension for all the elderly, including the unorganised sector (15th Finance Commission). Address economic vulnerability. • **Example:** Tamil Nadu has one of the widest pension coverages among Indian states.
Gender-Specific Policies	• Develop gender-sensitive schemes for elderly women, focusing on widowhood, healthcare (UNFPA 2023). Target rural women. • **Example:** Rajasthan's widow pension aids more than 20 lakh women (2025).
Digital Inclusion	• Expand digital literacy programs; simplify e-governance for the elderly (NITI Aayog 2023). Bridge the digital divide. • **Example:** Delhi's digital camps trained 10,000 elderly (2024). • **Example**: Kerala's "Akshaya" centers, which use local digital kiosks and doorstep volunteers to guide senior citizens through simplified e-governance services and digital banking.
Community Engagement	• Promote senior citizen clubs, intergenerational programs to reduce social isolation (UNFPA 2023). Strengthen community vigilance. • **Example:** Karnataka's 200 senior clubs engaged 50,000 elderly (2023).
Regulatory Framework	• Regulate private old age homes; pass the Senior Citizens Amendment Bill, 2019 (NITI Aayog 2023). Establish a monitoring body. • **Example:** Maharashtra regulates 100 homes (2024).

Ageing with dignity is not a burden, but a societal responsibility. Ensuring social security, healthcare and inclusion for the elderly transforms demographic challenge into demographic dividend, building a compassionate and just India for all generations.

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4 Persons with Disability

As per the Census 2011, they comprise 2.21% of the population, highlighting the need for inclusive policies and targeted interventions. The Indian Constitution, under Article 41 (Directive Principles of State Policy), directs the State to make effective provisions for securing the right to work, education, and public assistance in cases of disability, among others.

UPSC Mains PYQs

1. The Rights of Persons with Disabilities Act, 2016 remains only a legal document without intense sensitisation of Government functionaries and citizens regarding disability. Comment. **(2022)**
2. Does the Rights of Persons with Disabilities Act, 2016 ensure an effective mechanism for empowerment and inclusion of the intended beneficiaries in the society? Discuss. **(2017)**

1. Overview

Aspect	Details
Current Demographics	2.68 crore PwDs (2.21% of population, Census 2011); NFHS-5 estimates 4.52% (60 million).
Types of Disabilities	The RPWD Act, 2016 lists 21 disabilities, including locomotor, visual, hearing, intellectual, autism, mental illness, and acid attack victims.
Rural-Urban Divide	69% PwDs in rural areas; 31% urban. Rural PwDs face greater accessibility barriers.
Gender Disparities	1.5 crore male PwDs, 1.18 crore female (Census 2011). Women face higher discrimination and gender-based violence.
Age Group	7.62% PwDs aged 0-6 years; 45% illiterate (Census 2011). Early intervention is critical.

2. Challenges Faced by Persons with Disabilities

Challenge	Details	Examples
Educational Exclusion	Structural exclusion and lack of Universal Design for Learning (UDL) impede access. Inadequate teacher training and ableist pedagogy deepen educational marginalisation.	NSS (76th Round): Only 10.1% of PwDs (3-35 years) have ever attended pre-school intervention programs.
Employment Barriers	Labour market discrimination, violation of the principle of reasonable accommodation, and informal sector exclusion affect the economic inclusion of PwDs.	Out of ~1.3 Cr employable PwDs, only 34 Lakh (~26%) are currently employed (MoSJE).
Healthcare Hurdles	Inadequate implementation of inclusive health policies violate the right to health enshrined in the UNCRPD.	Studies indicate that only about 5% to 7.5% of PHCs in India are fully disability-friendly 80% of persons with disabilities in India do not have any health insurance as per National Centre for Promotion of Employment for Disabled People.
Social Stigma	Deep-rooted ableism, symbolic violence, and patriarchal intersectionality lead to social invisibility and higher vulnerability, especially for women PwDs.	NSS (76th Round): Percentage of persons with disabilities who were living alone was 3.7%.
Accessibility Issues	Non-compliance with Universal Design norms creates an infrastructural and design accessibility issue and restricts physical mobility.	Global audits (e.g., WebAIM) show that over 98% of top websites fail basic WCAG accessibility; Indian websites face similar issues.

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Policy Implementation	The gap between rights-based legislation (RPwD Act 2016) and its ground-level enforcement is due to institutional apathy, lack of inter-departmental coordination, and data deficit.	Nearly **50% of reserved posts** for PwDs in the government sector remain vacant. **DoPT**: Representation of PwDs in the Central Government workforce has remained stagnant between 0.5% and 1.15% for over a decade despite 4% reservation.

3. Rights of Persons with Disabilities (RPwD) Act, 2016

Aspect	Details
Definition of PwD	• A person with long-term physical, mental, intellectual, or sensory impairment that limits full participation in society.
Recognised Disabilities	• 21 types, including blindness, hearing impairment, intellectual disability, mental illness, acid attack victims, and more.
Rights of PwDs	• Right to equality, dignity, protection from abuse, right to family, reproductive rights, voting accessibility, and property ownership.
Persons with Benchmark Disabilities	• Individuals with at least 40% disability certified by authorities are eligible for special benefits and reservations.
Guardianship	• Limited guardian appointed if PwD cannot make decisions; based on mutual trust and respect for PwD's will; total support granted if needed.
Social Security	• The government must create programs to ensure adequate living standards and support independent or community living for PwDs.
Institutional Mechanisms	• Establishes Central and State Advisory Boards for policy guidance; • Strengthens the Chief and State Commissioners for monitoring and grievance redressal; • Creates National/State Funds and district committees; • Designates Special Courts for violations.
Penalties for Offences	• Violations attract fines ranging from ₹10,000 to ₹5 lakh and imprisonment from 6 months to 2 years; intentional insult, intimidation, or exploitation of PwDs carries imprisonment from 6 months up to 5 years (Section 89 and Section 92).
Implementation Challenges	• Inadequate resource allocation with stagnant budgets; • Poor coordination between the Centre and the states; • Lack of updated data for evidence-based policies; • Limited rural accessibility and rehabilitation services.

4. Rights of Persons with Disabilities (Amendment) Rules, 2024

Feature	Details
Notification and Authority'	• Notified by the Union Ministry of Social Justice and Empowerment under the RPwD Act, 2016. • Disability certificates issued by medical authorities at the district level, based on the applicant's residence.
Disability Certificate Application	• Requires proof of identity, a recent photograph (not older than 6 months), and an Aadhaar card. • Submitted via UDID portal.
Processing and Lapse	• Processing time extended from 1 month to 3 months for specialized cases. • Applications lapse if undecided for over 2 years; applicants must reapply or reactivate.
Colour-Coded UDID Cards	• White: Disability below 40%. • Yellow: Disability between 40% and 80%. • Blue: Disability of 80% or above.
Concerns with Amendments	• Aadhaar requirement may exclude rural or marginalised communities. • An extended 3-month processing time could delay access to services and entitlements.

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6. Measures to Improve Quality of Life

Aspect	Details	Examples
Inclusive Education	Implement NEP 2020 for inclusive schools; train teachers, provide assistive devices.	The National Institute for Empowerment of Persons with Intellectual Disabilities (NIEPID) trains teachers annually.
Employment Opportunities	Enforce 4% reservation in government jobs; promote private sector participation support self-employment and entrepreneurship.	PM-DAKSH empowers PwDs through skill development and training.
Healthcare Access	Expand rehabilitation centres, mental health services; integrate PwDs in Ayushman Bharat.	The Niramaya Health Insurance Scheme is designed to meet the diverse medical needs of persons with disabilities (PwDs).
Accessibility Standards	Under the Accessible India Campaign, the government aims to make major public buildings and infrastructure barrier-free; CPWD and other agencies are working towards retrofitting public buildings.	Following CCPD orders (2022-present), several tech companies including Paytm were directed to improve digital accessibility; many have made app redesigns.
Social Inclusion	State-level awareness programmes and school-based campaigns have been launched to reduce stigma; states like Kerala have undertaken disability-sensitisation initiatives.	Kerala's Vidya Jyothi program reduced stigma through school campaigns (2020).
Structured Negotiation	Alternative dispute resolution for accessibility issues; involve PwDs in solutions.	The Supreme Court has used structured negotiation and monitoring in PILs like Rajive Raturi v. Union of India to ensure accessibility in public places for PwDs.

7. Way Forward

Aspect	Details
Policy Enforcement	• Strengthen RPWD Act implementation (5% reservation for disabled students, partially implemented). • Strengthen the powers and independence of the Chief Commissioner and State Commissioners; consider establishing a robust National Commission for PwDs with stronger oversight powers.
Universal Accessibility	• Work towards 100% barrier-free public infrastructure by 2030, in line with SDG 11's goal of inclusive and accessible cities. • Enforce universal design in ICT.
Skill Development	• Scale up PM-DAKSH and other skill development programmes; aim to train a large number of PwDs (e.g., aiming for 1 million by 2030) as part of national skill goals.
Awareness Campaigns	• Nationwide sensitisation to reduce stigma and eliminate 'Ableism' by involving NGOs & Media. • Integrating disability sensitization in school curricula (NEP 2020) and Civil Services training.
Research & Development	• Invest in prosthetics, assistive technologies; improve disability data (MoSJE 2024). • Incentivize startups to develop affordable Assistive Technologies(Prosthetics, Screen Readers) under the **Make in India** initiative.
Community-Based Rehabilitation	• Expand CBR programs; integrate PwDs in rural communities. • Utilize the **ASHA/Anganwadi** network for early identification and home-based support in rural areas.

Creating an inclusive society for Persons with Disabilities requires equal opportunities, accessibility, and awareness. As **A.P.J. Abdul Kalam** remarked, "When I see children run around and cycle with the artificial limbs we designed, it is sheer bliss." Let us innovate and collaborate to empower PwDs, ensuring their dignity and contribution to nation-building.

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5 Transgenders

Transgender persons in India face systemic exclusion, social stigma, and lack of legal recognition for decades. The Transgender Persons (Protection of Rights) Act, 2019, and the Supreme Court's NALSA judgment (2014) have been pivotal in recognizing their rights. However, implementation gaps in healthcare, education, and employment persist, hindering their full integration into society.

1. Overview

Aspect	Details
Current Demographics	Approx 4.88 lakh transgender persons; around 0.04% of the total population. (Census 2011)
Literacy Rate	56% literacy rate among transgender persons (Census 2011), significantly lower than the national average(74%)
Employment Rate	Only 6% of transgender persons are employed in the formal sector (NHRC 2023). Most engage in informal work.
Social Exclusion	92% of transgender persons face social exclusion, including being denied housing and public services (NHRC Survey 2023).
Rights Recognition	NALSA vs. Union of India (2014) recognized transgender persons as the "Third Gender" and upheld their right to self-identify.

2. Constitutional and Legal Framework

Aspect	Details
Constitutional Provisions	• Article 14: Equality before the law. • Article 15: Prohibits discrimination. • Article 19: Freedom of expression. • Article 21: Right to dignity, life.
Transgender Persons Act, 2019	• Defines transgender; prohibits discrimination in education, employment, and healthcare; allows self-identification.
Landmark Judgments	• NALSA v. Union of India (2014): Recognised third gender, mandated reservations. • Navtej Singh Johar v. Union of India (2018): Decriminalized Section 377 IPC. • Ms. X v. State of Karnataka (2024): Allowed name, gender change on birth certificates.
Other Laws	• Mental Healthcare Act, 2017: Ensures mental health access. • Registration of Births and Deaths Act, 1969: Permits gender updates post-2024 ruling.
International Commitments	• UNCRPD (2007): Promotes non-discrimination. SDG 10: Reduced inequalities.

3. Challenges Faced by Transgender Persons

Aspect	Details
Social Stigma and Mental Health Issues	• Trans persons face structural violence rooted in normative and patriarchal norms, which label them as "abnormal" or "deviant". • Experiences of identity erasure and exclusion from social institutions are common. • Social death due to family rejection leads to homelessness and mental distress. • ***Example:*** Only 2% of transgender individuals live with their families; 92% of transgender persons face social exclusion (NHRC Survey 2023); Around 45% transgender youth suffer depression due to rejection and trauma.
Educational Barriers	• Education systems reflect institutional exclusion due to bullying, gendered infrastructure, and the absence of trans-affirmative pedagogy. • High dropout rates hinder access to higher education and job opportunities. • **Example:** Literacy rate is 56% vs. national average of 74%.
Employment Discrimination	• Labour market marginalisation persists due to bias and the absence of inclusive HR policies. • The NALSA judgment (2014) directed reservations for transgender persons, but enforcement has been negligible. • **Example:** Shanavi Ponnusamy, a transgender woman from Tamil Nadu, took on Air India in the SC for denying her a cabin crew position, allegedly based on her gender.

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Healthcare Access	• Trans persons face cis-normative medical practices, making gender-affirming care inaccessible. • Mental health services are inadequate, and conversion therapy is still practised. • **Example:** In S. Sushma v. Commissioner of Police case(2021) , the Madras High Court banned conversion therapy, calling it unconstitutional.
Prison Treatment	• Binary prison policies result in misgendering, isolation, and abuse. • Lack of gender-affirming accommodations leads to custodial trauma and violation of Article 21 rights. • **Example:** CHRI report (2024) documented torture/solitary confinement in UP jails.
Financial Exclusion	• Limited credit and banking services access due to documentation hurdles. NHRC advisory highlights that the lack of standardized, non-binary loan documentation frameworks at public and private financial institutions systematically excludes transgenders. • **Example**: Only 15% of transgender persons had access to formal bank loans.
Political Under representation	• Limited electoral participation and weak policy implementation hinder political empowerment. • Only three transgender candidates contested the 2024 Lok Sabha elections.

4. Measures for Transgender Persons

Category	Details
Schemes	• **Garima Gruh:** Provide shelter to Transgender persons with basic amenities like shelter, food, medical care and recreational facilities. • **SMILE (Support for Marginalised Individuals for Livelihood and Enterprise) Scheme:** Supports livelihood, education, and includes transgender welfare. • **National Portal for Transgender Persons:** A Digital platform for identity certificates. • **PM-DAKSH (Pradhan Mantri Dakshta Aur Kushalta Sampann Hitgrahi):** Skill training for transgenders in the private sector. • **Ayushman Bharat TG Plus:** Covers gender-affirming surgeries and transition-related healthcare for transgender individuals.
Legislative Measures	• **Transgender Persons (Protection of Rights) Act, 2019:** Prohibits discrimination, ensures self-identification. • **Mental Healthcare Act, 2017:** Protects mental health rights. • **Registration of Births and Deaths Act, 1969:** Updated post-2024 for gender changes.
Institutional Measures	• **National Council for Transgender Persons:** Formulates policies, monitors inclusion. • **Chief Commissioner for PwDs:** The CCPD extends its mandate to address and resolve grievances concerning the transgender community. • **Election Reforms:** "Others" option in voter forms. • **Banking:** No restrictions preventing LGBTQ+ individuals from opening joint bank accounts or naming their partners as beneficiaries/nominees.

5. Transgender Persons (Protection of Rights) Act, 2019(As Amended in 2026)

The Transgender Persons Amendment Act, 2026 marks a major shift from the 2019 framework of "self-perceived identity" to a rigid, bio-medical model. By requiring state-run Medical Boards to verify gender identity, the amendment streamlines administrative tracking and aims to prevent its misuse, stiffening penalties for forced mutilation. However, critics argue it strips away personal autonomy and directly violates the Supreme Court's landmark NALSA (2014) ruling on self-determination.

Feature	2026 Amended Provision	Concerns
Definition *(Sec 2k)*	Restricted to biological intersex variations, socio-cultural groups (kinnars/hijras), or forced/ mutilated individuals.	**Biological Reductionism:** Conflates psychological gender identity with biological intersex traits; excludes conventional trans-men/women.
Self-Determination *(Sec 4)*	The right to a "self-perceived gender identity" was formally deleted from Indian law with the passage of this Act.	**Unconstitutional:** Violates personal autonomy and directly defies the SC *NALSA (2014)* judgment.
Identity Authority *(Sec 6)*	Certificates issued by District Magistrates **only after approval from a Medical Board** (headed by CMO/Dy CMO).	**Medical Gatekeeping:** Pathologizes identity, strips away individual agency, and invites administrative harassment.
Retroactivity Clause	Declares self-perceived identity **"shall never have been so included"** under the law since inception.	**Legal Disruption:** Invalidates existing certificates and welfare access obtained via self-perception since 2019.

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Penalties & Offences *(Sec 18)*	Graded system: 6 months to 2 years for discrimination/forced labor; **10 years to life** for forced castration, child mutilation, or kidnapping.	**Disproportionate Focus:** Prioritizes a narrative of "forced conversion to identity" over systemic, everyday hate crimes.
Documentation & Reporting	Direct path to change first name across documents, but **medical centers must report** gender surgeries to the DM.	**State Surveillance:** Infringes upon right to privacy regarding personal medical and transition choices.
Affirmative Action	**Total omission** of reservations or quotas in education and public employment.	**Defiance of Courts:** Ignores SC mandate to treat transgender individuals as a Socially & Educationally Backward Class (SEBC).

6. Issues with Existing Mechanisms

Aspect	Details	Substantiation
Social Stigma & Social Exclusion	Fails to address deep-rooted societal prejudice, family rejection, and cultural stereotypes that lead to exclusion from education, housing, employment, and community life despite legal recognition.	**National Medical Commission (2021):** Issued guidelines directing medical colleges to remove stigmatizing and derogatory references to transgender persons from medical textbooks, acknowledging the persistence of institutional and societal stigma even after the 2019 Act; Only 6% of transgender persons are employed in the formal sector (NHRC 2023)
Persisting Gaps	Transgender Persons Act lacks reservation, marriage, and adoption provisions; begging is criminalised.	**Supriyo @ Supriya Chakraborty v. Union of India (2023)**: The Supreme Court declined to legalise same-sex marriage, observing that issues such as marriage, adoption, and civil unions require legislative action
Implementation Gaps	Weak institutional capacity, cumbersome certification procedures, and uneven State-level implementation hinder effective access to legal identity and welfare entitlements.	**National Portal for Transgender Persons (MoSJE):** Despite an estimated 4.87 lakh transgender persons (Census 2011), only around 25,000 transgender identity cards had been issued through the portal by 2024.
Awareness Deficit	Most of the transgender individuals remain completely unaware of the legal protections and welfare benefits guaranteed to them under the Transgender Persons (Protection of Rights) Act, 2019, with rural outreach remaining critically weak across India.	Around 50 percent of transgenders have never attended schools and faced discrimination (UNDP)
Underutilisation of Funds	Less than a fraction of the central budget—such as the funds allocated for the umbrella scheme SMILE—has successfully reached beneficiaries.	Only 11% of the transgender welfare budget was spent between 2021 and 2024, reflecting a massive systemic underutilization of allocated funds.

7. Best Practices for the Betterment of Transgender Lives in India

Initiative	Description	Key Features
International Level: Yogyakarta Principles (2006 & YP+10, 2017)	A set of 29 principles (2006) and 9 additional principles (2017) outlining the application of international human rights law to sexual orientation, gender identity, gender expression, and sex characteristics.	• Affirms rights to equality, non-discrimination, and protection from violence. • Referenced in India's Navtej Singh Johar case (2018). • Non-binding but guides states and courts globally.
Central Level: Third Gender Column in Forms	Introduced by the Ministry of Railways (2016) and the Ministry of Personnel (2020) to include 'transgender' as an option in forms	• Includes 'transgender' as an option in railway reservation and government job forms. • Enhances visibility and recognition of transgender identity.
State Level: Sakalyam Scheme (Kerala)	Imparts vocational training to transgender individuals to create employment opportunities.	• Targets economic upliftment of deprived transgender communities.

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8. Way Forward

- **Upholding the Right to Self-Identification:** Ensure legal frameworks fully recognize gender identity as a matter of individual choice and dignity, free from unnecessary medical or bureaucratic verification requirements.
- **Sensitizing Public Institutions:** Strengthen capacity-building and gender-sensitization programs for administrative officials, law enforcement agencies, and healthcare professionals to promote respectful and non-discriminatory engagement.
- **Expanding Inclusive Healthcare Services:** Develop accessible, affordable, and standardized gender-affirming healthcare services within the public health system, including specialized support for physical and mental well-being.
- **Strengthening Social Protection Measures:** Integrate transgender-specific healthcare and welfare needs into existing social security schemes while ensuring dedicated support mechanisms and counseling services.
- **Enhancing Economic Empowerment:** Promote affirmative action, inclusive recruitment policies, skill development initiatives, and entrepreneurship support to improve workforce participation and financial independence.
- **Creating Sustainable Livelihood Opportunities:** Encourage greater representation in public and private sector employment through targeted reservations, workplace diversity measures, and anti-discrimination safeguards.
- **Promoting Social Awareness and Acceptance:** Undertake sustained public awareness campaigns to challenge stereotypes, foster empathy, and build an inclusive social environment for transgender persons.

"Legal recognition without social acceptance results in formal equality but not substantive inclusion." Inclusion of the transgender community requires more than just laws—it needs social acceptance, equal opportunities, and dignity in daily life. As activist Apsara Reddy powerfully reminds us, 'We demand equality, not pity.' True progress lies not in policies alone but in changing mindsets.

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6 Homeless Persons

6.1. Overview

Aspect	Details
1. Current Demographics	1.77 million homeless persons (Census 2011).
2. Geographic Distribution	Uttar Pradesh and Maharashtra have the highest number of homeless persons (Census 2011).
3. Occupational Status	Most engage in low-wage casual labour, rag-picking, and begging. Lack of job security is chronic.
4. Gender Composition	India's homeless population is predominantly male, with men making up roughly 59% and women 41% of the total.
5. Constitutional Mandate	Right to Shelter is recognized as part of the Right to Life under Article 21 (Chameli Singh vs UP)..

Causes of Begging and Homelessness in India

- **Structural Poverty and Inequality:** Despite economic growth, deep-rooted inequalities in income, caste, land ownership, employment opportunities, education, and healthcare continue to marginalize vulnerable populations, forcing many into begging for subsistence.
- **Rural-to-Urban Migration:** Agrarian distress, declining rural livelihoods, and limited employment opportunities compel large-scale migration to cities, where inadequate job absorption and housing shortages often leave migrants destitute.
- **Mental Illness and Disability:** Lack of accessible healthcare, rehabilitation facilities, and social support systems leaves persons with mental illnesses and disabilities vulnerable to homelessness and street-based survival.
- **Erosion of Traditional Family Support Systems:** Urbanization, nuclear family trends, and changing social values have weakened traditional caregiving structures, increasing abandonment of the elderly, widows, and persons with disabilities.
- **Displacement:** Natural calamities and development-induced displacement frequently uproot vulnerable communities, resulting in loss of livelihoods, homelessness, and heightened socio-economic insecurity.

6.2. Challenges Faced by Homeless Persons

- **Lack of Adequate Shelter and Housing:** Homeless persons often lack access to safe, secure, and permanent housing, exposing them to extreme weather and unsafe living conditions.For example, Delhi has an estimated 2–2.5 lakh urban homeless people.
- **Livelihood Insecurity and Unemployment:** Most homeless individuals depend on informal, low-paying, and irregular employment, making income generation highly uncertain and perpetuating poverty.
- **Poor Access to Healthcare:** Homeless persons face significant barriers in accessing healthcare due to lack of identification documents, discrimination, transportation constraints, and inability to afford treatment.
- **Food, Water, and Sanitation Insecurity:** Access to nutritious food, safe drinking water, toilets, and hygiene facilities remains inadequate, increasing vulnerability to disease and malnutrition.
- **Social Exclusion and Stigma:** Homeless persons frequently face discrimination, social isolation, and exclusion from mainstream society, reducing access to opportunities and welfare services.
- **Lack of Identity Documents and Welfare Access:** Absence of Aadhaar, voter ID, ration cards, or proof of residence prevents access to welfare schemes, banking services, and social security benefits. For example, a 2019 study by Dalberg Policy Insights found that 30% of homeless individuals lacked any form of Aadhaar identification.
- **Exposure to Violence and Exploitation:** Homeless persons, especially women and children, are highly vulnerable to physical violence, trafficking, harassment, and exploitation.

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7 Scheduled Castes

Scheduled Castes (SCs) are historically disadvantaged communities recognized under Article 341 of the Constitution, who have faced social exclusion and untouchability, and are entitled to affirmative action for their upliftment. India's Scheduled Castes, marginalised as "untouchables," continue to face social exclusion and economic deprivation.

1. Constitutional Provisions

Aspect	Description
Abolition of Untouchability	Article 17 explicitly bans untouchability, a dehumanising practice that historically relegated SCs to menial roles.
Equality and Non-Discrimination	Article 14 guarantees equality before law, while Article 15 prohibits discrimination based on caste, countering systemic casteism.
Reservations	Articles 330, 332, 243D, and 243T reserve seats for SCs in Lok Sabha, state legislatures, panchayats, and municipalities, aiming to amplify their political voice.
Educational/ Economic Interests	Article 46 (DPSP) mandates the state to promote the education and economic upliftment of SCs, addressing their historical exclusion from resources and opportunities.
National Commission	Article 338 establishes the National Commission for Scheduled Castes (NCSC) to monitor safeguards, investigate grievances, and recommend policy reforms.
Appointments	Article 335 ensures SC representation in government jobs while maintaining administrative efficiency, addressing their underrepresentation in public services.

2. Issues Faced by Scheduled Castes

Aspect	Description	Examples
Social Discrimination	Persistent structural exclusion, rooted in Brahmanical patriarchy and graded inequality, manifests through untouchability, denial of access to public spaces, caste-based segregation, and hate crimes.	NCRB 2023: 45,961 registered crimes against SCs under the SC/ST (Prevention of Atrocities) Act.
Economic Marginalization	Continued occupational immobility, prevalence in informal labor, landlessness, and lack of asset ownership due to historical deprivation and exclusion from capital. Reflects 'economic untouchability'.	2015-16 Agricultural Census: SCs account for just 9% of India's total agricultural land, Average holding size is 0.78 hectares.
Educational Barriers	Affected by caste stigma, peer bullying, discriminatory pedagogy, and financial constraints, the absence of culturally responsive education and under-implementation of Right to Education Act provisions.	Over 2,424 students from the Scheduled Caste (SC) community have dropped out of Central Universities in 2023 according to MoE.
Political Under representation	Despite political reservations (Article 330/332), lack of substantive representation, marginalisation in decision-making bodies, and tokenism prevail due to the dominance of upper castes in parties.	SCs remain underrepresented in decision-making bodies, holding only 4% of senior positions in the government.
Healthcare Disparities	SCs face caste-based medical discrimination, under-provisioned public health infrastructure in SC-dominated areas, and intersectional neglect of caste with health policies.	Oxfam India Inequality Report, 2021: Over 50% of SC/ST individuals face extreme difficulties such as COVID-19 accessing healthcare during disruptions .
Manual Scavenging & Hazardous Labour	Reflects extreme caste-based occupational stratification and violation of Article 23. Practice persists despite prohibition under the Prohibition of Employment as Manual Scavengers Act (2013).	941 deaths have occurred since 1993 while undertaking 'hazardous cleaning'(as defined in the act) of sewers and septic tanks.

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Digital & Technological Exclusion	SCs face digital divide due to lack of access to devices, internet, and digital literacy, which deepens educational, economic, and civic exclusions.	According to the Multiple Indicator Survey (NSO), in 2020–21, a staggering 44 percent of ST adults and 33 percent of SC adults had no phone access at all.
Urban Marginalisation	Dalits in cities face invisible casteism, ghettoisation, and residential segregation in slums lacking basic amenities. Urban policies often ignore caste realities.	Mumbai's Dharavi studies show caste-clustered occupational traps.
Political Interference	NCSC and state bodies face political pressure, diluting their autonomy and ability to address SC issues effectively.	NCSC delayed numerous complaints due to funding cuts, political influence (Annual Report 2022-23).

3. Initiatives For SCs

Scheme	Description	Examples
Post-Matric Scholarship	Provides 100% financial aid for post-matriculation or post-secondary courses, reducing dropout rates and fostering social mobility among SC students.	During 2025-26, up to 30 November 2025, a total amount of ₹7,210.80 crore was released to 61.71 lakh Scheduled Caste beneficiaries under the Post-Matric Scholarship Scheme.
Pradhan Mantri Adarsh Gram Yojana	Develops SC-majority villages with roads, sanitation, and schools, aiming to bridge infrastructure gaps and improve living standards.	Transformed villages with electrification, water supply.
Stand-Up India	Offers loans to SC entrepreneurs for startups, promoting economic independence and reducing reliance on unorganised labour.	Sanctioned loans worth over Rs. 50,000+ crore, creating hundreds of thousands of jobs. Women entrepreneurs constitute over 75% of the total beneficiaries (SIDBI/DFS).
Ambedkar Social Innovation Incubation Mission	Incubates SC-led startups for social impact, providing mentorship, funding to innovate in education, health.	Incubated SC-led ed-tech startups in Maharashtra & Tamil Nadu.
Pradhan Mantri Anusuchit Jaati Abhyuday Yojana (PM AJAY)	Consolidates infrastructure development, skill training, and livelihood support in SC-dominated areas to promote holistic upliftment and sustainable development.	**Budget: Rs. 957 crore (2026-27).**

Legislative Measures

Legislation	Description	Examples
SC/ST (PoA) Act, 1989	Criminalises atrocities like violence, humiliation against SCs, with special courts for swift justice and no anticipatory bail for offenders.	SC/ST Amendment Act (2018) restored stringent provisions post-SC dilution. 57000+ cases filed (NCRB 2023).
Constitution (SC) Order (Amendment) Act, 2023	Updates SC list to include/exclude communities, ensuring accurate representation and access to reservations, schemes.	The **2023 Amendment Act** officially included the *Mahara* and *Mahra* communities as synonyms under the SC list for **Chhattisgarh**.
Bonded Labour System (Abolition) Act, 1976	Frees SCs from bonded labour, prevalent in agriculture, brick kilns, providing rehabilitation and livelihood support.	Since 1978, around 3 lakh bonded labourers have been officially released and rehabilitated.
The Code on Wages (Replacing Minimum Wages Act, 1948)	Formally codifies and unifies wage protections to the entire unorganized sector (construction, domestic work, sanitation) where an overwhelming majority of SC daily-wage workers are employed.	Mandates a statutory **National Floor Level Minimum Wage** set by the Central Government, legally ensuring that no state-level minimum wage can fall below this baseline boundary.

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Institutional Measures

Institution	Description	Examples
National Commission for SCs (NCSC)	Monitors constitutional safeguards, investigates atrocities, and advises on SC welfare policies, acting as a watchdog for social justice.	
National Commission for Safai Karamcharis	Investigates the working conditions of sanitation workers and advocates for their socio-economic upliftment and safety.	

4. Way Forward

- **Implementation of Constitutional Safeguards:** Ensure effective enforcement of constitutional provisions, anti-discrimination laws, and affirmative action policies to protect SC rights.
- **Quality Education and Skill Development:** Improve access to quality education, scholarships, digital learning, and market-oriented skill training for socio-economic mobility.
- **Economic Empowerment:** Facilitate entrepreneurship, access to credit, land ownership, and employment opportunities through targeted interventions.
- **Effective Land and Asset Reforms:** Address historical deprivation by ensuring land redistribution, secure land titles, and access to productive assets.
- **Political and Administrative Representation:** Ensure meaningful participation of SCs in governance, decision-making institutions, and public administration.
- **Address Caste-Based Discrimination and Untouchability:** Promote social inclusion through awareness campaigns, strict law enforcement, and community engagement initiatives.
- **Bridging the Digital Divide:** Enhance digital literacy, internet connectivity, and access to technology to reduce educational and economic inequalities.

True social justice demands that Scheduled Castes move from mere protection to empowerment. Ensuring education, economic opportunities and political participation will transform historical marginalisation into equitable progress, realising Ambedkar's vision of an inclusive India.

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8 Scheduled Tribes

India's Scheduled Tribes (STs), comprising about 8.6% of the population, face socio-economic marginalisation despite constitutional safeguards. Social justice efforts aim to address exploitation, displacement, and cultural erosion for their empowerment.

UPSC Mains PYQs

1. Why are the tribals in India referred to as 'the Scheduled Tribes'? Indicate the major provisions enshrined in the Constitution of India for their upliftment. **(2016)**

1. Constitutional Provisions

Aspect	Explanation
Definition and Recognition	• Article 342: President specifies STs for states/UTs after consultation with governors, ensuring legal recognition of tribal communities.
Protection Against Exploitation	• Article 23: Prohibits human trafficking, forced labor, and protects STs from bonded labor. • Article 24: Bans child labor in hazardous jobs.
Educational/ Economic Interests	• Article 46: Directs state to promote ST's' education, economic interests, and prevent social injustice.
Reservations	• Articles 330, 332, 243D, 243T: Reserve seats for STs in Parliament, state legislatures, panchayats, and municipalities to ensure political representation.
Safeguards for Land/ Culture	• Article 19(5): Restricts non-tribal land transfers in ST areas. Fifth/Sixth Schedules: Govern Scheduled Areas, protecting land, autonomy.
National Commission	• Article 338A: Establishes NCST to monitor ST safeguards, investigate rights violations, and advise on development.

2. Issues Faced by Scheduled Tribes

Aspect	Explanation	Examples
Land Alienation	• STs face dispossession from customary lands due to development-induced displacement, mining, infrastructure, and encroachment. • Lack of implementation of FRA 2006 weakens their tenurial security and violates their ethno-environmental rights.	• About a crore of Adivasis, 40% of all displaced people in India, have been displaced since independence on account of 'development'.
Weak Policy Implementation	• FRA, PESA, and schemes suffer from bureaucratic apathy, corruption, and low awareness, reducing impact.	• Only 40% FRA titles were distributed for forest land in the last decade.
Economic Backwardness	• Tribal economies are marked by subsistence farming, limited market access, and absence of livelihood diversification. • Structural poverty is perpetuated by forest-based dependence and exclusion from formal credit and skilling systems.	• **NITI Aayog National MPI:** STs exhibit the highest headcount ratio of multidimensional poverty among all social groups (~32.5%).
Educational Deprivation	• Poor infrastructure, language mismatch, and culturally irrelevant pedagogy hinder learning. • Remote habitations, teacher absenteeism, and seasonal migration worsen dropout rates.	• Higher secondary-level GER for ST is at a concerning 51.9% • Tribal literacy rate at 59% vs. national average of 74% (Census 2011).
Health Disparities	• Tribal populations face the double burden of undernutrition and disease, poor public health infrastructure, and cultural alienation from modern healthcare. Maternal and child health indicators remain poor.	• NFHS-5 (2020-21): ST IMR is 41.6/1,000 vs. national 35.2 • Anaemia among ST women is 64.6%.

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Atrocities & Exploitation	• STs are subjected to institutional discrimination, bonded labor, trafficking, and sexual violence, aggravated by their geographical isolation and lack of legal awareness.	• NCRB 2023: Crimes against Scheduled Tribes saw a jump of 28.8% from 10,064 cases in 2022 to 12,960 cases in 2023
Climate Vulnerability & Displacement	• Climate change-induced events like floods and droughts increase tribal vulnerability due to low adaptive capacity, dependence on natural resources, and exclusion from climate action plans.	• IPCC (2022) & UNDP India – tribal belts in Jharkhand and Odisha most climate-sensitive.

3. Initiatives For STs

A. Schemes

Scheme	Description	Examples
Dharti Aaba Janjatiya Gram Utkarsh Abhiyan aka PM Janjatiya Unnat Gram Abhiyan (PM-JUGA)	Aims for improving socio-economic conditions of tribal communities with basic entitlements under government schemes.	The program targets 549 districts, 2,911 blocks, and approximately 63,843 villages across 30 states and union territories, aiming to benefit over 5 crore tribal people.
Eklavya Model Residential Schools (EMRS)	Provides free education (Classes 6-12) in tribal areas, promoting literacy, skill development for ST students.	499 EMRS are functional, providing free residential education to more than 1.56 lakh ST students nationwide.
Van Dhan Yojana	Empowers STs by promoting self-help groups for forest produce value addition, ensuring livelihoods.	TRIFED has sanctioned 3958 Van Dhan Vikas Kendras associating 11.83 lakh beneficiaries since its inception in 2019-20.
Tribal Sub-Plan (TSP)	Allocates funds for ST development (education, health, infrastructure) proportional to the ST population.	The Ministry of Tribal Affairs received a record allocation of ₹14,925.81 crore in the 2025–2026 Union Budget.
PM Vanbandhu Kalyan Yojana	Holistic scheme for ST education, health, livelihoods, and cultural preservation in tribal areas.	The scheme is national in scope, with an outlay of over Rs. 26,000 crore from 2021–22 to 2025–26.

B. Legislative Measures

Legislation	Explanation	Examples
Forest Rights Act (FRA), 2006	Recognises ST's land rights, forest resource access, reversing colonial restrictions.	As of January 2026, total titles distributed reached ~24.5 lakh (cumulatively).
SC/ST (Prevention of Atrocities) Act, 1989	Punishes atrocities against STs; special courts ensure swift justice. The 2015 Amendment strengthened penalties.	Recent NCRB trends show registered cases remaining high, averaging ~52,000 to 75,000 annually for SC/ST combined.
PESA Act, 1996	Empowers Gram Sabhas in Scheduled Areas to manage resources, minor minerals, and enhance local governance.	Out of 10 Fifth Schedule states, 9 have now framed rules (Odisha remains the only state yet to notify).
Constitution (ST) Order (Amendment) Act, 2021	Updates the ST list to include new communities, ensuring benefits reach marginalised tribes.	Beyond the Hatti community, recent amendments added Bondo Porja, Khond Porja, and Konda Savaras (Andhra Pradesh) to the ST list.

C. Institutional Measures

Institution	Explanation
National Commission for STs (NCST)	• **Safeguard Monitoring:** Investigates, monitors, and evaluates the effectiveness of constitutional and legal safeguards provided to tribal communities. • **Grievance Redressal:** Looks into specific complaints regarding the deprivation of rights and safeguards of Scheduled Tribes. • **Policy Advisory:** Advises the Central and State Governments on the planning and socio-economic development of tribal areas.

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Tribal Cooperative Marketing Development Federation (TRIFED)	• Its core mandate is the socio-economic development of tribal communities by institutionalising the trade of Minor Forest Produce (MFP) and Surplus Agricultural Produce (SAP).
Ministry of Tribal Affairs	• Formulates policies, schemes for ST welfare, coordinating with states.

4. Way forward

Measure	Explanation	Committee Recommendations
Strengthen FRA Implementation	Expedite land title distribution, train officials to reduce rejections, empower Gram Sabhas.	**Xaxa Committee (2014):** Simplify FRA processes.
Enhance NCST Powers	Grant NCST statutory authority to enforce safeguards, penalise violations, and ensure accountability.	**Parliamentary Committee**: Independent administrative and financial powers for NCST, including a separate budget, instead of routing all expenditure through the Ministry of Tribal Affairs.
Boost Education Access	Expand EMRS, introduce tribal language curricula, and increase scholarships to curb dropouts.	**The Bhuria Committee (1995)** promoted tribal education by recommending the decentralization of education governance.
Promote Livelihoods	Scale up Van Dhan, skill training in IT, tourism, to reduce poverty, migration.	**Xaxa Committee (2014):** Link STs to markets.
Cultural Preservation	Document and preserve tribal languages, indigenous knowledge systems, and traditional practices while promoting community-led cultural tourism.	**UNDRIP (2007)**: Recognises indigenous peoples' right to preserve and strengthen their distinct cultural institutions and heritage.
Strengthening Tribal Identification & Governance	Periodically review and rationalise the Scheduled Tribe lists to reflect contemporary socio-anthropological realities and ensure that constitutional safeguards reach genuine beneficiaries.	**Lokur Committee (1965)**: Recommended a scientific and periodic revision of the lists of Scheduled Castes and Scheduled Tribes using objective criteria.

Empowering Scheduled Tribes requires balancing preservation of identity with integration into mainstream development. Ensuring land rights, education and sustainable livelihoods will transform tribal areas into engines of inclusive growth and cultural richness.

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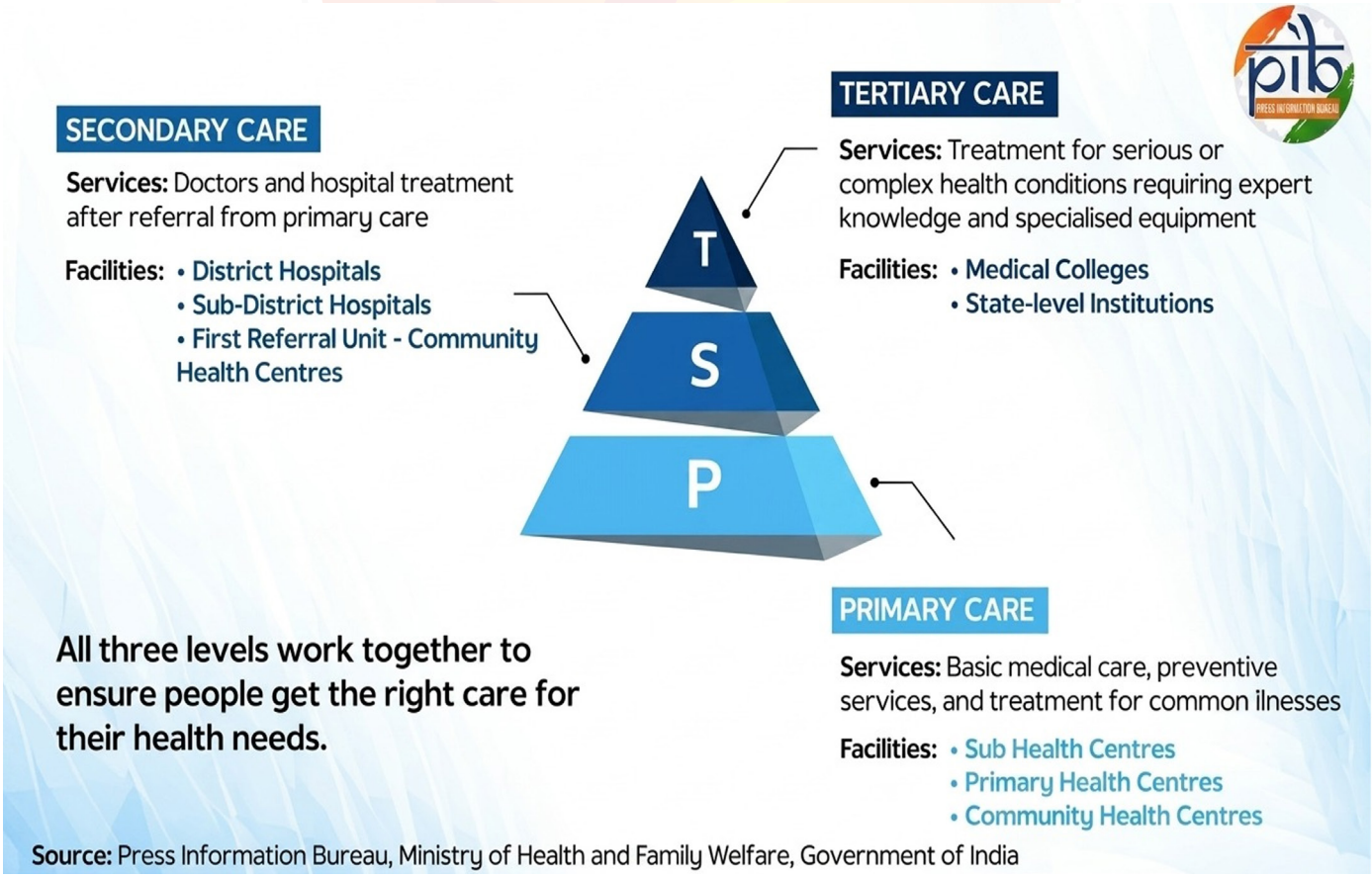
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9 Health

According to the World **Health Organisation (WHO)**, health is a state of complete physical, mental, and social well-being, not merely the absence of disease or infirmity. **India's public health expenditure remains low at 1.9% of GDP**, underscoring the need for strengthened healthcare systems to achieve comprehensive well-being.

UPSC Mains PYQs

1. In a crucial domain like the public healthcare system, the Indian State should play a vital role to contain the adverse impact of marketisation of the system. Suggest some measures through which the State can enhance the reach of public healthcare at the grassroots level. **(2024)**
2. "Besides being a moral imperative of a Welfare State, primary health structure is a necessary precondition for sustainable development." Analyze. **(2021)**
3. In order to enhance the prospects of social development, sound and adequate health care policies are needed in the fields of geriatric and maternal health care. Discuss. **(2020)**
4. Appropriate local community-level healthcare intervention is a prerequisite to achieve 'Health for All' in India. Explain. **(2018)**
5. The public health system has limitations in providing universal health coverage. Do you think that the private sector can help in bridging the gap? What other viable alternatives do you suggest? **(2015)**
6. Identify the Millennium Development Goals (MDGs) that are related to health. Discuss the success of the actions taken by the Government for achieving the same. **(2013)**



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1. Data related to health in India

Aspect	Details
Government Healthcare Spending	India's public health expenditure stands at 1.9% of GDP, below the National Health Policy 2017 target of 2.5% by 2025.
Budget Allocation for Healthcare	₹1,06,530 Crore allocated in the 2026-27 Budget, marking a 10% rise from the previous year.
High Out-of-Pocket Expenditure (OoPE)	The Out-of-Pocket Expenditure (OOPE) as percentage of Total Health Expenditure (THE) declined from 62.6% (2014-15) to 39.4% (2021-22).
Gaps in Insurance Coverage	Around 400 million people in India remain uninsured and exposed to health-related financial risks (NITI Aayog).
Infrastructure	India has 1.6 hospital beds per 1000 population (2021), compared to China's 5.6 (2023) and the US's 2.7 (2022) .
Urban-Rural Healthcare Disparities	2025–2026: Estimated doctor-population ratio in India is 1:811(including both registered allopathic doctors and AYUSH practitioners); disparity exists between urban and rural areas, with a shortage of specialized care.
NFHS-6	• **Total Fertility Rate (TFR):** 1.9 (below the replacement level). • **Child Malnutrition:** Stunting at 29.3%, underweight at 31.8%, and wasting at 19.0%. • **Full Vaccination Coverage (age 12-23 months)**: Increased to 87.1% from 83.8% in NFHS-5 • **Maternal and Child Health:** Institutional Deliveries increased from 90.6%, with urban areas at 95.2% and rural areas at 89%. • **Health Insurance:** Households with at least one member covered by a health insurance or financing scheme rose sharply to 60.2% from 41.0% • **Non-Communicable Disease Indicators (Adults):** High Blood Sugar in 17.8% of women & 22.1% of men, Hypertension in 19.4% of women and 22.1% of men.

2. Constitutional Provisions related to Health

Constitutional Provision	Description
Article 21	• Guarantees the right to life and personal liberty, which the Supreme Court has interpreted to include the right to health and medical care as essential for a dignified life.
Article 39(e)	• Directs the State to protect the health and strength of workers, including men, women, and children, preventing their exploitation.
Article 41	• Obligates the State to provide public assistance to those who are sick, disabled, or elderly, ensuring their welfare.
Article 42	• Mandates the State to secure maternity benefits and protect the health of mothers and infants.
Article 47	• Requires the State to improve nutrition, living standards, and public health, including measures to prohibit harmful substances like intoxicating drinks and drugs.

3. Public Health Care

Significance of Public Health Care	Issues in Public Health Care
• Improved healthcare access by enhancing the availability and quality of services, addressing India's low global ranking in healthcare access. ♦ **Example:** 1.81 lakh+ Ayushman Arogya Mandirs (AAM) are now operational (as of late 2025/early 2026)	• Limited access to healthcare due to shortages of infrastructure and healthcare professionals, especially in rural areas. ♦ **Example:** India ranks 145th in terms of quality and accessibility of healthcare. (Lancet 2022, HAQ Index 2019))
• Better health outcomes through **early detection and treatment**, reducing disease burden and improving population health.	• Low government expenditure on health, around **1.9% of GDP**, which is significantly less than peer countries, limiting system capacity.
• **Reduced financial burden** on households by lowering out-of-pocket expenses and preventing medical impoverishment.	• Insufficient emphasis on **preventive care** despite the triple burden of communicable, non-communicable, and emerging diseases.

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- Promotion of **social justice** by ensuring universal, publicly funded healthcare that provides equitable access regardless of socio-economic status.
- Strengthened **disease prevention and control** through improved sanitation, surveillance, and health education, preventing outbreaks and enhancing community health.
- Focus on **short-term results** like hospital construction rather than long-term development of primary and secondary healthcare weakens the system.
- **Over-reliance on physician-centric care** without integrating multidisciplinary expertise from public health, sociology, and environmental science.

4. Primary Healthcare

Issue Faced by Primary Healthcare	Solutions
• **Poor Infrastructure and Non-adherence to Standards:** With hundreds of thousands of Sub-Centres (SCs) and Primary Health Centres (PHCs) nationwide, a significant portion falls short of minimum benchmarks for care, diagnostics, and human resources.	• Strengthen infrastructure by upgrading PHCs and Sub Centres into **Health and Wellness Centres (HWCs)** under Ayushman Bharat, aiming for comprehensive primary care with adequate facilities.
• **Severe Shortage of Healthcare Professionals:** There is a shortfall of over 79.9% specialists at CHCs and significant vacancies among doctors, nurses, and paramedics, especially in rural areas.	• Expand medical and nursing education capacity, increase recruitment, and implement retention incentives for rural postings. (**Srinath Reddy Committee** Report)
• **Urban-Rural Disparity in Access:** Rural populations face limited access due to uneven distribution of facilities and staff, with PHCs and Community Health Centres (CHCs) serving populations far beyond norms.	• Promote **equitable resource allocation,** incentivise rural service, and use telemedicine and mobile health units to improve rural healthcare reach.
• **Low Quality and Limited 24x7 Functionality:** Less than half of AAMs operate round-the-clock, and many lack specialists or essential services, affecting quality and timely care.	• Strengthen **24x7 service delivery** at AAMs through improved staffing and infrastructure; implement quality assurance and monitoring mechanisms under the National Health Mission.
• **High Out-of-Pocket Expenditure and Dependence on Private Sector:** Public primary care remains under-resourced, with India's health spending at 1.9% of GDP—below the 2.5% target set by the National Health Policy, 2017.	• **Expand coverage and utilisation** of Ayushman Bharat PM-JAY for secondary and tertiary care; allocate at least 70% of healthcare budget to primary care, aligning with WHO's 50% recommendation.

Human Resources for Health: ASHA and Anganwadi Workers

Known as the grassroots-level crusaders of India's healthcare system, ASHA workers are non-medical health workers appointed and trained by the National Rural Health Mission. Anganwadi Workers/Helpers (AWWs) are defined as "honorary workers" from the local community rendering services on a part-time basis for child care and development. There were 13,48,135 Anganwadi workers (2023) and over 10 lakh Accredited Social Health Activist (ASHA) workers .

Role of ASHA and AWWs: The Backbone of Public Health

Role	Description
Link Worker / Facilitator	• **Healthcare Conduit:** Primary link between rural populations and government health systems to drive bottom-up healthcare access. ♦ **Example**: Under Ayushman Bharat (ABDM), they link rural citizens to digital health infrastructure by generating unique ABHA IDs.
Service Extension Worker	• **Last-Mile Delivery:** Provides a basic package of curative care, basic medicines, and first aid for minor ailments (like ORS, contraceptives, delivery kits, etc). • **Maternal Health Liaison:** Identifies pregnant women early and coordinates mandatory registration for health monitoring. ♦ **Example**: Frontline driver of the Janani Suraksha Yojana safe motherhood initiative to promote institutional deliveries and reduce MMR/IMR.
Community Health Advocate	• **Behavior Architect:** Mobilizes communities for health campaigns, challenges patriarchal norms, and raises awareness on social determinants of health. ♦ **Example:** Under **Beti Bachao Beti Padhao (BBBP)**, they lead village-level campaigns to counter sex selection, child marriage, and gender discrimination. • **Accountability Leader:** Organizes local health planning networks, tracks emerging risks, and screens for non-communicable diseases.

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Anganwadi Worker (AWW) Activities	AWWs manage local Anganwadi centers as the focal point for early child development and nutrition through three core pillars: • **Supplementary Nutrition:** Prepares and distributes hot meals and take-home rations for children (aged 0–6) and pregnant/lactating women. ♦ **Example**: Under Poshan 2.0, they distribute targeted hot-cooked meals and Take-Home Rations (THR) complying with NFSA norms. • **Pre-school Education:** Provides foundational, non-formal early childhood education for children aged 3–6. • **Growth Monitoring:** Tracks children's weights and assists medical staff during monthly Village Health and Nutrition Days. ♦ **Example**: They upload real-time infant height/weight logs on the **Poshan Tracker** ICT portal to map stunting and wasting patterns.

Structural Barriers & Challenges Faced

Compensation and Financial Insecurity	• **The "Volunteer" Trap:** Legally, they cannot be declared permanent government servants nor given any civil post, Classed as "informal laborers" receiving performance-based, irregular incentives rather than fixed wages. • **Protest Spark:** Indefinite strikes across Delhi, Haryana, Jammu, Bhubaneswar, and Assam (beginning August 28, 2023) demanding fixed regularized payment systems indexed to **State Minimum Wages**.
Operational Overload & Educational Gaps	• **Role Creep:** Tasked with over 30 jobs, causing intense work overload where administrative surveys frequently overshadow core health-related duties. • **Skill Deficit:** Performance is hindered by low education levels, inadequate digital skills for modern digital health applications, and a severe **training gap**.
Institutional Hierarchy & Toxic Work Culture	• **Subordination vs. Synergy:** Treated as absolute subordinates rather than partners by Medical Officers (MOs) and Auxiliary Nurse Midwives (ANMs). Cases reported by ASHAs rarely receive priority treatment or acknowledgement. • **Infra Deficiencies:** Absolute lack of basic workplace infrastructure, such as designated seating or rest areas at Primary Health Centers (PHCs) after handling late-night emergency deliveries.
Social and Safety Hazards	• **Vulnerability:** Placed at the bottom of the socio-economic hierarchy, facing severe class disadvantages while negotiating sensitive behavioral changes (contraception, child marriage, sex discrimination) within patriarchal village setups. • **Lack of Protection:** Subjected to widespread physical attacks, threats, and abuse during door-to-door community tasks. ♦ **Example:** Frontline ASHAs faced verbal abuse and over 50 documented physical attacks while conducting door-to-door COVID-19 screening and tracking vaccination hesitancy. • **Zero Social Security:** Lack of access to basic safety nets—including **maternity leave, health insurance, life insurance, gratuity, provident funds, or post-retirement benefits**. This creates severe domestic friction and low-income traps for their families.
Dashboard Governance and Digital Labor	• **Uncompensated Digital Labor:** The shift to "dashboard governance" forces ASHAs to execute heavy data entry and algorithmic tasks without internet allowances or extra pay. • **Flawed Data Dependencies:** Rigid tech metrics overlook ground realities and lack contextual design, causing systemic operational failures. ♦ **Example**: Glitches in the Poshan Tracker app cause missed automated nutrition alerts, causing critical delays in supplementary food distribution for malnourished children.

Steps taken by the Government for ASHA and AWWs

Financial Incentives	• Benefits of Life insurance, accident insurance and pension to eligible ASHAs and ASHA facilitators are extended by enrolling them under: PM Jeevan Jyoti Beema Yojana, PM Suraksha Beema Yojana PM Shram Yogi MaanDhan. • The government has also approved a cash award of Rs. 20,000/- and a citation to ASHAs who leave the programme after working as ASHAs for minimum of 10 years, as acknowledgement of their contribution

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Capacity Building	• Enhanced promotional avenues, allocating quotas for eligible Anganwadi Helpers to become Workers (50%), and for AWWs to become Supervisors (50%). • Thousands of Anganwadi Centres are actively being upgraded into state-of-the-art Saksham Anganwadis to improve infrastructure and early childhood care. • AWWs are provided up to 20 days of annual leave and 180 days of paid maternity leave.
Technological Empowerment	• AWWs have been equipped with smartphones to transition from physical registers to the Poshan Tracker Application, bringing transparency and efficiency to their daily reporting. • The government provides AWWs with ₹2,000 per annum to cover internet connectivity charges for real-time digital reporting on the Poshan Tracker.

The Way Forward & Policy Interventions

- **Financial Security and Formalization:** Transitioning ASHA workers from "honorary volunteers" to recognized contractual employees grants them fixed, reliable monthly honorarium or minimum wage & essential social security benefits
- **Transition to an "Ethical Employer" Model:** The government must bridge the fine line between "volunteer" and "formal employee" to grant robust labor rights, job security, and structured grievance redressal mechanisms.
- **Institutional Redressal Committees:** Constitute official committees with direct ASHA representation to safeguard rights and resolve operational concerns.
- **Regular Up-skilling & Recruitment:** Ensure continuous funding for regularized institutional training, digital literacy upgrades, and fresh recruitment to offset the severe workforce deficit.

5. Maternal Health Care

According to WHO, maternal health refers to the health of women during pregnancy, childbirth, and the postnatal period, encompassing care from family planning to postnatal support to ensure safe and fulfilling outcomes.

India has reduced the Maternal Mortality Ratio (MMR) from 130 in 2014–16 to 93 now per 1,00,000 live births (SRS Report 2021), showcasing significant progress toward achieving the SDG target of reducing MMR below 70 by 2030.(8 states have already achieved the SDG target of MMR < 70 including Kerala, Maharashtra, Telangana, and Andhra Pradesh).

Issues	Solutions
Significant **interstate disparities** persist despite overall decline in maternal mortality ratio (MMR). For example, Uttar Pradesh and Madhya Pradesh continue to report MMR significantly higher than the national average	Prioritise high-burden States through need-based fiscal transfers, outcome-based financing, and strengthened maternal health systems under the National Health Mission.
India's healthcare system is still very **doctor-centric**, in contrast to Scandinavian nations or the UK, where midwives oversee low-risk deliveries. Because the Nurse Practitioner in Midwifery (NPM) initiative is still in its early stages, current nurses are overworked.	Scale up the Nurse Practitioner in Midwifery (NPM) initiative by expanding training, certification, and deployment of midwife-led continuity care units to manage low-risk pregnancies.
Widespread **maternal anaemia** due to poor nutrition and lack of supplementation. According to NFHS-5, a staggering **57% of women (aged 15-49)** in India are anaemic.	Implement comprehensive nutrition programs focusing on screening, treatment of anaemia, and nutrition education for pregnant women.
Limited access to quality emergency obstetric care in rural and remote regions. 95.3% of urban births are attended by skilled health personnel, whereas this figure drops to 90.0% in rural areas and to 70.1% in Nagaland (NFHS-6).	Strengthen healthcare facilities with emergency obstetric services, improve referral systems, and upgrade labour rooms to ensure timely and quality care.
Early marriage and teenage pregnancies increase risks of maternal mortality and morbidity. Despite legal frameworks, 20.1% of women aged 20-24 years were married before the age of 18 (NFHS-6).	Promote girls' education, reproductive health awareness, and delay age of marriage.

6. Community Health Care

Community health, a subset of public health delivered outside hospitals by health workers and volunteers, plays a crucial role in bridging communities with formal healthcare systems. In India, it is essential for achieving the universal goal of 'Health for All' through localised interventions tailored to diverse population needs.

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Importance of Community Health

- **Universal Healthcare Access:** Extends essential healthcare services to underserved and remote populations. It can address these disparities by customising health programmes to suit the needs of specific communities. For example, Ayushman Arogya Mandirs provide primary healthcare at village level.
- **Reducing Healthcare Costs:** Early diagnosis and treatment reduce burden on secondary and tertiary healthcare facilities.
- **Health Equity and Social Justice:** Bridges healthcare disparities among marginalised and vulnerable communities.
- **Community Participation and Ownership:** Encourages local involvement in planning and implementation of health programmes. For example, Village Health Sanitation and Nutrition Committees promote community engagement.

Issues	Solutions
Limited access to healthcare in remote and underserved areas restricts timely and quality care. Rural populations face limited healthcare access compared to urban areas.	Establish health centres, mobile clinics, and telemedicine facilities to bring healthcare services closer to rural and tribal populations.
Inadequate infrastructure and shortage of trained healthcare workers hinder effective community health delivery; only 4,413 specialist doctors available against a need of 21,964 in rural CHCs (March 2023).	Strengthen primary healthcare infrastructure and expand community health worker programs like ASHA under the National Health Mission to improve outreach and service quality.
Low health literacy and lack of awareness contribute to poor health-seeking behavior and persistence of preventable diseases.	Conduct targeted health education campaigns and capacity-building initiatives to raise awareness and encourage adoption of healthy practices.
Fragmented service delivery and poor coordination between community and formal health systems reduce effectiveness.	Enhance the integration of community health programs with primary healthcare centres and leverage digital health platforms for better coordination and monitoring.
Socio-economic and cultural barriers limit community engagement and equitable access to healthcare.	Promote inclusive policies and culturally sensitive programs that engage marginalised groups and empower local leadership in health decision-making.

7. Mental Health Care

Mental health, as defined by the World Health Organization, is a state of well-being that enables individuals to cope with life's stresses, realise their potential, and contribute meaningfully to society. Ensuring good mental health is essential for social justice, as it promotes emotional resilience, reduces stigma, and supports equitable access to care for those affected by mental disorders and psychosocial disabilities.

In 2021, it was estimated that 1 in 7 people globally were living with a mental disorder, which equates to over 1.1 billion people. In India, about **11 in 100 people suffer from mental health disorders**. Moreover, suicide rates in India are on the rise. According to the 2023 National Crime Records Bureau (NCRB) report 'Accidental Deaths & Suicides in India': 171,418 suicides were reported in the country in 2023. The 2023 NCRB report revealed a significant gender disparity in suicides: **males accounted for 72.8% of all suicides**, while females accounted for 27.2%.

Issue	Solution
Low Policy Priority: Govt allocation remains low (₹1,000 crore in 2025-26 vs. ₹93,000 crore needed); the mental health budget is ~1% of total health spend.	Increase funding as a greater share of the health budget, and equitable distribution across all care levels. The **National Mental Health Programme (NMHP)** aims to integrate services at all levels.
Urban Stress: Rapid urbanization and overcrowding increase anxiety and depression. For instance, urban prevalence is 13.5% versus 6.9% in rural areas (NMHS).	Promote telemedicine, mobile mental health units, and community health worker-based interventions in rural and tribal regions.
Inadequate Infrastructure and Workforce: **Only 0.75 psychiatrists per 100,000 population**; shortage of facilities and professionals.	Strengthen infrastructure at district/community levels; Expand training through **NIMHANS, iGOT-Diksha**, and public-private partnerships.
High Treatment Gap: According to the National Mental Health Survey (2015–16), 70–92% of those affected do not receive any treatment due to social stigma, poor awareness & lack of accessible services.	Scale affordable access via National Tele Mental Health Programme **(Tele-MANAS)**; reduce OOPE through public provision and subsidies.

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Mental Health of Children and Adolescents: Academic pressure, cyberbullying, social media exposure, and family stress increasingly affect children's and adolescents' mental well-being.	Integrate mental health into school curriculum, establish school counselling services, life-skill education programmes, and adolescent-friendly mental health support mechanisms.
Stigma and Discrimination: Deep-rooted stigma discourages individuals from seeking treatment, leading to delayed diagnosis, social exclusion, and worsening mental health conditions.	Conduct mass awareness campaigns; integrate Kiran Helpline, MANAS App, and school-based education to improve mental health literacy.

Initiatives for Mental Well-being

- **Mental Healthcare Act, 2017: Mental health and physical health to be treated at par by the law, insurers to cover mental illness, treatment of mental illness to be rights based.**
- **National Mental Health Programme (1982) & District Mental Health Programme (1996):** To provide mental health services that are community-based, accessible and affordable through awareness at the grassroots level.
- **Tele MANAS:** A nationwide tele-mental health service providing 24×7 digital counselling and support, particularly in underserved and remote areas.
- **National Suicide Prevention Strategy (2022):** Aims to reduce suicide rates through public awareness, de-stigmatisation, early detection, and intervention mechanisms.

8. Universal Health Coverage

According to WHO, Universal Health Coverage means all people have access to the full range of **quality** health services they need, **when and where** they need them, **without suffering financial hardship.**

Significance of UHC	Example
Promotes Economic Growth: Universal healthcare improves workforce productivity, reduces absenteeism, and prevents poverty caused by medical expenses, thereby contributing to long-term economic growth and human capital development.	China's UHC reforms significantly reduced poverty while supporting rapid economic growth.
Reduces Out-of-Pocket Expenditure: UHC provides financial risk protection, preventing households from falling into poverty due to catastrophic healthcare expenses and ensuring affordable access to essential services.	AB-PMJAY generated ₹1.25 lakh crore OOP savings for beneficiaries.
Improves Health Outcomes: Greater access to preventive, primary, and specialized healthcare services enhances life expectancy, lowers mortality rates, and improves overall population health indicators.	Kerala and Tamil Nadu show superior health outcomes through stronger public healthcare systems.
Reduces Disease Burden: Preventive healthcare, screening, and early diagnosis help reduce the burden of communicable and non-communicable diseases, improving productivity and quality of life.	Early NCD interventions are necessary as by 2030, NCDs will account for almost three-fourth of all deaths.
Promotes Social Equity: UHC ensures healthcare access irrespective of income, caste, gender, or location, reducing disparities among vulnerable and marginalized populations.	NSS data shows marginalized communities bear the highest healthcare expenses.
Strengthens Human Development: Healthy populations perform better in education, employment, and social participation, contributing to broader developmental goals and national well-being.	Countries with robust UHC systems generally rank higher on human development indicators.
Boosts Medical Tourism and Healthcare Leadership: Strong healthcare infrastructure developed under UHC can attract international patients and strengthen a country's healthcare reputation globally.	India's medical tourism industry generated $7.69 billion in 2024.

Challenges to UHC	Description
Inadequate Public Health Financing	India's public health expenditure is low at 1.9% of GDP (2025-26), below the National Health Policy 2017 target of 2.5%. This limits the capacity to strengthen primary healthcare, leading to over-reliance on costly tertiary care and late diagnosis of manageable diseases.
Urban-Rural Healthcare Disparities	Healthcare infrastructure and professionals are concentrated in urban areas, while 65% of the population lives in rural regions with inadequate access, resulting in inequitable health outcomes and poor service delivery.

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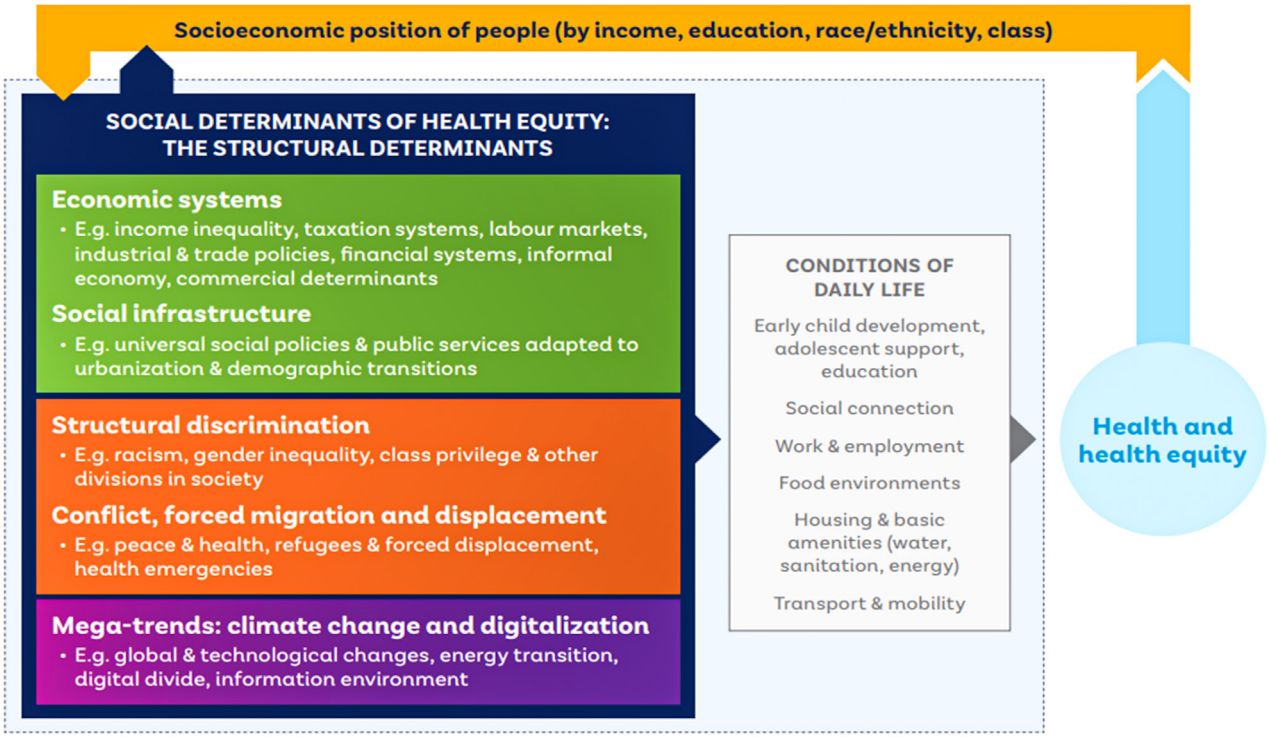
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Rising Burden of Non-Communicable Diseases (NCDs)	NCDs account for about 63% of deaths in India, aggravated by pollution, poor sanitation, and malnutrition, requiring comprehensive preventive and curative healthcare interventions at all levels.
Persistent Out-of-Pocket Expenditure	Despite government schemes, many households continue bearing substantial healthcare costs, weakening financial risk protection under UHC. OOP expenditure still accounts for nearly half of health spending.
Health Insurance Coverage and Financial Protection	Limited insurance penetration leaves most Indians uninsured, with high out-of-pocket expenses pushing millions into poverty; expanding schemes like Ayushman Bharat PM-JAY is critical for financial risk protection.
Excessive Dependence on Private Sector	Weak public healthcare infrastructure increases reliance on private providers, leading to affordability and quality concerns. Two-thirds of outpatient healthcare services are delivered by private providers.

8.1. Health Equity

As per the World Report on Social Determinants of Health Equity by WHO, Health equity is defined as the state where everyone, regardless of their circumstances, has the opportunity to enjoy the highest attainable standard of health and well-being. It goes beyond just the fair distribution of health outcomes to include a fundamental commitment to "leave no one behind," which is a central promise of the 2030 Agenda for Sustainable Development.

Social Determinants of Health Equity



1. Structural Determinants	2. Conditions of Daily Life	3. Individual Socioeconomic Position
These are the "upstream" factors that shape society, including economic systems (income inequality), social infrastructure (public services), structural discrimination (racism, sexism), and mega-trends like climate change and digitalization.	These are the specific circumstances in which people are born, grow, live, work, and age. They include Early childhood development and education, Housing, water, sanitation, and energy, work & employment, etc	Health is also dimensioned by a person's income, education level, race, ethnicity, gender, disability status, and geographic location

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Challenges to Achieving Health Equity

- **Global Crises:** Progress has been stalled by a series of **predictable and preventable crises**, including the COVID-19 pandemic, financial instability, climate change, and persistent conflict.
- **Widening Internal Gaps:** While gaps between countries have sometimes narrowed, **inequalities within many countries are widening**, which traps lower-income nations in cycles of under-development.
- **Economic Barriers:** High levels of **external debt** limit the "fiscal space" for governments to invest in essential public services like health and education.
- **Structural Discrimination:** Deeply embedded systems of **racism, sexism, and classism** intersect to undermine people's agency and health across generations.
- **Digital Divide:** While technology offers benefits, the **unequal access to digital services** and the rise of misinformation/ disinformation create new barriers to equity.
- **Commercial Determinants:** Practices by some commercial actors can escalate **ill health and environmental damage**

Strategies to Achieve Health Equity

Address Economic Inequality	• Use progressive taxation and well-being budgeting to fund universal public services and social protection programs (like unemployment benefits and pensions)
Tackle Structural Discrimination	• Dismantle systemic inequities through legal and institutional change, the realization of human rights, and restorative justice/reparations for affected communities
Steer Mega-trends Toward Equity	• Climate Action: Prioritize a "just energy transition" that does not worsen inequities for vulnerable populations. • Digitalization: Implement human rights frameworks and regulations to ensure generative AI and digital tools bridge rather than widen the digital divide
Adopt New Governance Approaches	• Empower Local Governments: Local entities are best positioned to identify and address specific social determinants. • Community Engagement: Co-design policies with those they aim to serve to ensure services are responsive to actual needs. • Primary Health Care (PHC): Use PHC approaches to orient health services toward population needs and create feedback loops to improve daily living conditions. • Data and Monitoring: Strengthen data systems to provide disaggregated data (by age, gender, race, etc.) to track progress and ensure accountability

Relationship between UHC and Health Equity

- **Goal vs. Strategy:** Universal Health Coverage (UHC) sets the quantitative goal—ensuring *everyone* accesses medical care without financial ruin. Health Equity is the strategy or "compass" that allocates resources based on varying needs, ensuring the most marginalized are reached.
- **Fixing Systemic Gaps:** Standard models (UHC 1.0) often suffer from "design slippage," where benefits are disproportionately hijacked by wealthier or urban areas. A health equity-driven approach upgrades this to "UHC 2.0," explicitly safeguarding the vulnerable and the "missing middle."
- **Progressive Financing:** While UHC provides the institutional vehicle (hospitals and insurance), equity determines how it is funded. It demands progressive mechanics, such as mandatory income-based contributions, so the rich subsidize care for the poor.
- **Clinical Care vs. Holistic Wellness:** UHC typically restricts its focus to clinical inputs like doctors, beds, and medicines. Health equity complements UHC by addressing non-medical social determinants (nutrition, clean water, housing), preventing patients from immediately falling sick again.

Major Ongoing Initiatives in India Promoting Health Equity

- **National Health Mission (NHM):** This serves as an umbrella for the **National Rural Health Mission (2005)** and the **National Urban Health Mission (2013)**. The NHM is built on the core principle of health equity, with the urban component specifically focusing on the poor and marginalized in urban slums.
- **National Health Policy (NHP) 2017:** This policy is fully aligned with the goal of achieving **Universal Health Coverage (UHC)** in India.
- **Ayushman Bharat (2018):** This program consists of two primary pillars designed to ensure a continuum of care:
  - **Health and Wellness Centers (HWCs):** A commitment to transform 150,000 facilities to deliver **comprehensive primary health care** closer to rural and urban communities.
  - **Pradhan Mantri Jan Arogya Yojana (PMJAY):** A cashless insurance system providing **₹5 lakh per family per year** for secondary and tertiary care. It targets approximately 10.74 crore poor and vulnerable families with no cap on family size or age.
- **Targeted Reporting:** The 2017 report *"India: Health of the Nation's States"* provided critical data on state-level disease burdens to help identify specific inequities. Similarly, an expert committee report on **Tribal Health** identified significant health outcome gaps for tribal populations.

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Proposed Policy Interventions to Achieve Health Equity

1. Strengthening Data and Monitoring	• **Routine Equity Monitoring:** Develop systems to record and report health data that can be analyzed to guide timely interventions. • **Academic Involvement:** Engage academic institutes to guide approaches and **train medical and nursing students** specifically on the concepts of health inequities
2. Targeted Interventions for Underserved Groups	• **Vulnerable Populations:** Use recommendations from expert groups (like the Tribal Health committee) to formulate policy. • **Program Integration:** Link health initiatives for underserved areas to broader national programs like the **Aspirational Districts program** and **Poshan Abhiyan**. • **Human Resources:** Prioritize filling human resource gaps in facilities located in these underserved settings.
3. Village-Level and Community Linkages	• **Community Workers:** Increase the number of community-level health workers and volunteers in under-served areas to improve health-seeking behavior. • **Village Services:** Strengthen primary services at the village level, including community-based screening through HWCs.
4. Infrastructure and Functional Capacity	• **Functionality Over Just Construction:** Move beyond merely creating infrastructure to ensuring these facilities are **fully functional** and located where utilization likelihood is highest. • **Assured Supplies:** Guarantee the provision of **medicines and diagnostics** at all government facilities. Since marginalized populations are most likely to use these facilities, increased funding for these supplies directly reduces their financial burden and health inequities.
5. Governance and State-Level Action	• **State Mechanisms:** Because health is a "state subject" in India, each state should develop its own specific mechanisms for tackling inequities in addition to participating in central union initiatives. • **Whole-of-Government Approach:** Adopt strategies that involve collaboration between the health sector and other sectors to address the root social causes of health gaps.

8.2. Health Insurance in India: A Path to Universal Health Coverage (UHC)

The Current Landscape

- **Coverage Levels:** NFHS-6: 60.2% of Indian households had any usual member covered by a health insurance or financing scheme.
- **The Trinity of Schemes (NITI AYOG, 2021):**
  - **Government-Subsidized (e.g., PM-JAY):** Targets the poor and vulnerable. Bottom ~51% (60 crore) of the population eligible.
  - **Social Health Insurance (e.g., ESIS, CGHS):** Targets formal sector workers. ~10% (14 crore) population eligible.
  - **Private Voluntary Health Insurance (PVHI):** Largely high-income groups or corporate employees. ~9% (11.5 crore) of the population eligible.
- **The "Missing Middle":** Approximately **30% of the population (40 crore individuals)** remain completely uncovered. This group is too wealthy for PM-JAY but too poor or informally employed to afford private plans.
- **Economic Reality:** India's **Out-of-Pocket Expenditure (OOPE)** is at 43.4% (NHA 2022-2023) of current health spending, significantly above the global average.

Key Government Initiatives

- **Pillar 1: PM-JAY:** The world's largest publicly funded health assurance scheme. It provides **₹5 lakh annual cover per family** for secondary and tertiary care, benefiting 12 crore socio-economically deprived families. In 2025, coverage was extended to **all senior citizens aged 70+**, regardless of income.
- **Pillar 2: Ayushman Arogya Mandirs (AAM):** Over **1.86 lakh centres** operationalized to deliver comprehensive primary care closer to communities.
- **Pillar 3: PM-ABHIM:** A ₹64,180 crore mission aimed at **pandemic preparedness** and strengthening health infrastructure.
- **Pillar 4: Digital Mission (ABDM):** Creating **ABHA IDs** (Digital Health Accounts) to ensure seamless, paperless portability of medical records and claims.

Challenges

- **High OOPE despite Coverage:** Between 2017-18 and 2025, OOPE on hospitalization has **more than doubled** in both rural and urban areas. This is often due to the **exclusion of Out-Patient Department (OPD)** expenses, which account for over two-thirds of total OOPE.
- **Utilization Gap:** Nominally, 60% of the population may be covered by various schemes, but **a fraction of insured hospital patients** actually use their insurance due to low awareness or complex clauses.
- **Market Failures:**
  - **Adverse Selection:** Sick individuals are more likely to buy insurance, leading to higher premiums that price out the healthy.
  - **Cream-Skimming:** Private insurers often exclude high-risk individuals via complex "Pre-Existing Disease" (PED) clauses.
- **Sustainability:** Excessive reliance on hospitalization-based insurance may divert resources from **primary healthcare, preventive care,** and public health infrastructure, affecting long-term sustainability.

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Way Forward: A Roadmap for Equity

- **Closing the Missing Middle Gap:** Develop **modular, low-cost products** (indicatively ₹4,000–₹6,000 per family) with lower waiting periods (1-2 years instead of 4).
- **Integrated OPD Care:** Transition from a hospitalization-only model to a **combined IPD+OPD subscription model**. This encourages early diagnosis and reduces the need for expensive surgeries.
- **Institutional Reforms:**
  - **Mandatory Health Claims Exchange (HCX):** A standardized digital platform to reduce claim processing time and minimize fraudulent billing.
  - **Regulatory Tightening:** IRDAI should mandate "Cashless Anywhere" facilities and further shorten moratorium periods to build consumer trust.
- **Leveraging Public Assets:** Use the **JAM trinity** (Jan Dhan, Aadhaar, Mobile) and assets like **post offices** or **ASHA workers** as low-cost distribution channels for insurance outreach in the informal sector

Features of Ayushman Bharat

Scale, Funding & Inclusivity	• **Scale & Cover:** Largest fully government-funded scheme; covers ~60 crore individuals (**12 crore families** + all citizens **70+** via Vay Vandana) providing **₹5 lakh** per family/year for secondary/tertiary care. • **No Limits:** Absolute inclusivity with no restrictions on family size, age, or gender; covers pre-existing conditions from **day one**.
Service Delivery & Care Continuum	• **End-to-End Care:** Covers **1,961 procedures** across **27 specialties** (drugs, diagnostics, food, OT/ICU, physician fees). • **Temporal Extension:** Extends financial protection by covering medical costs incurred **3 days before** hospitalization and **15 days after** discharge.
Access Model & Operational Framework	• **Seamless Access:** Completely **cashless and paperless** operations with **nationwide portability**, allowing beneficiaries to use any empanelled public or private hospital across India.
Target Population Selection	• **Database-Driven Integration:** Identifies and targets eligible beneficiaries objectively using the **SECC 2011** data (deprived rural households and specific urban occupational groups).
Institutional Parity & Governance	• **System Strengthening:** Reimburses public hospitals **at par with private hospitals** to incentivize internal infrastructure growth. • **Apex Oversight:** Maintained and monitored by the **National Health Authority (NHA)** through real-time digital auditing and anti-fraud mechanisms.

Performance of AB-PMJAY

- **Mass Coverage:** By July 2026, PM-JAY had issued more than 43.52 crore health cards and facilitated nearly 1,90,000 crore rupees worth cashless hospital treatments across the country.
- **Gender Inclusion:** The scheme has advanced healthcare equity, with women constituting almost half of the beneficiaries receiving cards and hospital care.
- **Elderly Protection:** Introduction of the Ayushman Vay Vandana Card expanded health coverage to all citizens aged 70 years and above, benefiting nearly 6 crore elderly individuals.
- **Digital Enablement:** Digital platforms such as the Ayushman App have streamlined beneficiary verification, registration, and health card generation processes.
- **Wider Reach:** Inclusion of Delhi and Odisha under PM-JAY in 2025 significantly enhanced the scheme's nationwide coverage and accessibility.
- **Inclusive treatment:** Around 50 packages were designed specifically for the transgender community, including packages on sex reassignment surgery (SRS).

Issues with AB-PMJAY

- **Limited OPD Coverage:** AB-PMJAY primarily covers hospitalization expenses, while outpatient consultations, diagnostics, and medicines—the largest contributors to healthcare spending—remain largely excluded.
- **Urban-Centric Infrastructure:** A significant proportion of empanelled hospitals are concentrated in urban areas, limiting access for rural and remote populations.
- **Low Awareness Levels:** Many eligible beneficiaries remain unaware of scheme benefits, eligibility criteria, and enrollment procedures, reducing effective utilization.
- **Fraud and Misuse Risks:** Instances of unnecessary procedures, fake claims, and overbilling by hospitals have raised concerns regarding transparency and accountability.
- **Weak Primary Healthcare Linkages:** Greater emphasis on tertiary and secondary care has diverted attention from strengthening preventive and primary healthcare services.
- **Supply-Side Constraints:** Shortages of doctors, hospital beds, diagnostic facilities, and healthcare personnel reduce the scheme's effectiveness in several regions.

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9. One Health Approach

The 'One Health' approach, as defined by WHO, emphasizes coordinated action across human, animal, and environmental health sectors to achieve optimal public health outcomes. With rising zoonotic infections and environmental challenges, this integrated framework has gained critical importance in recent years.

Need of One Health Approach for India

Reason	Explanation
Risk of disease transmission	• Growing human encroachment into animal habitats increases close contact with animals, raising the chances of zoonotic infections. For example, outbreaks of Nipah, Avian Influenza (H5N1), and Rabies show that over 60–70% of emerging infectious diseases in India are animal-borne.
Climate change and land use	• Land use changes, deforestation and global warming are altering the habitat of vector species and are expanding the geographical range of vector borne diseases like Dengue and Malaria to new areas and also bringing forest diseases like Kyasanur Forest Disease to the fringes of human habitation.
Food Safety and Livestock Health	• India's large livestock sector requires integrated monitoring to prevent animal diseases from affecting human health and livelihoods. FAO prioritized diseases such as Anthrax, Rabies, African Swine Fever, and Lumpy Skin Disease in India.
Zoonosis and drug resistance control	• Crucial for managing food safety, zoonotic diseases like flu and rabies, and controlling antibiotic resistance transmission. ♦ Example: NAP-AMR 2.0 (National Action Plan on Antimicrobial Resistance) now includes mandatory environmental auditing for pharmaceutical hubs to prevent antibiotic runoff into water bodies .
Preventing Future Pandemics	• Integrated disease surveillance and early warning systems can detect outbreaks before they become national or global health emergencies.

Challenges in Implementing One Health Approach	Measures to Strengthen One Health Approach
Fragmented Institutional Framework: Health, veterinary, and environmental agencies often operate in isolation, resulting in poor inter-sectoral collaboration and information exchange.	**Establish Robust Institutional Mechanisms:** Create a dedicated statutory platform bringing together relevant ministries and stakeholders for coordinated decision-making and policy implementation.
Insufficient Financial and Human Resources: Animal health and wildlife management systems face shortages of funding, infrastructure, trained professionals, and diagnostic facilities.	**Develop Integrated Disease Surveillance:** Build a comprehensive real-time surveillance network linking human, animal, and environmental health databases for timely detection and response.
Limited Public Health Investment: Public expenditure on healthcare remains relatively low, constraining preparedness, disease monitoring, and emergency response capabilities.	**Strengthen Primary Healthcare Systems:** Expand and modernize Ayushman Arogya Mandirs to improve disease prevention, early diagnosis, and community-level healthcare services.
Environmental Degradation and Habitat Loss: Deforestation, land-use change, and rapid urbanisation increase human-animal interactions, elevating risks of zoonotic disease spillovers.	**Promote Sustainable Environmental Management:** Strengthen conservation efforts, regulate land-use changes, and protect ecosystems to reduce disease emergence risks.
Growing Antimicrobial Resistance (AMR): Unregulated use of antibiotics in livestock and poultry contributes to rising antimicrobial resistance across sectors.	**Ensure Rational Antibiotic Use:** Enforce stricter regulations on antibiotic usage in animal husbandry and promote responsible antimicrobial stewardship practices.

Initiatives Related to One-Health Approach

- **National One Health Mission:** A multi-sectorial initiative integrating human, livestock, wildlife and environmental health to strengthen coordinated surveillance, diagnostics and outbreak response. Key pillars include research and development, clinical readiness, data integration, and community engagement.
- **Centre for One Health at NCDC:** Facilitates efforts, runs programs (Rabies, Zoonoses, Leptospirosis, Snakebite), and promotes the One Health approach in India.
- **One Health Joint Plan of Action:** Facilitates framework for 2022–2026 created by the Quadripartite alliance (FAO, UNEP, WHO), and WOAH) to promote one health approach globally.
- **One Health Supporting Unit within Department of Animal Husbandry & Dairying:** A specialised team of experts (veterinary, health, wildlife, data) established to implement the national One Health framework in India.

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10. Privatisation of Health Care in India

Benefits of Privatisation	Challenges of Privatisation
Improves access to specialised healthcare by bringing in private expertise, advanced technology, and modern equipment.	Increases inequality by widening the gap between rich and poor, favouring those who can afford private care.
Private providers complement government healthcare systems by handling a large patient load.	Healthcare may become a profit-making enterprise rather than a public service.
Achieves economies of scale by sharing services like ambulances, blood banks, and mortuaries between private and public hospitals, improving resource use.	Uninsured patients face high treatment costs in private facilities, leading to financial distress.
Ensures timely and better diagnosis through advanced medical devices, reducing regional disparities in healthcare services.	Private healthcare providers may avoid remote, tribal, and low-income regions due to limited profitability.
Promotes holistic medical growth by encouraging private investment in research, infrastructure, and job creation in the health sector.	Private healthcare providers often lack accountability to government regulatory bodies, leading to unchecked practices.

Medical and Wellness Tourism in India

Medical tourism (also called medical travel, health tourism or global healthcare) is a term used to describe the rapidly-growing practice of travelling across international borders to seek healthcare services. Services typically sought by travelers include elective procedures as well as complex surgeries, etc.

Current Status of the Sector

- **Global Standing:** India ranks **10th** in the **Medical Tourism Index (2020-21)** among 46 top destinations and **12th** among the world's top 20 wellness tourism markets.
- **Market Growth:** The sector was valued at approximately **USD 8.7 billion in 2025**, with projections reaching **USD 16.2 billion by 2030**.
- **Foreign Tourist Arrivals (FTAs):** According to PIB data for 2025, **507,244 foreign nationals** arrived specifically for medical treatment, accounting for roughly **5.5% of total FTAs** .
- **Top Source Markets:** Bangladesh is the largest contributor (3.25 lakh arrivals), followed by **Iraq, Uzbekistan, Somalia, and Oman**.
- **Regional Concentration:** Medical Value Travel (MVT) service providers are heavily concentrated in **Southern and Western states**, specifically in cities like **Delhi, Mumbai, Bangalore, and Chennai** (known as the Health Capital of India).

Drivers of Medical and Wellness Tourism

- **Cost Competitiveness:** Specialized treatments in India cost only **20%–30%** of what they do in Western markets; for instance, a heart bypass costing 100,000 USD *in the US is available for* 5,000 to 8000 USD in India.
- **Quality Infrastructure:** India boasts over **1,299 NABH accredited hospitals** (as of 2026) and several **JCI accredited** facilities, meeting global standards for clinical excellence.
- **Skilled Workforce:** Access to one of the world's largest pools of **English-speaking, globally trained doctors** (1.2 million registered) and health professionals.
- **The "Dual-Track" Advantage:** India uniquely blends modern tertiary care (**Allopathy**) with ancient healing systems (**AYUSH**—Ayurveda, Yoga, Naturopathy, Unani, Siddha, and Homeopathy).
- **Efficiency:** Unlike many Western nations, India offers **minimal to no waiting periods** for complex surgeries like liver transplants or oncology procedures.

Government Initiatives and Guidelines

- **Flagship Branding:** The **"Heal in India"** initiative positions the country as a premier destination for holistic healthcare, promoted as a sub-brand of "Incredible India".
- **Institutional Framework:** The **National Medical & Wellness Tourism Promotion Board (NMWTB)** serves as an umbrella body to coordinate between central ministries, states, and the private sector.
- **Union Budget 2026-27:** Proposals include establishing **five Regional Medical Hubs** (integrated complexes for care, education, and research) and the **Biopharma SHAKTI** outlay (₹10,000-crore) to lower treatment costs.
- **Digital Transformation:** The revamped **'Heal in India' Portal** acts as a "One-Stop Shop" for patients to find accredited hospitals, compare costs, and manage post-treatment care.
- **Visa Liberalization:** e-Medical and e-Medical Attendant visas are available for citizens of **172 countries**, alongside the newly introduced **Ayush Visa** for traditional healing.
- **Standards Adoption:** The Bureau of Indian Standards (BIS) has adopted **ISO 22525** to ensure MVT services meet international benchmarks.

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Challenges Associated

- **Data Fragmentation:** A lack of centralized mechanism to track state-wise or purpose-wise revenue hinders accurate economic planning.
- **Regulatory Gaps:** There is an absence of a uniform national framework for pricing, leading to cost variations and a potential trust deficit.
- **Insurance Barriers:** High-cost treatments often lack synchronization with **global insurance providers**, resulting in high out-of-pocket expenses compared to competitors like Thailand.
- **Unorganized Facilitators:** The presence of unregulated agents can lead to patient exploitation and unethical practices.
- **External Disruptions:** Geopolitical tensions, such as those in West Asia, have recently caused a decline in international patient footfall by up to **30%**.
- **Safety and Standards:** Ethical risks and occasional lapses in protocol can dent the sector's global image.
  - A tragic fire at an unregulated Malviya Nagar guest house exposed how budget medical tourist lodgings flout basic safety norms, heavily damaging India's reputation.

Recommendations for Way Forward

- **Mandatory Accreditation:** Establish a **National Accreditation Framework** for MVT facilitators to eliminate unregulated agents.
- **Narrative Shift:** Move the marketing strategy from "low-cost" to **"Clinical Excellence"** by highlighting high-end surgical outcomes and the **Ayush Quality Mark**.
- **Decentralization:** Rapidly operationalize the **Regional Medical Hubs** to reduce geographic concentration in the South and NCR.
- **Insurance Portability:** Collaborate with international Third-Party Administrators (TPAs) to enable **"Cashless" treatment** for foreign patients.
- **Medical Diplomacy:** Integrate healthcare services into bilateral trade agreements, particularly with **BIMSTEC, African, and CIS nations**.
- **Skill Augmentation:** Leverage the commitment to add **1 lakh Allied Health Professionals** over the next five years to bridge gaps in post-operative care

11. Digital Health

Digital health refers to the use of digital technologies and innovations to enhance healthcare delivery, improve health outcomes, and make health systems more efficient and accessible. It encompasses a broad range of tools such as electronic health records, telemedicine, wearable devices, artificial intelligence, and big data analytics to support personalized, equitable, and sustainable healthcare.

Significance of Digital Health	Concerns Related to Digital Health
• Interoperability & Continuity of Care: Enables seamless sharing of longitudinal health records across different levels of care (Primary to Tertiary). ♦ **Example:** The **Health Information Exchange (HIE)** under ABDM allows a patient in a rural clinic to share history with a city specialist instantly.	• Risk of unauthorised data access compromising patient privacy and leading to misuse of personal health information. ♦ **Example:** Despite the **Digital Personal Data Protection (DPDP) Act**, concerns remain regarding the "informed" nature of consent among non-literate populations.
• Provides a unique digital health ID for every citizen to streamline interactions with healthcare professionals. ♦ **Example:** The Ayushman Bharat Health Account (ABHA) has officially surpassed the 90 crore (900 million) registration mark, with over 100 crore medical records successfully linked across the network.	• **Algorithmic Bias** in AI models trained on skewed data may lead to discriminatory clinical outcomes for marginalized ethnicities or genders. ♦ **Example:** Global studies show AI diagnostic tools can underperform on darker skin tones if the training dataset is not diverse.
• Supports effective management of chronic diseases, a growing public health challenge. ♦ **Example:** The **Bhashini-integrated MANAS App** uses AI to provide mental health support in regional languages, bridging the treatment gap.	• Unequal access to digital tools and low digital literacy can exclude marginalised groups, especially in rural areas. ♦ **Example**: 80.5% of all men have used the internet, while only 58.6% of rural women have used the internet. (NFHS-6)

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- **Optimized Public Health Monitoring** using Real-time data allows for faster outbreak response and targeted resource allocation.
    - ♦ **Example:** The **U-WIN portal** (digitizing the Universal Immunization Programme) ensures no child misses a vaccine dose via automated reminders.
- Lack of trust in digital health platforms can hinder adoption, as seen in failed systems like the UK NHS digital project.

12. Government Schemes

Scheme	Description
Surakshit Matritva Aashwasan (SUMAN)	• Provide free, zero-expense, dignified, and high-quality maternal and newborn healthcare across all public health facilities. • The primary goal is to achieve zero preventable maternal and newborn deaths.
Pradhan Mantri Surakshit Matritva Abhiyan (PMSMA)	• Provides assured, comprehensive, and free antenatal care (ANC) to all pregnant women on the 9th of every month. • The e-PMSMA platform now allows real-time tracking of High-Risk Pregnancies (HRP) through red-sticker digital tagging.
Rashtriya Bal Swasthya Karyakram (RBSK 2.0)	• Screens children (0–18 years) for the "4 Ds": Defects, Diseases, Deficiencies, and Developmental delays. • In May 2026, RBSK 2.0 was launched to include lifestyle diseases (obesity, early diabetes) and mental health screening in the core framework. • It has introduced digital health cards, real-time data systems, and integrated platforms for tracking, monitoring and service delivery.
Mission Indradhanush	• The ultimate goal of the initiative is to achieve more than 90% full immunization coverage across the country. • As of January 2026, full immunization coverage reached 98.4%. A nationwide HPV vaccination campaign for girls (age 14) was added in early 2026 to prevent cervical cancer.
National Vector Borne Disease Control Programme (NVBDCP)	• Umbrella program for Malaria, Dengue, Kala-Azar, etc. • India has achieved a 97% reduction in Kala-Azar cases and is on track for the 2027 Malaria elimination goal.
Ayushman Vay Vandana	• **New Initiative:** Expanded to cover **all senior citizens aged 70+** regardless of income. • **Feature:** Provides a distinct **Ayushman Vay Vandana Card** with a ₹5 lakh top-up cover per year for seniors (not shared with the rest of the family).

13. Way Forward

Way Forward	Explanation & Strategic Priority
Implement NITI Aayog Action Plan	Increase expenditure to 2.5% of GDP (currently ~1.97%). Shift from predominantly curative care to preventive care via Ayushman Arogya Mandirs (AAM).
National Commission for Healthcare Cost Management (Proposed)	A proposed regulatory body to monitor hospital pricing and the efficiency of public insurance payouts to prevent medical inflation.
Health-Education-Livelihood (HEL) Campuses	Establishing integrated rural campuses that combine community health centers with Skill Development Centers for health-tech incubators.
Mental Wealth Initiative	Reframing mental health as an economic asset. Integration of Tele-MANAS with corporate and school curricula to reduce the 83% treatment gap.
Pharma-to-Plate Integrity Chain	Using Blockchain technology to track pharmaceuticals from raw material to patient, combating the counterfeit drug market—a priority for India's global pharma reputation.
Ayush in Action	Promoting "Integrative Medicine" by placing AYUSH practitioners in allopathic settings to foster holistic wellness and prevent NCDs.

The ancient shloka "Sarve Bhavantu Sukhinah, Sarve Santu Niramayah" (May all be happy, may all be free from disease) embodies the ideal of universal well-being and health. This vision aligns with the United Nations Sustainable Development Goal (SDG) 3, which strives to ensure healthy lives and promote well-being for all at all ages through universal health coverage, equitable access to quality healthcare, and financial risk protection.

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10 Education

Education in India, a **pillar of social justice and economic growth**, is influenced by constitutional mandates and policies such as NEP 2020. According to **PLFS 2023-24**, India's literacy rate was 80.9%, with males at 87.2% and females at 74.6%. This reflects progress but **persistent gender and access gaps**, which need to be addressed through **active reforms and community participation.**

UPSC Mains PYQs

1. What are the aims and objectives of recently passed and enforced, The Public Examination (Prevention of Unfair Means) Act, 2024? Whether University/State Education Board examinations, too, are covered under the Act?**(2024)**
2. Has digital illiteracy, particularly in rural areas, coupled with lack of Information and Communication Technology(ICT) accessibility hindered socio-economic development? Examine with justification.**(2021)**
3. Professor Amartya Sen has advocated important reforms in the realms of primary education and primary health care. What are your suggestions to improve their status and performance?**(2016)**
4. The quality of higher education in India requires major improvements to make it internationally competitive. Do you think that the entry of foreign educational institutions would help improve the quality of higher and technical education in the country? Discuss. **(2015)**
5. Should the premier institutes like IITs/IIMs be allowed to retain premier status, allowed more academic independence in designing courses and also decide mode/criteria of selection of students. Discuss in light of the growing challenges. **(2014)**

1. Constitutional Provisions

Aspect	Description
Fundamental Rights	Article **21A** mandates free, compulsory **education** for ages 6-14. Article **28** ensures freedom from religious instruction. Article **30** allows minorities to manage **institutions**.
Directive Principles	• **Article 45** now mandates early childhood care and education for children **below six years** following the 86th Constitutional Amendment. • **Article 46** emphasises the promotion of educational and economic interests of the weaker sections, especially SCs and STs.
Fundamental Duties	Article **51A(k)** requires parents to provide **education** for ages 6-14, emphasising social duty.

2. Facts on Education in India

Indicator	Data
Gross Enrollment Ratio (GER) (UDISE+) 2024-25	• Gross Enrolment Ratio (GER) increased to 90.3% at the middle level and 68.5% at the secondary level. • Student retention improved, though secondary dropouts remain a challenge, with only 47.2% of students completing the higher secondary stage
Pupil–Teacher Ratio (PTR) (UDISE+) 2024-25	• The Pupil-Teacher Ratio (PTR) at the Foundational, Preparatory, Middle, and Secondary levels are now reported as 10, 13, 17, and 21 respectively, all significantly improved ratio qua NEP's recommended ratio of 1:30.

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Government Expenditure on Education *(Economic Survey 2024-25)*	• Public expenditure remains around **4.1% – 4.6% of GDP** • Below **NEP 2020 target: 6% of GDP**
Dropout Rates (School Education) (UDISE+) 2024-25	• At the Preparatory stage, the rate declined from 3.7% to 2.3%, at the Middle stage from 5.2% to 3.5%, and at the Secondary stage from 10.9% to 8.2% in comparison with the previous year.
School Teachers Strength *(UDISE+ 2024-25)*	• For the first time, India's total number of teachers crossed the 1 crore mark, with women making up 54.2% of the teaching workforce.
Infrastructure Access in Schools *(UDISE+ 2024-25)*	• Despite infrastructure growth, internet connectivity (53.7%) and computer access (57%) remain uneven, leaving almost half of schools without digital resources. • While ramps are available in 54.9% of schools, dedicated toilets for children with special needs (CWSN) are only present in 35.6%. • Basic facilities are nearly universal: drinking water is available in 99.3% of schools and electricity in 93.6%

3. Challenges in Indian Education

Challenge	Description	Example
Inadequate Funding	Education spending remains below NEP 2020 target (6% GDP), limiting infrastructure, teacher capacity and research quality.	Public expenditure ~4.1 - 4.6% of GDP despite increased Union Education Budget 2025-26.
Teacher Shortages	**Insufficient teachers**, poor training lead to substandard teaching; outdated curricula, methods **fail to foster critical thinking.**	Nearly 10 lakh sanctioned teaching posts remain vacant across government schools nationwide.
Traditional Methods	Rote learning persists; foundational literacy & numeracy remain major challenges despite NEP reforms.	ASER 2023: 43% of 14-18-year-olds can't read English sentences; 25% in rural areas struggle.
Privatization Issues	**Private institutions** (50%+ education) prioritize profit, compromise quality; **high fees exclude low-income students**, widen inequities.	In private schools, urban fees range from ₹26,188 (pre-primary) to ₹49,075 (higher secondary).
Infrastructure Gaps	**Inadequate facilities** (classrooms, labs, sanitation) in rural government schools limit learning environments, especially for girls.	*Only 53.9% of schools in India have access to the internet, with rural areas facing significant barriers to digital learning*
Socio-Economic Barriers	Poverty, caste and regional disparities lead to higher dropout rates and low higher-education participation.	Marginalised groups under-represented in higher education.
Gender Disparities	Despite improvements, gender disparities persist due to socio-cultural norms, early marriage, safety concerns, and economic barriers.	As per the PLFS 2023–24, India has a literacy gender gap of 12.6% points.

4. Impacts of Educational Challenges

Impact	Description	Example
Low Learning Outcomes	Weak foundational literacy & numeracy reduce productivity and later employability.	**ASER 2024:** Only 45.8%, Class 8 students could solve basic division; learning gaps persist post-COVID.
Economic Inequality	Unequal education access sustains inter-generational poverty and income disparity.	Educational disparities and out-of-school populations cost the global economy over $10 trillion annually.
Social Exclusion	**Marginalised communities** (SC/ST, girls, rural) face limited opportunities, reinforcing caste, gender, and regional divides.	UDISE+: The secondary school dropout rate peaks near ~10-12%, heavily impacting rural areas and vulnerable marginalized demographics.

5. Higher Education Ecosystem

India has one of the largest higher education systems in the world that stands second in terms of the higher education network. The term 'higher education' with respect to India denotes the tertiary level education that is imparted after 12 years of schooling (10 years of primary education and 2 years of secondary education). The entire higher education ecosystem in India comprises around 1000+ universities and 42,000+ colleges imparting exceptional education. All these institutions fall under the purview of the Ministry of Education.

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Regulatory Bodies

University Grants Commission (UGC)
SStatutory organization established via Parliament Act in 1956 overseeing higher education and maintaining standards.

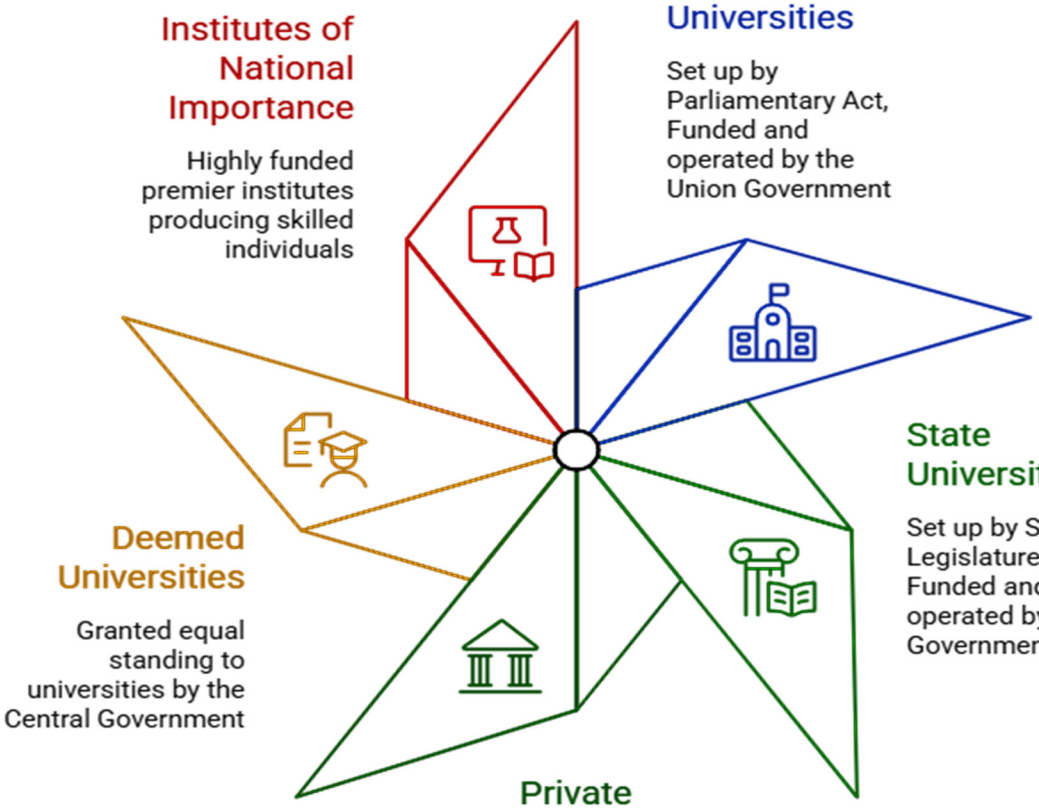
National Medical Commission (NMC)
Statutory body regulating medical education and ensuring quality professionals.

All India Council for Technical Education (AICTE)
National-level body under the AICTE Act, 1987 directing technical education and promoting development.

Bar Council of India (BCI)
Statutory body regulating legal education and representing the Indian Bar.

National Board of Accreditation (NBA)
Specialized unit under the AICTE evaluating technical and professional courses.

National Assessment and Accreditation Council (NAAC)
Autonomous council established by UGC assessing universities and colleges nationwide.



Institutes of National Importance
Highly funded premier institutes producing skilled individuals

Central Universities
Set up by Parliamentary Act, Funded and operated by the Union Government

Deemed Universities
Granted equal standing to universities by the Central Government

State Universities
Set up by State Legislature Act, Funded and operated by State Governments

Private Universities
Established by State Legislatures, often specialized

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Status of Higher Education in India

Theme	Status
Demographic Window	• India has a critical, time-bound window until **2047** to optimize its immense youth bulge for economic growth. • Median age is **28.2 years**; youth (15–29 years) absolute volume stands at **34.5 crore** individuals.
Institutional Transition	• The higher education ecosystem has transformed from an elite colonial model into a highly democratized **"massification"** network. • At the time of independence, India had only 17 universities and 636 colleges serving about 2.38 lakh students. ♦ Total Universities: 723 (2013–14) → 1,126 (2023–24) (+55.7% increase) • State Public Universities anchor absolute student volumes, but market-driven private universities are growing at the highest velocity. ♦ Government Universities: 504 (2013–14) → 733 (2023–24) (+45.4% increase) ♦ Private Universities: 219 (2013–14) → 546 (2023–24) (+149.3% increase)
Demographic Access	Total student enrollment has witnessed historic expansion since independence, establishing a deep foundation for future targets. • GER (Gross Enrolment Ratio): 23.7 (2014-15) → 30.0 (2023-24) (+26.6% increase) • Female GER: 22.9 (2014-15) → 31.2 (2023-24) (+36.2% increase) • SC Enrolment: 46.07 lakh (2014-15) → 69.72 lakh (2023-24) (+51.4% increase) • ST Enrolment: 16.41 lakh (2014-15) → 28.83 lakh (2023-24) (+75.7% increase) • OBC Enrolment: 1.13 crore (2014-15) → 1.80 crore (2023-24) (+60.2% increase)

AISHE Report 2023-24

CHART 1: This chart shows the overall enrolment and enrolment in undergraduate (UG) courses. UG enrolments declined for the first time in 2023-24. *Value within brackets is GER*

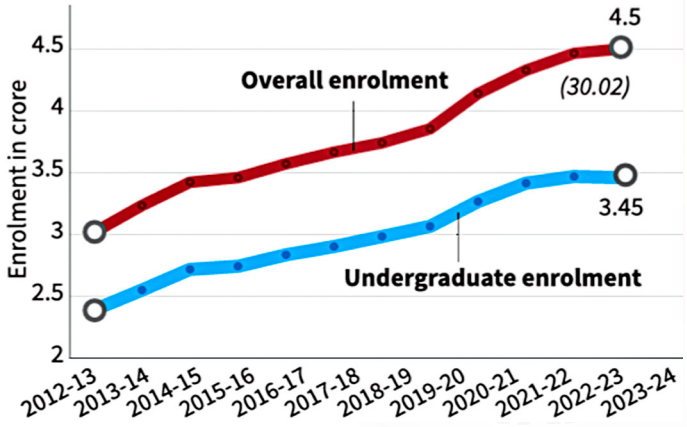
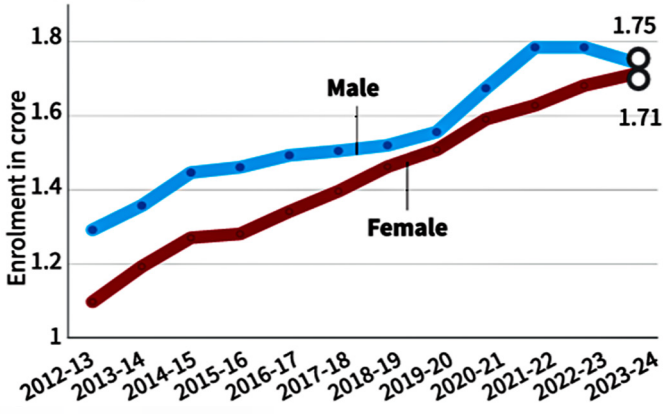


CHART 2: The gender-wise number of students enrolled in UG courses show that the decline in 2023-24 was noticed only among male students



Indicator	Key Highlights
Gross Enrolment Ratio (GER)	• **Overall GER (18–23 years): 30.0%**, up from 29.5% in 2022–23. • Enrollments in **Undergraduate courses** (75% of all enrolments), declined by 93,000 (0.27%) between 2022-23 and 2023-24 (despite increase in eligible youth population). • *NEP 2020 Target:* ***50% GER by 2035***.
Gender Inclusion	• **Gender Parity Index (GPI): 1.08**, remaining above 1.0 for the **seventh consecutive year**. • **Female GER: 31.2%**, exceeding **Male GER: 28.9%**, indicating sustained gender parity.
Social Inclusion	• **SC GER: 27.8%** and **ST GER: 22.8%**, reflecting continued improvement in access. • Enrolment of SC, ST and OBC students has steadily increased over the past decade.
STEM Education	• **Over 1 crore students** enrolled in STEM disciplines. • **Women constitute about 44% of STEM enrolment**, among the highest globally.
Higher Education Institutions (HEIs)	• India has **over 64,700 Higher Education Institutions**, including universities, colleges and standalone institutions. • AISHE achieved **over 90% institutional coverage**, improving data reliability.

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Faculty Strength	• **Faculty strength exceeded 16 lakh**, with women accounting for **about 44%** of teachers. • Female representation continues to improve across higher education institutions.
Research & Quality	• Doctoral enrolment continues to rise, reflecting expanding research capacity. • However, **GER remains well below the NEP 2020 target of 50% by 2035**, highlighting the need for further expansion and quality enhancement.

Issues and Challenges in Higher Education in India

Category	Description
Structural Massification vs. Quality Disparities	• **Geographic Inequality:** University density per 1 lakh eligible population is highly unequal, peaking at 10.3 in Sikkim while dropping to a low of 0.2 in Bihar. • **College Density Imbalances:** High concentration exists in states like Karnataka (66 colleges per lakh population), contrasted sharply by severe deficits in Bihar, Jharkhand, and Manipur. • **Regulatory Fragmentation:** The 2009 Prof. Yash Pal Committee highlighted operational inefficiencies caused by colleges juggling overlapping regulations from multiple bodies like the AICTE and UGC.
Demographic Dividend, Access, and Gender Parity	• **State-Level GER Disparities (for 2021-22):** Territory-specific GER varies significantly, led by Chandigarh at 64.8% and Tamil Nadu at 47.9%, while states like Bihar, Jharkhand, and Chhattisgarh register low GERs despite doubling their figures in the last decade. • **Gender Parity Progress:** The national Gender Parity Index (GPI) grew by ~16% over a decade, moving from 0.87 (2011-12) to 1.01 (2021-22). States like Kerala (1.44), Chhattisgarh, and Himachal Pradesh showcase higher female enrollment, whereas Odisha (0.88) and Tripura (0.89) trail behind.
Faculty Recruitment Dynamics and Pedagogical Health	• **Hierarchical Imbalance:** Out of ~16 lakh higher education teachers, a steep 68% are concentrated in junior roles as Lecturers/Assistant Professors, while Professors make up just 9.5%, and Temporary Teachers account for 5.7%. • **Stagnant PTR:** The national Pupil Teacher Ratio (PTR) has remained frozen at 23 over the last 4-5 years. • **Regional Discrepancies:** Tamil Nadu maintains an optimal PTR of 14, followed by Goa, Karnataka, Kerala, and Punjab at 15. In contrast, lagging states like Bihar, Jharkhand, Madhya Pradesh, and Chhattisgarh have seen their PTR worsen due to swelling student enrollment unaccompanied by faculty recruitment.
R&D Under-Resourcing and Innovation Ecosystems	• **Research Monopolization:** Higher education contributes just 10% of India's total research output, Elite institutions dominate outputs; 16 IITs command 24.3% of research publications and Private Deemed Universities hold 23.5%. Conversely, State Public Universities (SPUs)—despite having 23 institutions in the top 100—contribute a minor 14.7%. • **Geographic Clustering of Innovation:** To boost entrepreneurship, 14,269 Institution's Innovation Councils (IICs) were set up by September 2024, incubating 4,969 startups. However, 83.3% of these IICs are concentrated in just 10 states, led by Maharashtra, Tamil Nadu, Karnataka, and Uttar Pradesh.
Macro-Financing Gaps and Investment Targets	• **Target vs. Actual Spending:** While the Kothari Commission (1964-66), National Policy on Education (1968), and NEP (2020) all targeted public education spending at 6% of GDP, actual expenditure is a fraction of this. • **Sectoral Spending Deficits:** Dedicated **1.57% of GDP** strictly to tertiary education (2021). • **Funding Inequity:** 57 Central Universities receive disproportionately higher central funding, while the 482 SPUs—which educate over 80% of the student population—face severe resource constraints hindering infrastructure and research.

Comparative Global Analysis of Indian Higher Education

Dimension	India's Status vs. Global Ecosystem
System Scale & Enrollment	• **Third-Largest System:** Ranked 3rd globally by student enrollment, trailing only China and the United States.
Global University Rankings	• **Regional Strength, Global Lag:** Leads heavily in regional assessments (e.g., QS Asia University rankings) but trails Western and East Asian peers in top-tier global metrics like the QS World Rankings and Times Higher Education. Only IIT Delhi and IIT Bombay secure positions within the global top 150 in QS. • **Resource Gap:** Lacks the massive endowments, elite faculty-to-student ratios, and extensive research outputs characteristic of global leaders like MIT, Oxford, or Stanford.

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Research & Development (R&D)	• **Volume vs. Output Dominance:** Achieved a major increase in global research publication volume, yet global patent filings and high-volume international collaborations remain heavily dominated by the US, China, and the EU.
Student Mobility & Exchange	• **Net Exporter Profile:** Historically serves as a net exporter of students rather than an international hub (like the US, UK, or Australia), leading to large outward financial remittances.
Key Challenges	• **Pedagogical Health:** India's average faculty-student ratio stands at 1:23, falling short of the global benchmark of 1:16. • **Employability Gap:** Outdated curricula and systemic gaps in vocational training limit the employability rate of certain technical graduates despite high institutional graduation numbers. ***India Skills Report 2026:*** *Only ~56.35% youth considered employable.* • **Funding Concentration:** High-level research is disproportionately clustered in a few elite institutions, leaving state universities with severe research underfunding.

Way Forward

The following table summarizes the core recommendations and performance indicators from **NITI Aayog's "Policy Recommendations for Transforming Higher Education in India" report**. It condenses nearly 80 actionable points across four primary pillars designed to elevate India's academic landscape:

Thematic Area	Key Recommendations
I. Quality	• Establish the Anusandhan National Research Foundation (ANRF). • Restructure undergraduate curriculum to ensure 25% multi-disciplinary mobility. • Expand the Academic Bank of Credits (ABC) and One Nation One Subscription (ONOS).
II. Funding & Financing	• Prioritize capital and operational funding for State Public Universities (SPUs). • Progressively increase public education spending toward 6% of GDP. • Cultivate alumni endowments, corporate social responsibility (CSR) grants, and fee autonomy.
III. Governance	• Centralize regulation under a single umbrella like the Higher Education Commission of India (HECI). • Progressively phase out the massive college affiliation system. • Fill permanent faculty vacancies and mandate continuous pedagogical training.
IV. Employability	• Integrate vocational training and 21st-century skills into mainstream academic streams. • Deploy the National Credit Framework (NCrF) for skill-based credit accumulation. • Mandate structured corporate internships and scale up Institution's Innovation Councils (IICs).

6. Digital Education in India

Digital Education refers to technology-enabled teaching-learning systems using the internet, ICT tools, and digital platforms to deliver education beyond traditional physical classrooms. It marks a transition from conventional "chalk and talk" methods to personalized, independent, and blended learning models (e.g., flipped classrooms).

Drivers of Digital Education in India

- **Government Initiatives & Policy Backing:** Structural support from the *Digital India Campaign (2015)* and the *National Education Policy (NEP) 2020*, which formally integrates technology as a cornerstone for equity and quality.
- **The COVID-19 Catalyst:** The pandemic transformed online learning from an optional alternative to a baseline structural necessity.
- **Demographic & Market Penetration:** Mass availability of low-cost smartphones coupled with expanding internet connectivity. Internet users grew from 251.59 million in 2014 to 954.40 million in 2024.
- **Public Infrastructure Scale-Up:** High mobile-network rollout—with 95.15% of Indian villages covered by 3G/4G connectivity by 2024.
- **Indigenous Platform Ecosystems:** Development of free, open-access public platforms (e.g., DIKSHA, SWAYAM) reducing complete reliance on expensive private tools. SWAYAM hosts thousands of free MOOCs designed by top IIT and IIM faculty.

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Benefits of Digital Education

- **Democratization and Access:** Overcomes rigid geographical barriers, permitting students in remote or rural locations to learn from top-tier academic institutes.
- **Flexibility and Pacing:** Encourages asynchronous learning; students and working professionals can learn at their own pace and time.
- **Cost Efficiency:** Drops the operational overhead of physical capital infrastructure, long-distance student travel, and printed textbooks.
- **Skill Orientation & Alignment:** Seamlessly incorporates 21st-century vocational and technological skills directly into interactive courses.
- **Teacher Empowerment:** Offers digital training modules and curriculum-aligned, standardized open resources to upgrade teaching standards.

Challenges associated

- **The Digital Divide (Divided Realities):** Deep disparities between urban-rich and rural-poor learners. *TRAI 2026:* 59.63% of rural households have internet access.
- **Severe Institutional Deficits:** Out of 14.7 lakh schools in India, **only 32.4% have functional computers**, and **just 24.4% host smart classrooms**. The gap is heavily skewed against state and rural schools.
- **Digital Illiteracy:** As per the National Sample Survey (NSS 78th Round 2020-21), computer literacy among individuals aged 15+ is a mere **24.7%**.
- **Gender and Social Inequalities:** Girls face a disproportionate disadvantage in device ownership and internet access within low-income families.
- **Commercialization and Creeping Privatization:** Over the past decade, government schools shrank by 8% while private schools grew by 14%. This turns quality digital education into a structural privilege rather than a primary right.

Key Government Initiatives

- **PRAGYATA Guidelines:** The Ministry of Education's direct policy framework setting operating guidelines for online and blended learning.
- **PMGDISHA & NDLM:** The *Pradhan Mantri Gramin Digital Saksharta Abhiyan* and *National Digital Literacy Mission*, targeting structural digital literacy at the household level.
- **Swami Vivekananda Youth Empowerment Scheme:** A localized state-level enforcement example (UP Government) distributing over 46.75 lakh smart devices to close the hardware gap.
- **PM eVidya & SWAYAM Prabha:** Multimodal frameworks deploying 34 dedicated DTH television channels to reach students lacking stable internet

Way Forward

- **Accelerate Rural Infrastructure (BharatNet):** Expeditiously achieve the BharatNet target of connecting 2.5 lakh Gram Panchayats with 100 Mbps broadband to stabilize rural e-learning.
- **Scale Up Educational Funding:** Enforce the NEP 2020 target of ramping up overall education expenditure to **6% of GDP** to structurally upgrade hardware in state schools.
- **Hardware Equalization:** Deploy public-private partnerships (PPP) and community centers (like *Common Service Centres - CSCs*) to provide free hardware access points to vulnerable socio-economic groups.
- **Targeted Faculty Training:** Launch continuous, large-scale digital capacity-building workshops for teachers to prevent a drop-off in pedagogical quality.
- **Mainstream Blended Learning Architecture:** Transition to long-term "online-offline hybrid models" utilizing advanced adaptive learning technologies and AI to personalize outcomes safely.

7. Emerging Issues and Trends

Competitive Exam Paper Leaks

Competitive exam paper leaks are the unauthorized disclosure of question papers before an exam, often for financial gain by organized networks.

Reasons for Paper Leaks

- **Systemic Corruption:** Organized syndicates bribe or compromise insiders with legitimate access to secure exam documents. *Example:* **The 2024 NEET-UG leak involving multi-state syndicates bribing insiders.**
- **Flawed Physical Security:** Weak control protocols during the printing, transit, and storage phases of the question paper lifecycle. *Example:* NEET 2026 paper handwritten by insider, scanned and converted to PDF for mass distribution.
- **Digital & Cyber Vulnerabilities:** Malicious actors exploit weak IT infrastructure, unencrypted digital storage, and insecure online testing platforms. *Example:* **The UGC-NET 2024 leak, where question papers were traded on the dark web.**
- **Rapid Digital Dissemination:** Encrypted messaging applications and social media platforms scale the reach of leaked materials nationwide within minutes. *Example*: 2026 NEET paper circulated via Telegram and WhatsApp 15-30 days before May 3 exam across 23 lakh students
- **High-Stakes Pressure:** A hyper-competitive educational climate creates immense pressure to perform, fueling a lucrative black market for academic shortcuts. *Example:* IBPS Clerk 2025 solver gang using AI face-matching for ₹10 lakh per candidate across 10 states

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Potential Implications on the Education & Socio-Economic System

- **Erosion of Meritocracy:** Pre-accessed materials decouple exam performance from actual knowledge, directly penalizing honest, hard-working students.
- **Loss of Institutional Trust:** Recurrent, high-profile leaks degrade public confidence in the fairness and transparency of testing bodies. **Example***:* Mass student protests following the 2026 NEET-UG irregularities.
- **Severe Financial Burden:** Exam cancellations incur massive logistical costs (reprinting, venue booking, staff re-employment) while forcing families to bear extended coaching expenses.
- **Psychological Distress on Youth:** Unpredictable cancellations, delays, and rescheduling trigger severe anxiety, depression, and mental exhaustion among aspirants, prompting many to commit suicide.
- **Decline in Human Capital Quality:** Compromised testing allows underqualified individuals to pass, causing a long-term drop in workplace competency and macroeconomic productivity.
- **Amplified Socio-Economic Inequality:** Illicit paper sales act as a wealth filter, granting affluent students an unfair advantage and halting social mobility for lower-income peers.

Initiatives Taken by the Government

- **Enhanced Surveillance Frameworks:** Deploying CCTV cameras at printing, transit, and storage points to tightly log physical handling.
- **Technological Interventions:** Use of Aadhar & biometric authentication systems, to explicitly prevent candidate impersonation.
- **Targeted Administrative Crackdowns:** Mobilizing specialized law enforcement wings (e.g., Economic Offences Units) to dismantle cheating syndicates and arrest masterminds.
- **Enactment of the Public Examinations (Prevention of Unfair Means) Act, 2024:** Parliament passed this landmark legislation specifically to curb paper leaks, organized syndicates, and administrative malpractices in major central exams.
- **Uncompromising Deterrent Sanctions:** Imposing severe criminal, financial, and legal penalties on racketeers, complicit officials, and cheating candidates.

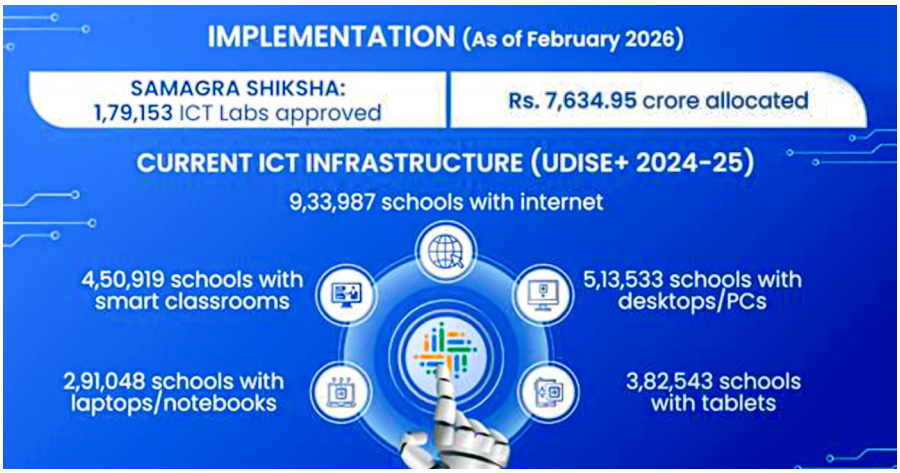
Way Forward

- **Secure Digital Examination Transition:** Phasing out vulnerable physical papers in favor of highly monitored computer-based tests with real-time network tracking.
- **Blockchain Integration:** Utilizing decentralized blockchain architecture to establish an immutable, transparent, and completely tamper-proof electronic audit trail for exam delivery.
- **AI Proctoring & Cybersecurity:** Constructing regulated digital testing environments complete with browser lockdowns and AI software to flag anomalous behavior instantly.
- **Mandatory Security Audits:** For operating personnel alongside independent security compliance audits.
- **Values-Based Education & Advocacy:** Integrating ethics and individual accountability into core curricula while running national campaigns to reduce societal tolerance for cheating.

AI in Education

Drivers of AI in Education

- **Scale Overload:** Navigating a massive load of **250M+ students** (*MoE, 2023*) against crowded classrooms of **50–60 students per class**.
- **Systemic Leakages:** Combatting a high **14.1% secondary dropout rate**
- **Teacher Administrative Fatigue:** High volumes of non-teaching administrative chores (grading, attendance tracking) pulling teachers away from instruction.
- **Infrastructure Maturation:** A projected **70% rural internet penetration by 2030** (*TRAI, 2023*) via 5G rollout, creating a viable deployment baseline.



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Significance of AI in Education

- **Pedagogical Overhaul:** Shifts India's educational framework from legacy rote learning to continuous, formative & hollistic evaluations.
- **Democratic & Equitable Access:** Delivers high-grade, cost-effective interactive content 24/7 to underserved rural pockets via **DIKSHA** and **PM e-VIDYA**.
- **Bridging Language Barriers:** AI acts as a critical translator, allowing high-quality materials to be instantly converted into regional languages to fulfill localized learning needs via national AI platforms like BHASHINI and AICTE's Anuvadini.
- **Data-Driven Governance:** Powers macro-level planning platforms (**UDISE+ / SDMS**) to predict dropouts and map school-wise allocations precisely.
- **Quantifiable Efficiency:** Converts passive classrooms into active spaces, yielding an empirical **15% bump in student test scores** (*IIT Bombay Study, 2022*).

Government Initiatives

- **NEP 2020 Directives:** Mandates advanced technology integration to assure equitable learning, AI-driven school governance, and continuous holistic evaluation.
- **DIKSHA Platform:** Serves as the national digital foundation, incorporating multilingual AI tools to provide free, high-quality regional language curricula (*MHRD, 2020)*.
- **SWAYAM Portal:** Uses AI analytics to drive personalized course recommendations and performance metrics for higher education.
- **UDISE+ & SDMS Integration:** Leverages the automated Student Data Management System (SDMS) to optimize macro planning and tackle dropout indicators.
- **AI Centres of Excellence (CoE):** Set up via the Union Budget to innovate indigenous educational AI tools, emphasizing infrastructure parity for public schools.

Advantages of AI in Education

- **Hyper-Personalization:** Tailors difficulty, pacing, and exercises by analyzing student performance in real time.
  - ♦ **Example**: China uses **Squirrel AI** to customize homework and lessons for millions of students simultaneously.
- **Teacher Efficiency & Automation:** Automates grading, attendance, and lesson planning, shifting educators from rigid instructors to active facilitators.
- **Early Diagnostic Intervention:** Flags learning gaps and cognitive development vulnerabilities early in the educational cycle.
  - ♦ **Example: Finland** deploys privacy-protected AI to diagnose early learning anomalies like dyslexia.
- **24/7 Inclusive Support:** Provides round-the-clock virtual tutoring, enabling remote or working students to pursue self-paced learning.
- **Quantifiable Academic Gains:** Validated by an IIT Bombay study showing a **15% bump in test scores** via personalized platforms over legacy methods.

Challenges & Vulnerabilities

- **Deep Digital Divide:** Critical infrastructure asymmetry persists. 59.63% of rural households have internet access (TRAI 2026).
- **Severe Privacy & Surveillance Risks:** Pervasive tracking via facial recognition or keystroke logging creates algorithmic surveillance creep.
- **Data Vulnerabilities:** Commercialization risks via illicit third-party advertiser sharing and data breaches, such as a **1.2 million student profile leak in 2023** (*Cybersecurity India*).
- **Consent Illiteracy:** True parental opt-in is heavily compromised. A CIS survey revealed **68% of parents are unaware** of how EdTech apps use child data (*CIS, 2022*).
- **Teacher Inertia:** Systemic resistance due to poor digital literacy and legal anxieties surrounding compliance with the DPDPA 2023 (*Kumar, 2023*).

Way Forward

- **Strict DPDPA Enforcement:** Eliminate the regulatory grey zone by standardizing strict, child-centric data protection rules for schools and EdTech firms.
- **Democratize Technology:** Scale assistive AI and digital infrastructure to remote/tribal areas to bridge the digital divide, aligning with the IndiaAI Mission.
- **Infrastructure Parity:** Reallocate capital to equip under-resourced rural schools with foundational hardware like projectors and tablets, preventing an urban-elite monopoly on advanced tools
- **Continuous Teacher Upskilling:** Build mandatory, AI-centric capacity building modules directly into DIKSHA and SWAYAM platforms.
- **Ethics-by-Design:** Mandate privacy-first frameworks by scaling the IndiaAI Mission's **"Safe & Trusted AI"** projects, such as IIT Delhi's *Privacy-Preserving Models* and IIIT Delhi's *AI Ethical Certification Framework,* to eliminate invasive surveillance creep.

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Internationalization of Higher Education

The higher education system in India is the world's second-largest by institutional count— 58,000 institutions and 4.33 crore students, & a GER of 28.4 per cent. It produces the world's second-largest outflow of internationally mobile students (MEA: 13.36 lakh in 2024) according to MEA Rajya Sabha data) and it aspires, under NEP 2020, to become a global hub for higher education. The evidence, however, reveals a structural paradox: India is simultaneously one of the largest exporters and one of the smallest importers of higher education.

Potential of Internationalisation of Higher Education for India

- **Substantial Domestic Base:** With **97% of Indian higher education students continuing to study domestically**, there is a massive internal engine to cultivate a globally integrated workforce without relying purely on outward migration.
- **GIFT City / Mainland Twin-Regulatory Models:** The establishment of fully autonomous regulatory special economic zones (IFSCA Regulations 2022) alongside mainland liberties (UGC Regulations 2023) has already successfully institutionalized international branches (e.g., Australia's Deakin and Wollongong Universities) on Indian soil.
- **Aspirational Scalability Metrics:** In accordance with the mandates of the NEP 2020, it is estimated that targeted policy implementation can scale international student enrollment in India up to **1.5 lakh by 2030 and 7.89 lakh by 2047**.

Significance

- **Achieving NEP 2020 Strategic Objectives:** Internationalisation acts as the cornerstone to elevate domestic academic quality, boost knowledge exchange, drive indigenous innovation, and propel Indian universities into elite global rankings.
- **Fostering "Internationalisation at Home":** It creates a globally integrated ecosystem domestically , ensuring that the vast majority of local students who do not travel abroad acquire high-quality, globally benchmarked competencies.
- **Bridging the Macro Mobility Deficit:** Balancing the stark structural asymmetry between inbound and outbound numbers helps India retain premium local talent while actively attracting diverse international mindshare.

Challenges

- **Geographic Equity Deficits:** Branch campus permissions are heavily localized inside GIFT City or major Tier-1 metro pockets, entirely bypassing Tier-2 and Tier-3 youth who confront the steepest educational access barriers.
- **Severe Affordability Barriers:** With annual tuition costs running between **₹16–25 lakh**, foreign branch campuses remain structurally unaffordable for the overwhelming majority of average middle-income Indian families.
- **Absent Tier-1 Research Institutional Interest:** No top-50 global research institutions or Ivy League universities have committed to brick-and-mortar operations in India, limiting any immediate reputational rub-off on the local research paradigm.
- **Missing Domestic Post-Study Visa Pathway:** Unlike international travel destinations, home-country employment and post-study work paths do not replicate domestically for students completing degrees on Indian soil.
- **The Domestic Capacity Paradox:**
  - **Faculty Vacancies:** Central HEIs suffer from a **28.56% overall faculty shortage**, while senior professor-level vacancies stand at an alarming **56.18%**, deeply crippling indigenous research capacity.
  - **Research Sub-Scale Underinvestment:** At just **0.64% of GDP**, India's Gross Expenditure on R&D (GERD) sits at one-third of the global average, heavily constrained by direct state fiscal limitations.
  - **Poor Employability Footprints:** The *India Skills Report 2025* highlights that only **55% of Indian graduates are industry-employable**, creating severe credibility constraints amid an overall graduate unemployment landscape.

Recommendations as per NITI Aayog's Internationalisation of Higher Education in India

Strategy	• **National Implementation Roadmap:** Establish an Inter-Ministerial Task Force to deploy systemic funding, launch a single-window *National Foreign Degree Equivalence Portal*, and track progress via real-time digital dashboards. • **Ecosystem-Driven Academic Hubs:** Cluster regional innovation networks around core STEAM tracks, replicating the GIFT City economic blueprint to integrate with *Startup India* and *Make in India*.
Regulation	• **Mobility Simplification:** Digitize outbound analytics, mandate a *Verified National Agent Registry*, offer faculty tax moratoria, and drastically reduce e-FRRO visa processing turnaround windows. • **Liberalized Onshore Entry:** Relax the strict Top 500 ranking cap using expert review panels, authorize cross-border e-signatures, and offer brownfield "Campus-within-a-Campus" options with a 10-year sunset clause. • **Global Ranking & Regional Alignments:** Add internationalization metrics to the NIRF index. Launch the **'Tagore Framework'** (Erasmus+ model) to anchor regional mobility across BIMSTEC, ASEAN, and BRICS blocks.

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Finance	• **Sovereign Impact Fund:** Launch **'Bharat Vidya Kosh'**, a **USD 10 billion** state-diaspora matched trust fund to bankroll global AI research, overseas Indian campuses, and Tier-2/3 infrastructure upgrades. • **Flagship Mobility Incentives:** Operationalize the **'Vishwa Bandhu Scholarship'** for inbound master's cohorts and the **'Vishwa Bandhu Fellowship'** offering international talent 5-year renewable, globally matched tenure tracks. • **Institutional Resource Tapping:** Mandate top central and state HEIs to establish dedicated internal research offices to directly leverage multilateral joint funding pools from international academic bodies.
Branding, Communication, and Outreach	• **Targeted Regional Corridors:** Build country-specific marketing campaigns emphasizing clear credit-transfer loops (e.g., 1+1 or 2+2 programs) while ensuring rapid banking and onboarding access for inbound international students.

Madarsa Education in India

Madarsa education in India comprises a dual-structured framework of traditional Islamic learning and modernized institutions that integrate theology with secular streams like mathematics, computing, and English. Overseen by over 20 state boards and backed by constitutional minority rights under Article 30(1), this system serves millions of socio-economically marginalized Muslim students, acting as a crucial grassroots pipeline for localized academic and cultural access.

Significance of Madarsa Education in India

- **Preserving Minority Educational Rights:** Safeguards minority choices under Article 30(1), acting as primary hubs for community identity, classical languages (Arabic, Persian), and theological legacy transmission.
- **Grassroots Educational Access:** Serves as a vital accessible pipeline for socio-economically marginalized and rural Muslim cohorts, preventing complete exclusion from basic academic circles.
- **Fulfilling Sachar Committee Mandates:** Connects historically underrepresented groups to structured institutional learning. Over 4% of Muslim school-age children attend madarsas, addressing acute localized literacy deficits.

Recent Developments

- **Uttarakhand Abolishing Madarsa Board (2026):** The state dissolved its Madarsa Board, creating the Uttarakhand State Minority Education Authority (USMEA) to mainstream 40,000+ students across 452 registered madarsas.
- **UP Board of Madarsa Education Act, 2004:** The SC (November 2024) partly upheld the Act, overturning the Allahabad HC's total ban. It preserved basic secondary education but struck down higher-tier madarsa degrees for violating the UGC Act, 1956, declaring that madarsas cannot autonomously award higher university-equivalent degrees.

Challenges Facing Madarsa Education

- **Absence of Modern Secular Curricula:** Overemphasis on foundational Islamic theology restricts deep foundational exposure to core modern streams like mathematics, computing, and social sciences.
- **Severe Employment Disadvantage:** Broad non-recognition of traditional certificate nomenclature (*Munshi, Moulvi*) across regular administrative recruitments drives severe unemployment traps for graduates.
- **Infrastructural & Pedagogical Deficits:** Suffers from sub-scale teaching allocations, severe resource constraints, and an absence of formally trained modern subject educators.
- **Exclusionary Child Rights Paradigms:** NCPCR assessments highlight severe structural gaps matching standards set under the **Right to Education (RTE) Act, 2009**, keeping students isolated from formal mainstream frameworks

Way Forward

- **Twin-Track Curricular Integration:** Scale uniform models requiring mandatory state-approved general educational texts during formal hours, keeping religious pedagogy for optional supplemental sessions.
- **Formal Certificate Equivalence:** Harmonize madarsa high school credentials with regional boards to grant formal legal eligibility for public sector career avenues.
- **Teacher Modernisation Missions:** Roll out specialized capacity building schemes to upskill madarsa instructional staff in digital literacies and STEM methodologies.
- **Targeted Welfare Integration:** Funnel institutional development capital through schemes like PM-JVK to improve libraries, sciences infrastructure, and digital computer setups

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8. Private sector

Status and Trends

- **Enrolment Share Across Tiers:** Private institutions cater to nearly half of India's school-going demographic. In Higher Education, private providers represent **60% of total student enrolment** and constitute **nearly 78% of all professional colleges** nationwide.
- **The Private School Boom:** India is witnessing an unprecedented spike in low-fee, unaided private schools, which now serve as the dominant choice across both urban slums and rural hinterlands.
- **The Flight from Government Schools:** As per the report ***School Education System in India*** (May 2026) by NITI Aayog, long-term data shows that the absolute number of government schools declined from **11.07 lakh (73% share) in 2014-15 to 10.13 lakh (68.9% share) in 2024-25** due to state rationalization and consolidation. Conversely, private unaided recognized schools grew significantly from **2.88 lakh (19% share) to 3.39 lakh (23% share)** over the same decade, confirming a steady migration driven by a societal perception of superior structural quality.

Role and Significance of the Private Sector

- **Bypassing Public Capacity Bottlenecks:** Steps in where state capacity falls short, accommodating the student bulge in tertiary systems (*PRS Legislative Research*).
- **Superior Infrastructure & Operational Outcomes:** Delivers higher perceived infrastructure quality, stronger accountability, and better student learning metrics. ASER 2024 found that Grade III student reading fluency stood at 35.5% in private schools versus 23.4% in government setups
- **Market-Aligned Skill Delivery:** Private colleges & professional institutions, excel at dynamically aligning their technical and vocational curricula with real-world industry requirements. This bridges the traditional industry-academia employment gap much faster than public sector colleges.
- **Pioneering EdTech Innovations:** Private market entrants act as primary catalysts for instructional technology, deploying advanced digital platforms to scale custom learning feeds, remote test tracking, and continuous data evaluation.

Regulation of the Private Sector in Education

- **The Non-Profit Legal Imperative:** Grounded in socialist ideals, laws mandate education remain a philanthropic endeavor. Courts permit generating a reasonable surplus for institutional growth but legally forbid commercial profiteering (Unni Krishnan Case, 1993 & T.M.A. Pai Case, 2002).
- **The Right to Education (RTE) Mandate:** Section 12(1)(c) requires private unaided schools to reserve **25% of entry-level seats** for economically weaker sections (EWS) against state fee reimbursements.
- **Strict Operational Controls:** Private providers face multi-layered state interventions, including rigid fee ceilings, frequent bureaucratic compliance audits, and input-heavy land and infrastructure mandates.
- **Higher Education Regulatory Layer:** Dual oversight by the UGC and AICTE strictly governs institutional affiliations, infrastructural frameworks, and mandatory course validation requirements (*PRS Legislative Research*).

Opportunities

- **Corporate Social Responsibility Flows:** Under Section 135 of the Companies Act, private educational ecosystems can tap into corporate capital to build state-of-the-art public-private hybrid vocational training schools.
- **Systemic Resource Optimization:** Operates at a structurally lower per-student cost than the public sector, optimizing assets and filling access gaps in underserved regions.
- **Fiscal Freedom & Infrastructure Reinvestment:** Bypasses political and union constraints that lock **over 70% of public budgets** into salaries, allowing private entities to fluidly reinvest in facilities and learning tools.
- **Decentralized Operational Flexibility:** Avoids top-heavy, rigid bureaucratic structures, enabling localized management to dynamically deploy staff and adjust training based on immediate student needs.
- **Proven Fiscal Scalability (PPP Models):** Scales educational choice while reducing direct state financial burdens, as seen in successful voucher schemes like in **Andhra Pradesh**.

Challenges

- **Arbitrary Financial Caps:** Rigid state fee limits squeeze operating margins, preventing reinvestment in infrastructure, learning aids, or competitive wages. This creates extreme salary variances. Low-fee private school teachers face temporary contracts and low entitlements, triggering attrition and a "Low Professional Status" perception (NITI Aayog, 2026).
- **The Accountability vs. Accessibility Paradox:** High-end institutions remain unaffordable for low-income families, while accessible budget schools often compromise on learning standards to survive financially (UNESCO, 2024).
- **Burdensome Administrative Red Tape:** Multi-layered regulations cause paperwork gridlocks, deflecting management focus from instructional quality (*Centre for Civil Society*).
- **Inclusion Vulnerabilities:** Commercialized EdTech expansion widens the digital divide between wealthy urban cohorts and marginalized students. Low-income families suffer device and connectivity deficits, worsening geographic learning gaps during digital or pedagogical transitions (*UNESCO, 2024*).

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Way Forward	
Stakeholder	**Recommendation & Source**
Union & State Governments	• **Outcome-Based Auditing:** Shift state supervision away from input checklists to tracking actual, measurable student learning metrics (*Centre for Civil Society, WP 342*). • **Single-Window Clearance:** Streamline complex administrative licensing to eliminate long delays in securing annual fire and building safety certificates (*Centre for Civil Society, WP 342*). • **Risk-Based Oversight:** Adopt risk-focused state inspections to maximize regulatory enforcement resources specifically on chronically underperforming academic setups as done in the Netherlands (*UNESCO Etico*). • **National Higher Education Regulatory Council:** Set up an overarching council to replace multi-agency hurdles with simplified, single-window academic approvals (*NEP 2020*).
School Networks & Clusters	• **Integrated School Complexes:** Cluster public-private schools to share high-cost assets like laboratories, playgrounds, and specialized subject instructors (*NITI Aayog*). • **Industry-Aligned Skilling:** Partner with local enterprises to build functional career mentorships, vocational apprenticeships, and real-world training pathways (*NITI Aayog*).
School Management	• **DPI Infrastructure Integration:** Onboard student records onto unified national platforms like **SDMS** and **DIKSHA** to trace transitions and prevent dropouts (*NITI Aayog, 2026*). • **Financial Margin Relief:** Re-evaluate rigid fee caps to allow budget private setups to sustainably reinvest in facilities and competitive wages (*Centre for Civil Society*).
Teachers & Workforce	• **Workforce Incentive Reforms:** Launch performance-linked career advancement tracks to address low motivation, contractual insecurity, and temporary employment conditions (*NITI Aayog*).
Community & Local Bodies	• **Grassroots Governance:** Empower local parent-teacher associations to democratize day-to-day school decision-making and build bottom-up accountability mechanisms. • **Fiscal Inclusivity Incentives:** Provide targeted tax subsidies to private schools to actively accelerate entry-level enrollment from economically weaker sections.

9. Committees related to Education

Committees	Recommendations
Kasturirangan Committee (2019)	Recommended holistic & flexible education system forming basis of NEP 2020. Emphasised technology integration & digital learning ecosystem
Sam Pitroda Knowledge Commission (2005–09)	Suggested expansion of knowledge economy through institutional partnerships
Kothari Commission	Recommended common school system and equality in education opportunities.
National Curriculum Framework (NCF 2023)	Advocates holistic development and well-being centred education
National Policy on Education 1986 Committee	Highlighted teacher as central pillar of educational reform.

10. Way Forward

Strategy	Description
Policy Implementation	Effective execution of **NEP 2020** ensures multidisciplinary education, vocational training, and universal access, aligning with global standards.
Public-Private Partnerships	**Collaborations leverage private expertise**, public funding to enhance infrastructure, teacher training, technology, and reduce disparities.
Inclusive Policies	**Targeted schemes for girls, SC/ST**, and **disabled promote equity;** scholarships, safe infrastructure curb dropouts.
Digital Equity	Universal **internet access**, device provision, and bridging the digital divide, ensuring e-learning reaches rural, marginalised communities.
Mental Health Support	Counselling **services**, stress-free curricula address student well-being, integrating life skills for resilience, emotional health.

Transforming India's education system requires tackling chronic underfunding, systemic disparities, and pedagogical shortcomings through holistic, inclusive reforms. **Achieving Sustainable Development Goal 4 (Quality Education)** will **cultivate equitable, innovative, and resilient learning ecosystems**, advancing **social equity** and sustainable progress for all.

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11 National Education Policy 2020 and Three Language Formula

National Education Policy (NEP) 2020, announced on July 29, 2020, is a **transformative framework** aimed at **overhauling India's education system**. It **replaces the 1986 National Policy on Education** and aligns with the **2030 Agenda for Sustainable Development.** Built on the **pillars of Access, Equity, Quality, Affordability, and Accountability,** NEP 2020 seeks to make India a global knowledge superpower by fostering holistic, flexible, and multidisciplinary education.

Emphasising **India's linguistic diversity**, the policy promotes the **three-language formula**, encouraging the use of regional languages while **ensuring flexibility** and no imposition of any particular language on states or students.

UPSC Mains PYQs

1. National Education Policy 2020 is in conformity with Sustainable Development Goal-4 (2030). It intends to restructure and reorient the education system in India. Critically examine the statement. **(2020)**

1. Overview of NEP 2020

Aspect	Details
Background	• It was formulated by a committee led by **Dr. K Kasturirangan** aiming to transform India into a vibrant **knowledge society**. • NEP 2020 provides **Access, Equity, Quality, Affordability, Accountability**, aligned with **SDG 4** for inclusive and equitable education.
Vision	• **Holistic**, flexible, **multidisciplinary** education to bring out the **unique capabilities** of each student, with provisions like **multiple entry/exit** options in higher education.
Targets	• **100% Gross Enrolment Ratio (GER)** in preschool to secondary by 2030. • **50% GER** in higher education by 2035. • **Universal foundational literacy and numeracy** by 2025 through initiatives like **NIPUN Bharat**. • Replace rote-exam models with **360-degree Holistic Progress Cards** and the **PARAKH** assessment center.

2. School Education Reforms

Category	Details	Significance
Universal Access	• Targeted reintegration of millions of out-of-school children through open schooling, NIOS, vocational pathways, and community outreach.	• Broadens **educational reach**, significantly reducing **dropout rates** and fostering inclusivity across all levels.
Early Childhood Care and Education (ECCE)	• Replaces the rigid 10+2 system with a flexible 5+3+3+4 structure. It directly incorporates ages 3-8 into the foundational stage.	• Lays a **strong foundation** by prioritising **mental development** during the critical early years of 3-6.
Foundational Literacy and Numeracy	• **National Mission** on Foundational Literacy and Numeracy to ensure skills by **Grade 3** by 2025, supported by **NIPUN Bharat**.	• Strengthens **core skills**, ensuring every child achieves **proficiency** in reading and math.
Curriculum and Pedagogy	• Promotes **holistic development** with no rigid separation between arts, sciences, vocational, and academic streams.	• Cultivates **versatile thinkers** equipped with **critical 21st-century skills** for a dynamic world.
Multilingualism	• **Mother tongue** as medium of instruction till Grade 5 (preferably Grade 8). • Sanskrit and foreign languages as options, with projects like **'Ek Bharat Shrestha Bharat'** and **Indian Sign Language** standardisation.	• Celebrates and **preserves** India's **rich linguistic diversity**, enhancing cultural connect.

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Assessment Reforms	• Shifts to **competency-based**, **formative assessments**. Board exams twice yearly with **PARAKH** as the standard-setting body.	• Eases **exam pressure**, encouraging **deeper understanding** through holistic evaluation.
Equitable Education	• Focus on **Socially and Economically Disadvantaged Groups (SEDGs)** via **Gender Inclusion Fund**, **Special Education Zones**, and **Bal Bhavans**.	• Builds an **inclusive system** where every child, regardless of background, thrives.

3. Higher Education Reforms

Category	Details	Significance
GER Target	• Increase **GER** from 29.5% to **50% by 2035**, adding **3.5 crore seats**. • **Multidisciplinary streams** introduced in institutes like IITs, such as IIT Delhi offering non-engineering courses.	• Opens doors to **higher education** for millions, creating a more **skilled workforce**.
Holistic Education	• Flexible, **multidisciplinary UG programs** with **multiple entry/exit** options (Certificate after 1 year, Diploma after 2 years, Degree after 3/4 years). • **Academic Bank of Credit** for credit transfer.	• Empowers students with **flexible learning paths**, accommodating **diverse aspirations**.
Research Promotion	• **Anusandhan National Research Foundation** (previously National Research Foundation) and **Multidisciplinary Education and Research Universities (MERUs)** to match IIT/IIM standards.	• Sparks **innovation**, positioning India as a **global leader** in cutting-edge research.
Regulation	• Transitioning to a unified regulatory model via the **Viksit Bharat Shiksha Adhishthan Bill** (superseding HECI). It merges the functions of the UGC, AICTE, and NCTE, excluding medical and legal streams • **'Light but Tight' regulation** to reduce bureaucracy.	• Simplifies **governance**, ensuring **efficiency** while maintaining high standards.
Internationalization	• Top Indian universities to set up campuses abroad; **global universities** to operate in India through **institutional collaborations**.	• Elevates India's **global academic stature**, fostering **cross-cultural exchange**.
Open and Distance Learning	• Expansion of **online courses**, **MOOCs**, and digital repositories to boost GER. UGC's 2021 regulations for **online degrees** and **NDEAR** for digital infrastructure.	• Expands **access** to quality education, especially for **remote and underserved** areas.

4. Teacher Education, Technology, and Governance Reforms

Category	Details	Significance
Teacher Training	• Transitioning to the **4-year Integrated Teacher Education Programme (ITEP)** , which becomes the mandatory minimum teaching qualification by 2030.	• Elevates **teaching standards**, preparing educators for **modern classrooms**.
Recruitment and Career Path	• **Transparent**, **merit-based** recruitment and promotions. **National Professional Standards for Teachers (NPST)** by 2022 with **multi-source performance appraisals** and progression to educational administration.	• Builds a **motivated**, **accountable** teaching workforce with clear career paths.
Mentoring	• **National Mission for Mentoring,** operationalised through the release of the official "NMM-The Bluebook" guideline framework.	• Nurtures **new teachers**, fostering **professional growth** and confidence.
EdTech Integration	• **National Educational Technology Forum (NETF)** for idea exchange. • Technology in **classrooms and administration** via **National Digital Education Architecture (NDEAR)** for digital infrastructure.	• Transforms **learning**, making education **accessible** and engaging for all.
Online Education	• Dedicated unit for **digital content** and capacity building. • UGC's **online degree programs** and **MOOCs** for education during pandemics.	• Ensures **resilience**, delivering education seamlessly during **disruptions**.

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Public Investment	• Increase to **6% of GDP** for education through **Centre-State collaboration**.	• Fuels **infrastructure growth**, enabling a **robust education ecosystem**.
Governance	• **School complexes/clusters** for resource sharing. **State School Standards Authority (SSSA)** for regulation and **SCERT's School Quality Assessment Framework** for transparent self-disclosure.	• Streamlines **operations**, promoting **transparency** and resource efficiency.
Anti-Commercialization	• Education institutions as **'not-for-profit'** with **standardized audits** and checks to curb commercialization.	• Safeguards **education's integrity**, prioritizing **learning** over profit.

5. Criticisms and Challenges

Challenge	Details	Implication
Implementation Scale	**Systemic complexity** and diversity of India's education sector, with **institutional inertia** in education ministries, pose challenges in **coordinated execution** across states.	Creates a **scalability bottleneck**, potentially delaying transformative outcomes.
Linguistic Diversity	**Multilingual mosaic** of 22 official languages and dialects complicates **curriculum localisation** for mother tongue instruction.	Leads to **fragmented implementation**, risking uneven learning experiences.
Digital Divide	While **over 85% of households** now own a smartphone (per MoSPI/TRAI telecom data), a sharp divide persists in actual high-speed internet penetration and functional computer literacy.	Causes **digital exclusion**, leaving marginalised groups behind.
Resource Constraints	**Fiscal competition** for 6% GDP target amid other priorities delays **infrastructural upscaling**.	Results in **resource scarcity**, slowing the pace of systemic change.

6. Outcomes and Initiatives

Outcome/ Initiative	Details	Impact
Universalization	**Inclusive outreach** to achieve 100% GER from ECCE to secondary by 2030.	Drives **equitable access**, aligning with SDG 4's vision of inclusive education.
Skill Development	**Skill empowerment** ensures every child learns at least one skill through vocational education from Grade 6.	Boosts **employability**, preparing students for diverse career opportunities.
Assessment Reforms	**Competency-driven** testing of core concepts via SAFAL for **formative evaluation**.	Promotes **holistic learning**, nurturing creativity and analytical skills.
Digital Infrastructure	**Tech-enabled education** through **NDEAR and UGC's online degree programs** to support **digital inclusivity**.	**Democratizes access**, bridging urban-rural educational divides.

Three Language Formula

The **Three Language Formula (TLF)**, a key component of India's language education policy, was **first introduced** in the **National Policy on Education (NPE) 1968** and retained with modifications in the **National Education Policy (NEP) 2020.** Aimed at promoting multilingualism, national integration, and cognitive development, TLF has faced significant implementation challenges due to **India's linguistic diversity and federal structure**.

Constitutionally, **Article 351** mandates the promotion of Hindi, while **Article 29** safeguards minority languages and cultural rights. Additionally, with education on the **Concurrent List**, tensions often arise between the Centre and States over language policy implementation.

Evolution Stage	Description
Kothari Commission (1964-66)	First proposed the **Three-Language Formula** for multilingualism and national integration.
National Education Policy (1968)	Adopted the formula; emphasized regional languages in primary/secondary levels, encouraged in universities.

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1992 Programme of Action	Recommended **mother tongue/regional language** for pre-school communication.
Right to Education Act (RTE), 2009	Mandates that the medium of instruction in schools shall, as far as practicable, be in the child's mother tongue.
National Education Policy (NEP), 2020	Advocates **home/mother tongue/local language** as the medium of instruction until **Grade 5 (preferably Grade 8+).**
Latest Flagship Initiatives (2024-2026)	Operationalised via **Project ASMITA** (targeting 22,000 academic books in 22 languages) and the **Bharatiya Bhasha Pustak Scheme** providing digital native textbooks.

2. Key Features of TLF in NEP 2020

Features	Details	Significance
Language Selection	Students will learn three languages, with flexibility for states and learners; at least two should be Indian languages.	Promotes **inclusivity**, easing fears of **Hindi imposition**.
Medium of Instruction	**Mother tongue** as medium till **Grade 5** (preferably **Grade 8**).	Boosts **comprehension** and **cognitive growth**.
Teacher Training	Multidisciplinary programmes under the Integrated Teacher Education Programme to train bilingual and multilingual educators.	Enhances **teaching quality** for TLF success.
Digital Resources	Centralised hosting of cross-translated academic coursework onto national portals like **DIKSHA** and the National Digital Library of India (NDLI).	Makes **multilingual education** accessible.

3. Advantages and Implementation Challenges

Advantages	Implementation Challenges
Cognitive Development: Learning multiple languages enhances **brain plasticity** and **problem-solving**.	**Resource Constraints**: **Lack of teachers** and funding. For example, the lack of Southern language educators in Northern states .
National Integration: Exposure to diverse languages fosters **cultural understanding**. **Example:** *Exchange programs promoting Hindi learning in Kerala schools enhance inter-regional understanding.*	**Linguistic Diversity**: **19,500+ dialects** and **121 languages** complicate **curriculum adaptation**. **Example:** *Tribal-language inclusion in Jharkhand and Odisha requires localized textbooks and teacher training.*
Career Opportunities: Proficiency in **English** and regional languages offers a **competitive edge**. **Example:** *Tamil Nadu's English skills* drive IT success.	**Federal Tensions**: **State autonomy** causes conflicts. **Example:** *Tamil Nadu's reaffirmation on two-language policy*
Cultural Preservation: Promotes **regional languages** to prevent extinction.	**Declining Teaching Standards**: Poor **teacher proficiency** in English and regional languages.
Global Competitiveness: Multilingual skills meet **global demands**. **Example:** *Bilingual and multilingual graduates from metropolitan schools increasingly enter global service sectors.*	**Academic Burden**: Three languages increase **student workload**. **Example:** *Students in Hindi-speaking states rarely receive exposure to southern languages, creating imbalance in language learning.*

4. Recommendations for Effective Implementation

Recommendation	Details
Strengthen Teacher Training	Invest in **multilingual training programs** like training teachers in Tamil for Haryana schools to promote southern languages.
Leverage Technology	• Use **AI tools** like **BHASINI and ANUVADINI** for translations and develop **digital repositories** in regional languages such as **Bharatiya Bhasha Pustak Scheme and e-KUMBH portal** that provides **digital textbooks** and study materials in multiple Indian languages. • Actively implement **Project ASMITA** (*Augmenting Study Materials in Indian Languages through Translation and Academic Writing*) launched by the UGC and Ministry of Education, which targets the creation of 22,000 localized higher education books across 22 Scheduled Languages over 5 years.

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Cooperative Federalism	• Encourage **dialogue between the Centre and States** to ensure smooth **NEP 2020 implementation**. • Avoid structural deadlocks by separating core administrative infrastructure funding (like Samagra Shiksha grants) from rigid linguistic mandates.
Data-Driven Policy	• Collect **sociolinguistic data** to tailor TLF to regional needs like **Bahubhasha Shabdkosh** that aims to create a **multilingual dictionary repository.**
Reduce Academic Burden	• Integrate **language learning** with subjects like social studies by teaching **Tamil culture** alongside language to make learning engaging.

The **National Education Policy 2020** envisions a future-ready, inclusive, and knowledge-driven India. As **Prime Minister of India** rightly said, **"NEP will lay the foundation of a New India — an India of the 21st century."**

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12 Skill Development

India's demographic dividend represents a transformative economic opportunity, with its working-age population (15-59 years) projected to peak around 2030, covering approximately 65% of the country's demographic profile. It serves as a **catalyst for bridging education** and employment, fostering **sustainable economic growth, social mobility,** and **global competitiveness**. Despite robust initiatives, systemic challenges like low participation and skill-job mismatch persist.

UPSC Mains PYQs

1. "'Earn while you learn' scheme needs to be strengthened to make vocational education and skill training meaningful." Comment. **(2021)**
2. Skill development programmes have succeeded in increasing human resources supply to various sectors. In the context of the statement, analyse the linkages between education, skill and employment. **(2023)**
3. "Demographic Dividend in India will remain only theoretical unless our manpower becomes more educated, aware, skilled and creative." What measures have been taken by the government to enhance the capacity of our population to be more productive and employable? **(2016)**

1. Overview of Skill Development

Aspect	Details
Definition	Skill Development entails the **strategic** acquisition or enhancement of **technical**, **vocational**, and **soft skills** through targeted training to boost **employability** and **productivity**.
Context	India has the largest youth population in the world, with about 65% of its people under the age of 35, necessitating adaptive skills to meet Industry 4.0 demands. Only 4.1% of the workforce is formally skilled (PLFS 2023-2024), underscoring the urgency.
Objectives	Facilitate **economic empowerment**, **social equity**, and **entrepreneurial innovation**, aligning competencies with **dynamic market needs**.
Significance	Underpins **Viksit Bharat 2047** by addressing **unemployment-employability disparities** and fostering a **high-skills ecosystem**.

2. Advantages of Skill Development

Advantage	Details	Impact
Economic Growth	Equips youth for **high-demand sectors** like **renewable energy** (3.5 million jobs in India by 2030) and **AI** (USD 7.8 billion market by 2025).	Catalyses **GDP growth** and **labor productivity**.
Social Mobility	Enhances **employability** for marginalised communities. **Example:** *Data from the Jan Shikshan Sansthan (JSS) scheme shows that approx 83% of beneficiaries are women, effectively driving financial independence at the grassroots.*	Mitigates **poverty** and fosters **social inclusion**.
Entrepreneurship	Promotes **self-employment** through skills like **digital literacy**. **Example:** *Common Service Centres (CSCs) and the PM Vishwakarma scheme transform traditional artisans into digitally integrated nano-entrepreneurs.*	Stimulates **innovation** and **job creation**.
Global Competitiveness	Aligns skills with **international benchmarks**. **Example:** The National Credit Framework (NCrF) integrates academic, vocational, and experiential learning into a unified system where 1,200 notional learning hours equal 40 credits.	Positions India as a premier **global talent and skill repository,** facilitating orderly **cross-border labor mobility.**

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3. Challenges of Skill Development

Challenge	Details	Implication
Low Participation	Only **4.1%** of youth (15-29 years) receive formal vocational training. (PLFS 2023-2024) **Example:** *NITI Aayog (2023)* notes rural enrollment in skilling programs is *far lower than in urban areas.*	Constrains **skill diffusion**, undermining employability.
Skill Mismatch	**India faces a severe paradox: despite producing millions of graduates annually, roughly 55-60% of employers report candidates lack job-relevant skills.** **Example:** *ITI curricula* often lag behind AI-driven industry needs.	Diminishes **placement outcomes**, with only 21.4 lakh placed from 1.1 crore trained.
Informal Sector Dominance	With over 92% of the total workforce operating informally, the vast majority rely on on-the-job learning rather than structured vocational or technical schooling. **Example:** *MSMEs prioritise on-the-job* learning over certifications.	Weakens **formal skilling efficacy** and wage growth.
Limited Access	Rural areas face shortages of **training facilities** and **qualified instructors**. **Example:** *Currently, only about 10% of the rural workforce has received formal vocational training under PMKVY.*	Exacerbates **regional disparities**, excluding marginalised groups.
Technological Disruption	Rapid advancements in **AI** and **automation** outpace skill updates. **Example**: 63% of employees require reskilling by 2030 **(Future of Jobs Report, 2025)**	Creates a **reskilling deficit**, challenging programme relevance.

4. Linkages Between Education, Skill, and Employment

Linkage	Details	Example	Analysis
Education to Skill	**NEP 2020** integrates **vocational education** from Grade 6, fostering a **synergy** between academic and practical competencies.	*Vocational electives* in government schools prepare students for *advanced skill programs.*	**Strength**: Establishes foundational skills. **Gap**: Rural schools lack infrastructure for effective integration.
Skill to Employment	Skill initiatives like **PMKVY** and **NAPS** deliver **market-oriented training**, enhancing job readiness. **1.64 crore** trained under PMKVY.	*Industry-driven courses* at training centres secure placements in *IT* and *renewable energy*.	**Strength**: Aligns with industry needs. **Gap**: Limited placements **(21.4 lakh from 1.1 crore trained)** due to skill mismatches.
Education-Skill-Employment Cycle	A **holistic framework** combines **formal education**, **vocational training**, and **on-the-job exposure** to ensure sustainable employability.	*ITI graduates* with apprenticeships secure roles in *manufacturing.*	**Strength**: Produces a competent workforce. **Gap**: **Weak industry-academia Collaboration** hinders seamless transitions.

5. Key Government Initiatives

Initiative	Details	Significance
Flagship Schemes		
Pradhan Mantri Kaushal Vikas Yojana (PMKVY)	Launched in 2015, trained **1.64 crore** youth in over 35 sectors. Version 4.0 emphasises **AI**, **robotics**, and **soft skills**.	Enhances **employability** through **industry-aligned** training.
SANKALP	World Bank-assisted program to improve **short-term training quality** and include marginalised sections.	Strengthens **institutional capacity** for **inclusive skilling**.
Training Infrastructure		
Pradhan Mantri Kaushal Kendra (PMKK)	PMKKs serve as model skill training centres equipped with modern infrastructure and industry-relevant courses.	Creates **sustainable ecosystems** for quality training delivery.

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Industrial Training Institutes & PM SETU	DGT-governed vocational ecosystem. Upgraded via the **PM SETU scheme** (₹60,000 cr outlay) targeting 1,000 ITIs through a Hub and Spoke model.	Scales up technical manpower output for core industries like electronics and smart manufacturing.
Specialized Programs		
Jan Shikshan Sansthan (JSS)	Offers **vocational training** to non/neo-literates and dropouts, benefiting **32,53,965 beneficiaries** from 2018 to October 2025.	Improves **livelihood opportunities** for undereducated groups.
National Apprenticeship Promotion Scheme (NAPS)	Launched in 2016, supports **apprenticeships** with financial incentives (₹1,500/month stipend) for MSMEs.	Bridges the **theoretical-practical divide** via **on-the-job training.**
Emerging Sectors		
TEJAS Skilling Project	Focuses on **new-age competencies** like **3D printing**, **IoT**, and **drones** to meet Industry 4.0 requirements.	Prepares youth for **future-ready occupations** in high-growth sectors.

6. Skill Development in the Digital Era

Opportunities	Challenges
Scalable Digital Platforms: Online portals like **Skill India Digital Hub** democratize access to **industry-aligned courses** (such as AI, IoT), enabling **flexible**, **self-paced learning** for millions. **Example:** Under PMKVY 4.0, over 5.10 lakh candidates have been trained in 162 futuristic job roles	**Digital Divide**: Although digital connectivity has improved, a substantial urban-rural digital divide persists.
New-Age Skill Development: Standardized national frameworks, such as the National Programme on Artificial Intelligence (NPAI) Skilling Framework by NCVET, align domestic training with global AI, 3D printing, and cybersecurity requirements.	**Skill Obsolescence**: Rapid **technological advancements** (such as automation) render skills outdated, with nearly 63% of **employees** needing reskilling by 2030 (Future of Jobs Report 2025). **Example:** *ITI curricula lag* behind AI-driven industry needs.
Gig Economy Opportunities: Digital literacy and **self-management** courses prepare youth for the **gig economy**, projected to reach **23.5 million jobs** by 2029-30.	**Digital Illiteracy**: Lack of **ICT skills** among rural and marginalised groups hinders engagement with digital platforms.
Global Competitiveness: Certifications in **quantum computing** and **cybersecurity** position India as a **knowledge economy**.	**Infrastructure Gaps**: Shortages of **computers** and **qualified trainers** in rural areas limit digital skilling. **Example:** According to NSSO data, only 4.4% of rural households own computers.
Entrepreneurial Innovation: Digital skills enable **e-commerce** and **tech startups**, fostering **self-employment**. **Example:** *Common Service Centres* train rural youth in digital marketing, boosting local businesses.	**Funding Constraints**: Limited budgets for **digital infrastructure** and **trainer upskilling** hamper programme scalability.

7. Way Forward for Skill Development

Recommendation	Details
Demand-Driven Skilling	Leverage **AI-driven labor market analytics** to forecast skill demands. **Example:** *Singapore's Skills Future model* informs national strategies.
Industry-Academia Synergies	Establish **Skill Centres of Excellence** with private sector collaboration. **Example:** *ITI Upgradation Scheme 2024* modernises training.
Rural Inclusion	Transform **Common Service Centres** into Digital Skill Hubs for rural training. **Example: CSC-Gramin Digital Saksharta Abhiyan (PMGDISHA):** Scaling basic ICT literacy into certified micro-courses across tier-3 and rural setups.
Gender Equity	Implement **women-centric programs** to boost participation.
Future-Ready Reskilling	Introduce **modular certifications** for **AI**, **quantum computing**, and **green skills**. Expand green skill programmes aligned with renewable energy, sustainable construction, waste management, and circular economy sectors.

Skill development in India is crucial for harnessing the **demographic dividend** and building an **Aatmanirbhar Bharat.** By promoting **industry-relevant, inclusive, and future-ready skills**, India aims to **boost employability, entrepreneurship, and global competitiveness** across all sectors.

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13 Poverty

Poverty is a **state of extreme deprivation** where people lack basic necessities in life, like food, clothes, and shelter. It is often **characterized by social exclusion, limited opportunities for individual growth**, and a **lack of access to resources and services** that are needed for a decent standard of living.

According to the **World Bank (2025)**, **India lifted 171 million people out of extreme poverty** in a decade, **reducing extreme poverty** from 16.2% (2011-12) to 2.3% (2022-23).

UPSC Mains PYQs

1. Poverty and malnutrition create a vicious cycle, adversely affecting human capital formation. What steps can be taken to break the cycle? **(2024)**
2. Can the vicious cycle of gender inequality, poverty and malnutrition be broken through the microfinancing of women SHGs? Explain with examples. **(2021)**
3. "Incidence and intensity of poverty are most important in determining poverty based on income alone". In this context, analyse the latest United Nations Multi-Poverty Index report. **(2020)**
4. Though there have been several different estimates of poverty in India, all indicate a reduction in poverty over time. Do you agree? Critically examine with reference to urban and rural poverty indicators. **(2015)**

1. Causes of Poverty in India

Cause	Description	Example
Historical		
Colonial Policies	Colonial exploitation entrenched **structural inequality** by **prioritising resource-rich regions**, perpetuating **regional disparities** and **economic marginalisation**.	*Port cities* like *Bombay and Calcutta* developed, while others lagged.
Caste System	**Social stratification** through the caste system fosters **systemic marginalisation**, limiting access to resources and opportunities for lower castes.	NCRB 2023: **57,500+ crimes against Scheduled Castes**, indicating continuing socio-economic vulnerability.
Historical Conflicts	**Conflicts and displacements** disrupted livelihoods, reinforcing **cycles of deprivation** and **social disorganisation.**	*World Bank (2017) partition of India displaced 12-20 million,* with lasting economic impacts in Punjab and Bengal.
Economic		
Low Agricultural Productivity	**Structural dependency** on **traditional agriculture**, characterised by low capital investment and **outdated techniques**, perpetuates **economic vulnerability.**	*FAO: low productivity* traps farmers in poverty.
Population Explosion	**Demographic pressure** exacerbates **resource scarcity**, intensifying competition and economic exclusion.	*India's population crossed **1.43 billion (UN Population Report 2024)**.*
Low Per Capita Income	**Persistent underdevelopment** and **unequal income distribution** limit economic mobility, **reinforcing poverty traps.**	*Nearly 60% of India's population live on less than $3.10 a day, the World Bank's median poverty line*
Social		
Lack of Education	Educational exclusion, rooted in **social inequities, restricts human capital development** and perpetuates **intergenerational poverty.**	*PLFS 2023-24: Literacy 80.9%; female literacy significantly lower in rural regions.*

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Gender Inequality	**Patriarchal structures** and gendered **social norms limit women's access** to education and employment, **deepening the feminisation of poverty.**	*World Economic Forum (2025):* India ranks 131/148 in gender equality, with women earning 27% less in regular jobs.
Lack of Healthcare Access	**Social determinants** of health, including **inadequate healthcare infrastructure**, **exacerbate morbidity** and **reduce productivity.**	*Household Out-of-Pocket Expenditure still 43.4% of total health spending.*
Political		
Corruption and Nepotism	**Institutional corruption** and **clientelism** skew resource allocation, reinforcing **political exclusion.**	*Vyapam Scam:* Manipulation in admissions and job recruitment disadvantaged deserving candidates.
Unequal Resource Allocation	**Political favouritism** in resource distribution perpetuates regional and social inequities.	*NITI Aayog (2021):* Disparities in PM-KISAN fund allocation across states despite uniform goals.
Political Polarization	**Polarised political discourse** undermines governance, hindering poverty alleviation policies.	*West Bengal elections* saw violent clashes, disrupting development initiatives.
Environmental		
Climate Change	**Environmental injustice disproportionately** impacts marginalised communities, disrupting livelihoods through **extreme weather.**	Climate Risk Index: India ranks 9th globally for long-term vulnerability to extreme weather events .
Land Degradation	**Ecological degradation**, driven by unsustainable practices, **undermines agrarian livelihoods** and reinforces **environmental poverty.**	Over 29% of India's land is degraded, severely threatening agricultural productivity and rural livelihoods.
Pollution	**Environmental health burdens,** such as air and water pollution, exacerbate morbidity among the poor, entrenching poverty.	*State of Global Air 2025:* Air pollution caused over 2 million premature deaths in India.

2. Causes of Rural and Urban Poverty in India

Causes of Rural Poverty	Causes of Urban Poverty
Low Agricultural Productivity: Structural dependency on traditional, low-yield farming methods perpetuates economic vulnerability and income instability.	**Informalisation of Urban Economy:** Majority of workers employed in the informal sector lacking job security, social protection and stable income.
Limited Access to Education and Skills: Educational exclusion in rural areas restricts human capital development, limiting job opportunities.	**Limited Access to Affordable Housing**: High urban housing costs force the poor into slums or homelessness, deepening economic hardship.
Example: *NSSO 2020-21:* Rural literacy rate lower than urban, with only 70.3% for women.	**Example**: *India has more than 1.7 million homeless residents, of which 938,384 are located in urban areas.*
Dependence on Rain-Fed Agriculture: Environmental vulnerability due to reliance on monsoon rains increases income uncertainty.	**Unemployment and Informal Employment**: High competition for jobs and dominance of informal sector jobs with low wages perpetuate urban poverty.
Example:**Climate Risk Index:** India ranks 9th globally for long-term vulnerability to extreme weather events; 60% agriculture is rain-fed.	**Example**: *CMIE (2020) Urban unemployment* spiked to 23.5% during the COVID-19 lockdown.
Land Degradation: Ecological degradation from overuse and deforestation reduces agricultural output, reinforcing poverty.	**Gender & Social Discrimination in Labour Markets:** Women, SC/ST and migrants face barriers in accessing stable urban employment.
Spatial Inequality and Poor Infrastructure: Marginalisation due to remote locations and limited access to services like roads and markets hinders development.	**Weak Urban Social Security Coverage:** Migrant and informal workers lack access to PDS, healthcare, pensions and welfare portability.
Example: *Vidarbha, Maharashtra suffers from high farmer suicides* due to lack of jobs and services.	

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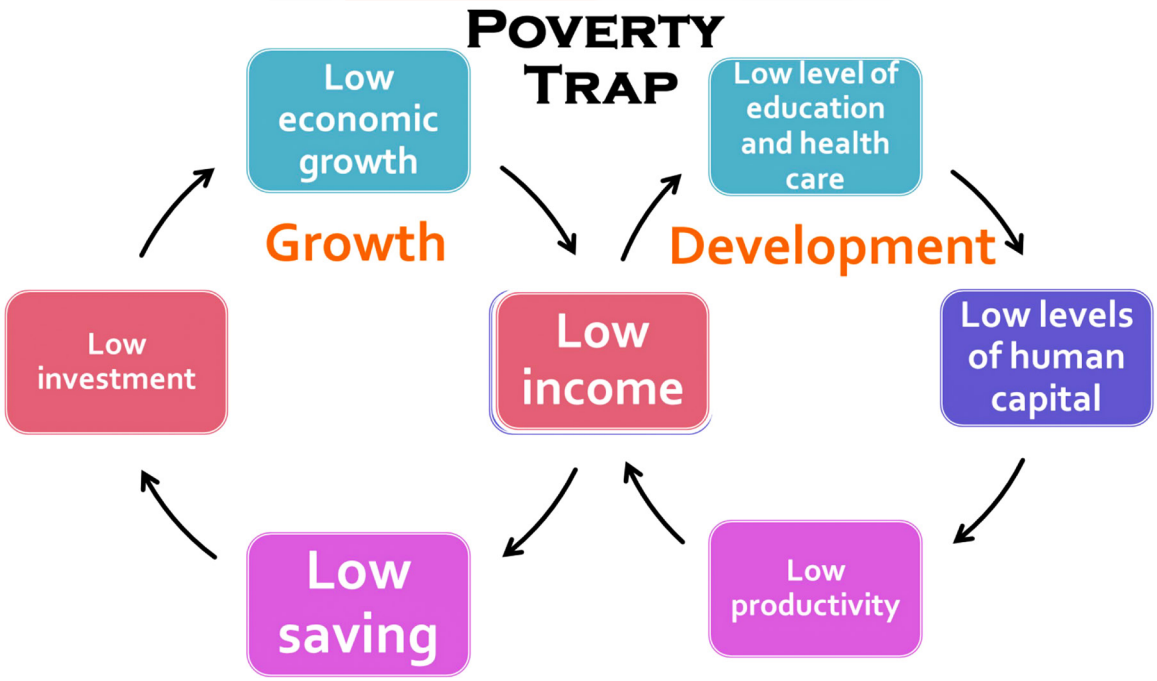
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3.Feminisation of Poverty in India

Reasons	Description
Gendered Wage Disparities	**Patriarchal norms and occupational segregation** result in women earning significantly less than men, **deepening economic vulnerability.** **Example: PLFS 2025:** Average monthly regular salaried earnings show distinct gaps—Men earn ₹24,217 compared to ₹18,353 for women
Limited Access to Education	**Gendered inequality in education** restricts women's human capital development, **limiting employability and perpetuating poverty cycles.** **Example**: *NSSO 2020-21: Women's literacy rate at 70.3% compared to 84.7% for men*, with rural women particularly disadvantaged.
Restricted Employment Opportunities	**Social exclusion** and **discriminatory practices** confine women to low-paid, informal sector jobs, reinforcing economic marginalisation. **Example**: *WEF's Global Gender Gap report(2025): India ranks 131/148 in gender equality* .
Unpaid Domestic and Care Work	**Patriarchal expectations** burden women with **unpaid household and caregiving responsibilities**, reducing time for income-generating activities. **Example**: *NFHS-5 (2019-21): women spend 5-6 hours daily* on unpaid domestic work, compared to under 1 hour for men.
Lack of Property Ownership	Gendered land and asset ownership norms **limit women's economic security and access to credit**, entrenching poverty. **Example**: *Only 14% of agricultural land in India* is owned by women, despite their significant role in farming.
Vulnerability to Health Risks	**Limited healthcare access** and **gender-specific health issues** (such as maternal health) **exacerbate women's poverty** and **reduce productivity.**
Social Discrimination	**Caste, religion, and gender-based discrimination amplify poverty** among women from marginalised groups, restricting opportunities.
Impact of Early Marriage and Childbearing	**Cultural practices** like **early marriage increase** dependency and **limit education** and **economic opportunities**, perpetuating poverty. **Example**: *NFHS-6 (2023-24)* 20.1% of women aged 20-24 were married before age 18, particularly in rural areas.

The impact of the **Feminisation of Poverty** is that **women experience disproportionate poverty** due to systemic gendered disadvantages, resulting in socio-economic disempowerment.

4. Impact of Poverty on Human Capital Formation



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Impact of Poverty	Details	Example
Education		
Restricted Access to Quality Education	- **Multidimensional poverty** limits access to schooling due to direct costs (fees, uniforms) and opportunity costs (child labor), exacerbating **educational inequity and** reducing **intergenerational mobility**.	India recorded 65.7 lakh school dropouts in the last 5 years.
Impaired Cognitive Development	Poverty-induced **malnutrition** and **psychosocial stress** hinder cognitive growth, per **Amartya Sen's capability approach**, leading to poor learning outcomes and higher dropout rates, limiting **employability** and **economic agency**.	Half of the global population with micronutrient deficiencies lives in India.
Health		
Compromised Nutrition and Well-being	• **Multidimensional deprivation** causes **undernutrition** (stunting, wasting), reducing physical and cognitive **capabilities**, as per **Gary Becker's human capital theory**. • This diminishes productivity and **life expectancy**, critical for **sustainable development**.	NFHS-6: Over 29.3% of children under five in India are stunted.
Inadequate Healthcare Access	• Poverty restricts access to **healthcare services**, leading to untreated illnesses and lower **health capital**, perpetuating **poverty traps**. • Lack of **universal health coverage** exacerbates vulnerability.	Household Out-of-Pocket Expenditure (OOPE) to 43.4% of total health spending.
Skills Development		
Limited Skill Acquisition	Poverty curtails access to **vocational training** and higher education, trapping individuals in low-skill, **informal sector** jobs, hindering **inclusive growth** and **economic diversification**.	World Bank: The 24.2% NEET (Not in Education, Employment, or Training) rate for youth in India is a major structural bottleneck.
Digital Exclusion	The **digital divide**, driven by inability to afford technology, excludes the poor from **digital literacy** and modern job markets, limiting **technological human capital**.	Only 6.8% of the poorest 20% households have computer and internet access, compared to 66.3% among the richest 20%.
Social and Psychological Effects		
Erosion of Social Capital and Aspirations	Poverty erodes **social capital** (networks, trust) and fosters **learned helplessness**, reducing **motivational capital** and aspirations for education or skills, as per Sen's **capability deprivation**.	Over *one-third of tribal boys in Odisha drop out at the secondary level*.
Gendered Capability Deprivation	Poverty amplifies **gender disparities**, with families prioritizing boys' education, limiting women's **human capital contribution** and **economic empowerment**, reinforcing **patriarchal structures**.	*India is home to an estimated 223 million child brides, accounting for roughly one-third of the global total.*
Economic Productivity		
Diminished Workforce Efficiency	Poor **health capital**, **educational attainment**, and **skill levels** reduce **labor productivity**, hampering **economic growth** and **global competitiveness**, as per Becker's framework.	India ranks *116th out of 174 countries in the World Bank's Human Capital Index (HCI) 2020 report*.
Intergenerational Poverty Traps	Limited **human capital investment** traps families in low-wage cycles, reducing **social mobility** and perpetuating **multidimensional poverty**, undermining **inclusive development**.	Bihar has the maximum number of poor people in India, with 51.91 per cent of the population in the lower-income class.

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5. Government Policies and Initiatives

Policy/Initiative	Description
Viksit Bharat – Guarantee for Rozgar and Ajeevika Mission (Gramin) Act, 2005	Guarantees **125 days** of wage employment annually to rural households. Links employment generation with infrastructure creation and climate, addressing **unemployment and structural inequality** while promoting rural infrastructure development.
Pradhan Mantri Awaas Yojana-Gramin 2016	Aims to provide **affordable housing to rural poor**, reducing spatial inequality and enhancing living **standards for Below Poverty Line (BPL) families**.
Pradhan Mantri Jan Dhan Yojana (PMJDY)	**Promotes financial inclusion** by ensuring bank accounts for the unbanked, facilitating **direct benefit transfers (DBT)** to combat economic exclusion. **Example**: *Total accounts have reached over 53.5 crore, with total deposits crossing ₹2.35 lakh crore(55% accounts held by women)*.
Deendayal Antyodaya Yojana-National Urban Livelihoods Mission, 2013	**Reduces urban poverty** by providing skill training and self-employment opportunities, addressing urban marginalisation. **Example**: *Trained 1.2 crore urban poor by 2024*, with *4 lakh linked to jobs via Employment* through Skills Training & Placement (EST&P).
Pradhan Mantri Garib Kalyan Anna Yojana (PMGKAY), 2020	Provides **free food grains to the poor to mitigate food insecurity** during crises, reinforcing social safety nets. **Example**: *Benefited 80 crore* people with *5 kg free rice/wheat and 1 kg pulses monthly during the covid-19* lockdowns.

6. Challenges in Poverty Alleviation in India

Challenge	Description
Lack of Effective Targeting	**Structural inefficiencies** in identifying and reaching the poorest households lead to benefits being misdirected, **undermining equitable resource distribution.** **Example**: *PM-KISAN scheme* included non-eligible beneficiaries, diluting funds for deserving farmers.
Unsustainability of Programs	**Financial constraints** and **inconsistent funding** hamper the long-term viability of poverty alleviation initiatives, perpetuating economic precarity. **Example**: *MGNREGA faced wage delays* in 2022 due to budgetary shortfalls, affecting rural workers' livelihoods.
Capacity Constraints	**Limited institutional capacity**, including **inadequate skilled staff and infrastructure**, restricts the effective implementation of anti-poverty programmes. **Example**: *National Health Mission* struggles with a shortage of healthcare professionals in rural areas, limiting service delivery.
Lack of Coordination	**Fragmented governance** and overlapping mandates among agencies lead to inefficiencies and duplication of efforts, hindering systemic impact. **Example**: *Swachh Bharat Mission* saw coordination issues between rural and urban departments, delaying sanitation projects.
Exclusion of Marginalized Groups	**Intersectional barriers**, such as **caste, gender, and ethnicity-based discrimination**, exclude vulnerable populations from accessing benefits, reinforcing social exclusion. **Example**: *Marginalised communities are* often excluded from PDS due to documentation issues and bureaucratic bias.
Corruption and Leakages	Institutional corruption and clientelism **divert resources meant for the poor**, eroding trust and program effectiveness. **Example**: *Leakages in PDS →subsidised grains* not reaching intended beneficiaries in some states.
Inadequate Monitoring and Evaluation	**Weak monitoring systems** fail to assess program impact, allowing inefficiencies to persist and **limiting adaptive policy-making.** **Example**: *lack of real-time data in NRLM* delayed course corrections for SHG empowerment initiatives.
Regional Disparities	**Spatial inequality** in resource allocation favors developed regions, **leaving backward areas underserved** and **perpetuating uneven development**. **Example**: *Aspirational Districts Programme (2023)*: Eastern states like Bihar lag in health and education outcomes compared to southern states.

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7. Way Forward for Poverty Alleviation in India

Strategy	Description
Conditional Cash Transfers (CCT)	Implement **targeted cash transfers** contingent on behaviors like school attendance or health check-ups to **promote human capital development** and **structural equity.** **Example**: *Brazil's Bolsa Família program*
Microfinance and Self-Help Groups (SHGs)	**Expand access to microcredit** and **strengthen SHGs** to **foster entrepreneurship**, particularly among women, enhancing **economic empowerment** and **social inclusion**. **Example**: *Bangladesh's Grameen Bank model*
Community-Driven Development (CDD)	**Empower local communities** to manage resources and design poverty alleviation programs, ensuring contextual relevance and **participatory governance.**
Vocational Training and Skill Development	Scale up skill-building programs to **enhance employability** and **economic mobility**, addressing skill deficits and labor market exclusion. **Example**: *Germany's dual vocational training system*
Safety Net Programs	Strengthen social safety nets with **cash and food transfers** during crises to protect vulnerable populations from economic shocks and precarity. **Example**: *Ethiopia's Productive Safety Net Programme*
Inclusive Growth Policies	**Promote progressive taxation** and **equitable resource allocation** to reduce income inequality and address intersectional disparities in wealth distribution.
Digital Inclusion	**Bridge the digital divide** by **expanding access to technology and digital literacy**, enabling the poor to access education, jobs, and services.

India has made remarkable progress in reducing poverty through **inclusive policies and targeted interventions** aligned with **SDG 1**. With continued focus on empowerment, sustainability, and equal opportunity, the nation is steadily moving towards a future rooted in **Sabka Saath, Sabka Vikas**.

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14 Hunger

Hunger, as defined by the **Food and Agriculture Organization (FAO)**, is an uncomfortable or painful physical sensation caused by insufficient consumption of dietary energy. It becomes chronic when a person does not consume enough calories regularly to lead a normal, active, and healthy life.

In India, hunger remains a significant challenge despite economic growth and advancements in food production. The country continues to grapple with issues of poverty, malnutrition, and unequal food distribution. According to the **2025 Global Hunger Index, India ranks 102nd out of 123 countries.** With a score of 25.8 in the 2025 Global Hunger Index, India has a level of hunger that is **serious**.

UPSC Mains PYQs

1. There is a growing divergence in the relationship between poverty and hunger in India. The shrinking of social expenditure by the government is forcing the poor to spend more on non-food essential items, squeezing their food budget. Elucidate. **(2019)**
2. Hunger and Poverty are the biggest challenges for good governance in India still today. Evaluate how far successive governments have progressed in dealing with these humongous problems. Suggest measures for improvement. **(2017)**

1. Causes of Hunger in India

Cause	Description	Example
Poverty	The primary driver, that families below the poverty line **struggle to afford nutritious food.**	*"State of Food Security and Nutrition in the World" (SOFI) 2026, FAO:* 35.5% of India's population was unable to afford a healthy diet.
Unequal Land Distribution	Large landowners dominate, limiting small farmers' access to fertile land, **exacerbating food insecurity.**	Approx 89.4% agricultural holdings are small & marginal (<2 ha).
Gender Inequality	**Patriarchal norms** prioritise men/ boys, limiting women's/girls' access to nutritious food.	*NFHS-5:* 59.1% of girls are anaemic, reflecting intra-household disparities, per the Ministry of Health.
Child Marriage	**Young mothers lack nutritional knowledge**, leading to malnourished infants.	*NFHS-6:* 20.1% of women aged 20-24 married before 18, linked to low birth weight babies.
Climate Change	**Droughts, floods, and erratic monsoons disrupt agricultural production**, causing food shortages.	*Economic Survey 2024:* climate change could reduce agricultural yields in India by up to 15% to 16% by 2030
Poor Implementation	**Corruption, leakages in PDS,** and a **lack of monitoring** hinder food distribution.	~30% of PDS grains are diverted to the open market in some states.
Poor Sanitation	**Diarrhoeal diseases** from **inadequate sanitation** impair nutrient absorption.	Around 12.5% of Indian households, or over 162 million people, still lack access to basic household toilet facilities.

2. Impact of Hunger and Malnutrition

Impact	Description	Example
Physical and Mental Health	**Reduces cognitive function,** weakens immunity, and increases disease susceptibility.	*Lancet Study (2023):* Stunted children in India lose 4-6 IQ points by adulthood.
Child Development	**Stunting and wasting** impair physical and cognitive growth, limiting potential.	Nearly *a third of Indian children under five years* of age are stunted and underweight.
Intergenerational Cycle	**Malnourished mothers** give **birth to low birth weight infants**, perpetuating hunger.	*UNICEF (2023):* Malnutrition is a leading cause of preventable child mortality in India.

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Productivity Loss	**Reduces workforce efficiency,** costing up to 4% of GDP and 8% of productivity.	**The World Bank estimates** India loses **over $12 billion in GDP annually** due to undernutrition-related productivity losses.
Social Unrest	**Hunger can fuel conflict** and **instability** as people struggle for basic needs.	*2018 Marathwada farmers' protests* linked to drought-induced hunger, per NCRB data.

3. Government Initiatives to Tackle Hunger

Initiative	Objective	Impact
Pradhan Mantri Garib Kalyan Anna Yojana	**Ensure food security** through subsidised food grains via TPDS, covering 75% rural and 50% urban populations.	**Covers 81.35 crore people**, providing 5 kg/person/month rice/ wheat/coarse grains which are free of cost.
Saksham Anganwadi & Poshan 2.0 with ICDS	Reduce stunting, wasting, low birth weight, and anemia via behavioral change, convergence, and fortified rice distribution across all welfare schemes.	Over 9 crore beneficiaries registered on the digital Poshan Tracker App.
Pradhan Mantri Poshan Shakti Nirman	**Provide nutritious meals** to schoolchildren to improve nutritional status, **extended to pre-primary in 2021.**	Serves over 118 million children daily across 1.12 million schools.
Pradhan Mantri Matru Vandana Yojana (PMMVY)	**Cash incentives** for **pregnant/lactating mothers** to improve maternal health, under ICDS.	Over 4.26 crore women supported via DBT.

4. Challenges in Eradicating Hunger

Challenge	Description	Example
Low Status of Women	*Limited decision-making power* and lack of control over resources restricts women's access to nutrition.	*NFHS-5 (2019–21): 59.1% of women* aged 15–49 are *anaemic*.
Low Budget Allocation	Reduced funding *hampers nutrition programs'* reach and quality.	*Budget 2024-25:* Nutrition allocation at 0.2% of total expenditure.
Lack of Monitoring	Poor real-time tracking significantly impacts the effectiveness and results of the schemes implemented.	Mandating Aadhaar e-KYC and bank account linking has blocked access for needy families.
Food Wastage	Inefficient supply chains significantly contribute to the loss of fruits and vegetables.	*UNEP Food Waste Index Report:* India's household food waste is estimated at 55 kg per capita annually.
Maternal Knowledge Gaps	Insufficient knowledge regarding breastfeeding and nutrition affects maternal and child health.	Decline in exclusive breastfeeding among children aged under six months, from 63.7% in NFHS-5 to 55.8% in NFHS-6

5. Strategies to Eliminate Hunger

Strategy	Description
Multisectoral Approach	Integrate water, sanitation, education, and agriculture for holistic solutions, as per **NITI Aayog's New India @75** and **Swaminathan Committee (2006)** for **nutrition-sensitive agriculture.**
Dietary Diversification	Promote millets, leafy vegetables, and eggs under NFSA to reduce hidden hunger, with Haryana's **Rs 7,000/ acre incentive for pulses/oilseeds**, aligned with **the 2023 International Year of Millets**.
Climate-Smart Agriculture	**Adopt Zero Budget Natural Farming (ZBNF)** and climate-resilient crops, supported by **NMSA's 25.79 crore soil health cards**.
Strengthen PDS	**Reform supply chains, reduce leakages, and ensure quality grains** via **'One Nation One Ration Card'**, as recommended by the **Justice Wadhwa Committee (2009).**
Gender Justice	**Empower women through education** and resource access to **break malnutrition cycles.**
Awareness	**Diverse stakeholders**, including NGOs, academic institutions, PRIs, SHGs, etc., along with governments, should promote public awareness on nutritional security and food wastage.

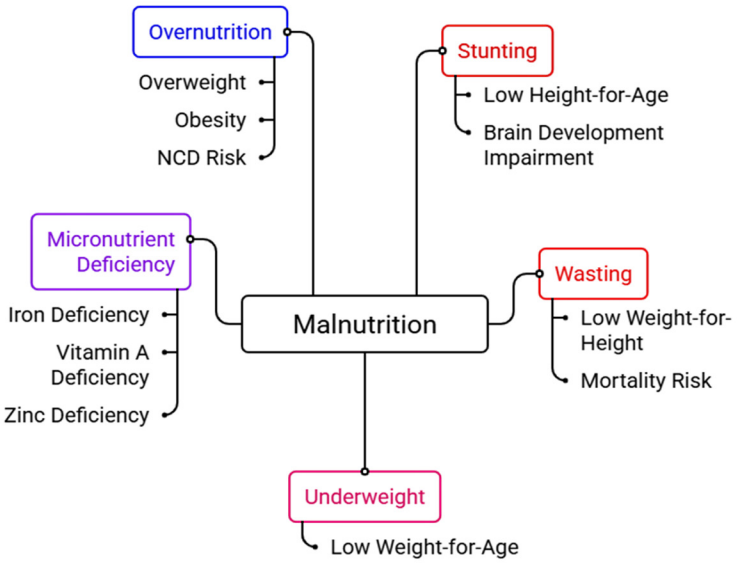
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India has made progress in reducing hunger, with **undernourishment declining to 13.7%**. Efforts to improve nutrition and food security are showing results, but sustained action is needed to **achieve Zero Hunger by 2030** and ensure a brighter future for all.

6. Nutritional Crisis in India

India is navigating a complex "triple burden" of malnutrition, characterized by the simultaneous prevalence of undernutrition (stunting and wasting), widespread micronutrient deficiencies (hidden hunger), and a rapidly growing crisis of overnutrition (obesity).



Statistics of Malnutrition

- **Stunting:** Declined substantially to **29.3%** in NFHS-6 (2023-24) from 35.5% in NFHS-5.
- **Wasting:** India recorded the **highest global rate of child wasting at 18.7%** in 2024, affecting over 21 million children. However, NFHS-6 reports **severe wasting** has declined from 7.7% to **5.2%**.
- **Anemia:** Over half of Indian women (15-49 years) are anaemic, reaching **53.7% in 2023**, impacting 203 million women.
- **Obesity:** A rising crisis where obesity now affects **30.7% of women** and **27.3% of men** according to NFHS-6, up from 24% and 22.9% respectively in NFHS-5.
- **Breastfeeding:** A concerning trend shows **exclusive breastfeeding** for infants under six months dropped to **55.8%** in NFHS-6 from 63.7% in NFHS-5

Causes of Malnutrition

- **Poverty and Unaffordability:** Nearly **35.5% of the** population cannot afford a **nutritionally adequate diet** (*State of Food Security and Nutrition in the World 2026, FAO*).
- **Poor Dietary Diversity:** Diets are heavily cereal-based (rice/wheat), which constitute **70-75% of intake**, leaving them deficient in proteins and micronutrients.
  - **Example**: Over **80% of children** aged 6-23 months did not consume any protein-rich food for an entire day ("zero-protein").
- **Environmental and Physiological Factors:** Poor sanitation leads to **Environmental Enteric Dysfunction (EED)**, where an inflamed intestine cannot absorb nutrients even if food is consumed.
- **Decline in Optimal Breastfeeding Practices:** Recent data from **NFHS-6 (2023-24)** reveals a concerning decline in **exclusive breastfeeding** for infants under six months, which dropped to **55.8%** from 63.7% in NFHS-5.
- **Rising Consumption of Ultra-Processed Foods (UPFs):** Indian diets are shifting away from traditional patterns toward calorie-dense, nutrient-poor UPFs, contributing to both hidden hunger and the rising obesity crisis. (Eco Survey 2025-26)
- **Social Norms:** Low maternal literacy, child marriage, and inadequate childcare practices contribute to intergenerational malnutrition.

Consequences of Malnutrition

- **Impaired Human Capital:** Malnutrition during the first 1,000 days causes irreversible damage to **cognitive ability and physical growth**. Stunted children often have reduced learning capacity, which translates to **lower school readiness and reduced economic productivity** in adulthood.
- **Increased Disease Burden:** The "double burden" links undernutrition in childhood to a higher risk of NCDs later. Rising obesity is directly linked to the surge in **diabetes, heart disease, and hypertension** in India.
- **Higher Mortality:** Wasted children are significantly more vulnerable to frequent infections and death.
- **Surge in Non-Communicable Diseases (NCDs):** With obesity now affecting **30.7% of women and 27.3% of men**, there is a significantly increased risk for chronic conditions such as **diabetes, heart disease, and hypertension**
- **Long-term Economic and Human Capital Loss:** Malnutrition during the "window of opportunity" (the first few years of life) leads to **irreversible cognitive impairment and lower school readiness.**

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Measures to Tackle the Nutrition Crisis

- **POSHAN Abhiyaan (National Nutrition Mission):** Focuses on technology-based monitoring (Poshan Tracker) and inter-ministerial convergence to reduce stunting and anemia.
- **Integrated Child Development Services (ICDS):** Provides supplementary nutrition and health check-ups through **Anganwadi centers**.
- **Mid-Day Meal Scheme (PM POSHAN):** Provides cooked meals in schools to address classroom hunger and improve nutritional status.
- **Anemia Mukt Bharat:** A specialized strategy to reduce anemia through prophylactic iron and folic acid supplementation.
- **FSSAI Initiatives:** Programs like **'Eat Right India'** and front-of-pack labeling to combat the obesity crisis.

Why Measures Fail to Address the Crisis

- **Implementation Gaps:** Frontline workers often face **limited digital literacy** and inconsistent data reporting, hindering the effectiveness of technology-led missions like POSHAN Abhiyaan.
- **Inadequate Funding:** India's public health expenditure remains low at approximately **1.3% of its GDP**.
- **Quality and Infrastructure:** Anganwadi centers often lack the necessary infrastructure, and there are persistent concerns regarding the **quality of food** provided in supplementary programs.
- **Supply-Side Focus:** Most programs focus on providing food rather than addressing the **demand-side factors** like maternal empowerment and behavior change

Way Forward

- **Granular/Community Approach:** Empowering **Self-Help Groups (SHGs)** to drive community-level awareness and maternal caregiving behaviors. (NITI Aayog)
- **Nutritional Literacy:** Integrating nutrition education into schools and mass media to promote healthy dietary choices.
- **Agriculture-Nutrition Linkage:** Shift public procurement (PDS) and mid-day meals to include **locally produced millets, pulses, and oilseeds** rather than focusing solely on rice and wheat.
- **Nutritional Labeling:** Enforce mandatory **Front-of-Pack Labeling (FoPL)** and nutritional scoring on all packaged foods to help consumers identify high salt, sugar, and fat content.
- **Taxation and Incentives:** Implement **higher taxes on ultra-processed foods (UPFs)** and sugar-sweetened beverages while using GST revisions to make healthy, nutrient-dense products more affordable.
- **Improved Monitoring:** Moving beyond height and weight to measure **gut health and nutrient absorption** through better primary healthcare screening.
- **WASH Integration:** Harmonize nutrition interventions with services to prevent the cycle of infection of water**, sanitation, and hygiene (WASH)** and nutrient malabsorption.

Eradicating hunger demands moving beyond welfare to rights-based entitlements. Integrating PDS digitisation, bio-fortified crops and climate-resilient farming will ensure zero hunger, making India a global leader in food justice and human dignity.

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